Mental Health notes

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INTRODUCTION TO MENTAL HEALTH

OBJECTIVES

- By the end of this unit the student nurse should be able to:-
- Define mental health and mental illness
- Outline the characteristics of mental health
- Outline characteristics of mental illness
- Review psychoanalytic and psychosocial theory of development (psychology)

Objectives ct…

- Describe history of psychiatry/mental health
- Discuss trends in psychiatry nursing in Kenya
- Highlight factors contributing to mental illnesses
- Explain factors that contribute to mental health
- State principles of psychiatric nursing
- Highlight roles of a psychiatric nurse

Objectives ct…

- Discuss the admission process of mentally ill patients
- Highlight the mental health act cap 248
- Describe various treatment modalities used in psychiatry
- Highlight classifications of mental health disorders/psychiatry disorders

- Recognize and manage common mental health conditions
- Discuss community mental health

Definitions of mental health

- Mental health is defined as the simultaneous success at working, loving, and creating with the capacity for mature and flexible resolution of conflicts between instincts, conscience, other people and reality (Evelyn and Wasili, 1986).

- According to the World Health Organisation (WHO), mental health is a state of emotional well-being which enables one to function comfortably within society and to be satisfied with one’s own achievements.

ct

- Mental health also refers to the ability of the individual to carry out their social role and to be able to adapt to their environment (Johnson, 1997).

ct

- A state of emotional and psychological well being in which an individual is able to use his or her cognitive and emotional capabilities,function in the society and meet the ordinary demands of everyday life.

ct

- A branch of medicine that deals with the achievement and maintance of psychological wellbeing.

Definitions of mental illness

- A mental illness is defined as a disorder with psychological or behavioural manifestations and/or impairment of functioning due to a social, psychological, genetic, physical, chemical or biological disturbance (Evelyn and Wasili, 1986).

ct

- Any condition that affects a persons mood, behaviour or thoughts in a way that is distressing or impairs functioning.

Definition of pyschiatry

- Psychiatry- the branch of medicine that deals with the diagnosis, treatment and prevention of mental and emotional disorders.
- It was first coined by Johann Christian Reil in 1808.It comes from the greek word “psyche” meaning **soul** and “iatriyos” meaning **to heal.**

Factors contributing to mental health

- Good nutrition-balanced diet and foods rich in omega 3,6 and 9.Fish,nuts,proteins.
- Parental upbringing-good parenting results to better mental health during childhood and adulthood.
- Ethical and moral upbringing-whats taught at school level and community as people grow up.
- Physical health-physically fit individuals are likely to have sober minds

- Exercise- helps in relaxation and proper blood circulation to the brain.Relieves stresses and calms individuals.
- Personal efforts to practice mental hygiene-keeping off mind from unnecessary life stressors.Good thinking and watching right things.
- Healthy environment-calm and unpolluted environment.free from toxic fumes and substances,free from noise.

Characteristics of mental illness

A mentally ill person may have at least one of the following characteristics:

- Being dissatisfied with one’s abilities and accomplishments
- Having ineffective or unsatisfying interpersonal relationships
- Dissatisfaction with one’s place in the world
- Having ineffective coping/adaptation mechanisms and lacking personal growth

Factors that contribute to mental ilness/causes of MI

- A complex interplay of many factors cause mental illness. Contributing factors include:-
- Biological factors
- Psychological factors
- Environmental factors
- Nutritional factors
- Socio-economic factors

Biological factors

- Some mental illness have been linked to an abnormal balance of neurotransmitters.When these chemicals are out of balance or not working properly,messages may not be transmitted properly through the brain,leading to symptoms of mental illness.

- In addition defects in or injury to certain areas of the brain have been linked to certain conditions.
- Other biological factors include:
- **Genetics/Heredity**- many mental illnesses run in families. Susceptibility is passed on in families through genes.

- Infections- certain infections are linked to brain damage, hence developing mental illness.streptococcus bacteria has been linked to the development of obsessive complulsive disorder and other mental illnesses in children.

- Brain defects or injury-some injury occurs due to birth asphyxia when brain is deprived oxygen resulting to death of cells.This can result to autism or cerebral palsies later in life.

Psychological factors

- This includes:-
- Severe psychological trauma suffered as a child,such as emotional,physical or sexual abuse.
- An important early loss e.g loss of a parent
- Neglect of a child especially in early years.
- Poor ability to relate with others.

Environmental factors

- Toxic environment or environment with fumes can trigger mental illness.
- Very noisy environments can contribute to stress and altered sleep patterns which after a long time could lead to depression.
- Fumes from marijuana can affect the non-passive smokers and can trigger psychotic episodes.

Socio-economic factors

- Violence within families and peer pressure influence(substance use) can predispose one to mental illness.
- Sudden economic/finacial shifts can trigger depressive episodes.
- Social expectations like “thinness for beauty” can subject people to eating disorders.

Nutritional factors

- Poor nutrition can cause some eating disorders like-anorexia nervosa and bulimia nervosa.

- The causes of mental illness can be classified into three categories:
- Predisposing factors
- Precipitating factors
- Perpetuating factors

- **Predisposing Factors**
  These factors determine the likelihood of one getting a mental illness. Usually they are adverse experiences one undergoes in early life. They include physical, psychological and social factors in infancy and early childhood.

- **Precipitating Factors**
  These are events that take place shortly before the onset of a disorder. Physical precipitants include cerebral tumours, malaria or drug abuse. Social and psychological precipitants include misfortunes such as loss of a job, losing a loved person, or sudden change in routine activities.

- **Perpetuating Factors**
  Once the disorder has been triggered, these factors do prolong the course of the disease. They are secondary in nature since they may appear long after the original predisposing factors have been treated. Examples include secondary demoralisation and withdrawal from social activities.

Factors that Influence Attitudes towards Mental Health and Mental Illness

- **Culture**
  The way people think, behave or feel is shaped by their culture. Culture also determines the features of insanity, for example, who is labelled as insane and under what circumstances.
- What is considered insane in one culture may be considered perfectly normal in another. Culture also gives guidelines on the nature of treatment and the identity of the helper.

- **Education**
  The level of education also influences attitudes towards mental health and mental illness.
- An educated person has a better understanding of health and mental illness, thus making their attitude more positive.

- **Health Beliefs**
  These will determine whether the individual's attitude is positive or negative. It will depend on how the patient explains the illness to themselves, that is, whether they believe in germ theory, evil spirits or an imbalance of some kind.

- **Religion**
  A patient’s reaction to mental illness will often depend on whether or not the patient believes in God or a particular religion, for example, some religions believe that ill health is caused by evil spirits. Usually, religion encourages the followers to be empathetic to sick people.

Principles and Qualities of Psychiatric Nursing

- **Respect for the Patient**
  This is achieved by accepting the patient as they are.
  The therapist should take time to listen to the patient and provide privacy for all conversations.

  Minimise situations and experiences that might humiliate the patient and be honest in providing information on medicines, privileges, length of management and stays in hospital if indicated.

- **Availability**
  The nurse must be constantly available to assist the patient to attain their basic needs and alleviate suffering.
- **Spontaneity**
  You should avoid being overly formal. Instead, you should be comfortable with yourself, be flexible and aware of the therapeutic goals.

- **Acceptance**
  Even if the patient behaves in a way that does not please the nurse, they should be accepted as they are, but taking care not to reinforce their behaviour.
- **Sensitivity**
  You should do your best to show genuine interest and concern.
  You should be persistent and patient even if no observable improvement is made

- **Accountability**
  Since mentally ill patients are vulnerable due to their distorted thinking and behaviour, accountability is required more in a psychiatric setting than any other type of health care (Peplau, cited by Wilson and Kneisl, 1988). You are also accountable to yourself as well as professional colleagues and peers.

- **Empathy**
  This is the process of putting yourself in another’s shoes and remaining emotionally detached. The nurse should strive to understand the patient’s perspective, and work toward mutually developed goals.
- The most important function of empathy is that it enables you to give the patient the feeling of being understood and cared about.

- **Self-understanding**
  This involves recognition and acceptance of your own behaviour and how it affects your relationship with other people.
- This will inevitably help you, as a therapist, to understand other peoples' behaviour, needs and problems.

- **Permissiveness and Firmness**
  Although you have been told to accept the patient as they are, this does not mean that you are in a position to allow them to do whatever they like. The therapist is expected to set limits and to be firm in implementing them.

- **Skill in Observation**
  It is important for a psychiatric nurse to be alert and observant at all times of the patient’s behaviour, attitudes and how they react to staff, relatives and fellow patients.

HISTORICAL TRENDS IN PSYCHIATRIC NURSING

- **Demonological period**
  - King Nebuchadnezzar ate grace behaved like an ox
  - Ajax impaled himself , believed he was tormented by demons
  - Treatment was harsh ,beating , locking up in dark room, throwing into rivers , ponds and forests

Political period

- Associated with king Edward 11 of England
- Law passed in parliament to protect the mentally ill.
- In 1403,sister of the order of Saint Mary managed to start a facility to care for mentally sick at Bedlam

Cont…

- A facility that accommodated six .
- Thought of metal illness as demonic continued
- Treatment remained as in demonological period.
- Facility were dark , humid and infested with lice

Cont.

- 18th century 1st qualified nurse was appointed to look after mentally ill by Edward Tyson
- Qualified nurses were harsh to patients
- Both men and women were housed together

Cont..

- Overcrowded ,disease outbreaks were common
- Members of public visited the facilities to view patients as a form of entertainment.

Cont..

- At St. Lukes hospital this form of entertainment was banned
- Members of public were issued with a ticket
- They were to see patients in the presence of an attendant

Humanitarian period

- Reforms of system began in France , followed by Britain and finally America
- 1793 at Bicetre Hospital in Paris Dr.Philippe Pinepil unchained a group of patients who were in chains for 30 years.
- Dr. Philippe advocated kindness for mentally ill persons

Cont…

1796 William Tuke started reforms in Britain

He advocated for human treatment of the mentally sick.

Introduced **Occupational Therapy**

Men were involved in gardening and women in sewing.

Cont…

- Men and women assisted attendants in their daily activities
- Members of the staff were not specifically trained to deal with mental health issues
- 1808 a bill was passed to regulate treatment of mental patients

Cont..

- Dorothea Lynda Dix introduced reforms of mentally ill care in America
- Visited Britain earlier before reforms and witnessed great improvement of patients in Britain

Cont..

- 1841 she passed a bill to regulate treatment of mentally sick in America
- 1853 Dr. W.A.F Browne started informal mental health lectures to nurses

Cont..

- 1882,formal training on mental health began in America at Mclean Hospital , Boston

- 1884 formal training in mental health started in Germany
- Other countries followed to train.

Scientific period

- Modern treatments based on scientific findings were developed.
- Many forms of treatments were discovered. hydrotherapy, insulin therapy and leucotomy were abandoned later

Cont..

- Surgical cutting of white nerve fibres
- Within the brain , especially prefrontal lobotomy

Cont..

- Physical treatments-chemotherapy and electroconvulsive therapy developed
- Psychotherapy , behavior therapy , occupational therapy and rehabilitation were also developed.
- Integration of mental health services with other health services

Cont..

- Trained manpower was developed to care for mentally sick
- Men and women were managed separately
- Children managed separately
- With available resources different health conditions were managed separately in different units.

DEVELOPMENT OF PSYCHIATRY AND MENTAL HEALTH SERVICES IN KENYA

- 1910 Mathare hospital was started as a lunatic asylum
- Before it was a smallpox isolation centre
- 1924 asylum was named Mathare

Cont..

- Custodial care in dark gloomy and damp atmosphere

- Europeans Asians and Africans were managed separately
- Care determined by one’s racial background

Cont…

- 1962 decentralization of mental health care was started.
- 1962 Nakuru 22bed psychiatric unit was built.
- 1963 Machakos, Nyeri and Muranga were started
- 1965 Kakamega and Port Reitz were started

Cont..

- Currently all provincial hospitals have operational psychiatric units
- Some district hospitals have units
- Mathare hospital is being redeveloped

Cont..

- Future is to have community psychiatric services all other the country.
- In Nairobi psychiatric services started in 1983

Training of mental health workers

- 1961 training of enrolled psychiatric nurses began
- Later it was turned into post-enrolled psychiatric nursing course.

Cont..

- 1963 1st two registered psychiatric nurses were trained
- 1979 post-basic diploma in psychiatric nursing started

Cont..

- Between 1972 and 1982 most psychiatrists received overseas training at institute of psychiatry in England.
- 1982 training of psychiatrists started in Nairobi university

THEORIES OF PERSONALITY DEVELOPMENT

Theories of personality development

- Psychoanalytic theory by Sigmund Freud
- Social learning theory by Erik Erickson

Psychoanalytic theory

- The theory posits that all human behaviour is determined by earlier life events and that human behaviour is driven by energy known as libido. It is also argued that people function at three levels of awareness:

Levels of awareness

- Conscious level where immediate events and perceptions are stored
- Pre-conscious level where thoughts and feelings are not accessible with ease
- Unconscious level where material is not accessible most of the time

- According to the proponents of psychoanalysis, people develop through five stages:

Stages

Stage	Age	Focus or Interests
Oral	0 - 1	Focus on mouth, teeth and gums
Anal	1 - 3	Focus on control and self-control (primarily toilet training and defecation)
Phallic	3 - 6	Focus on genital stimulation and gender differentiation
Latency	7 - 11	Energy focused on non-sensual activity
Genital	12+	Development of mature sexual interests

Ct…

- **One: Birth to 1 year**
- Oral phase, where sexual satisfaction is obtained through the mouth by actions of sucking or biting.
- **Two: 1 year to 3 years**
- Anal phase, where sexual satisfaction is obtained through the anus.

- **Three: 3 years to 6 years**
- Phallic phase, during which interest in the genitalia is developed and the individual acquires gender roles.
- **Four: 7 years to 11 years**
- Latent phase, where there is no sexual interest, this divides boys and girls into different groups.

- **Five: 12 years and onwards**
- Genital phase, when the capacity for objective and mature sex is developed.

- According to this theory, the human personality is made up of three hypothetical structures:
- Id
- Ego
- Superego

ID
Ego
Super Ego

- **Id:** The id is the most primitive and is driven by impulses. It demands immediate gratification of the needs.
- **Super Ego:** The super ego, whose main function is to oppose the id. It contains values, legal, moral regulations, and social expectations.

- **Ego:** The ego, which unlike the id, is in contact with reality. It is able to delay the satisfaction of a need until an appropriate time, place, or object is available. It mediates between the id and the super ego.

Socio-learning theory

- Erikson postulated eight human developmental stages which one has to go through in normal life. He called them psychosocial stages. Each stage is characterised by tasks or ego qualities, which one has to develop in normal life.

Trust versus Mistrust

- This is attained during infancy. Trust is developed when the mother breastfeeds the infant as well as meeting other needs of the infant such as changing wet diapers. The approximate age at which the individual goes through this stage is 0 - 18 months.

Autonomy versus Shame and Doubt

- During this period the child wants some independence from the mother. This happens approximately at the age of 18 months to
- 3 years and this period is referred to as toddlerhood.

Initiative versus Guilt

- This occurs at approximately 3 to 6 years, otherwise known as the pre-school years. The child is able to initiate action on its own in an effort to manipulate the environment.

Industry versus Inferiority

- This takes place during school years between the ages of 6 to 12 years. The child is able to acquire skills in sports, calculate in figures and language skills are well developed.

Identity versus Role Confusion

- This occurs around the age of 12 to 20 years and is referred to as adolescence. During this time an individual acquires an identity as a male or female corresponding to specific roles in the society.

Intimacy versus Isolation

- This occurs at an approximate age of between 20 to 40 years.
- Here, one has productive work and has satisfactory sexual relations with an intimate member of the opposite sex.

Generativity versus Stagnation

- This usually occurs between the ages of 45 to 65 years. During this period, one is involved in establishing and guiding the next generation. Productivity and creativity is at its peak.

Integrity versus Despair

- This refers to the phase from around the age of 65 till death.
- During this time one develops new and different love for one’s parents. One also develops emotional integration and is able to defend the dignity of one’s own life-style against threat.
- The individual is capable of fellowship and will often take leadership responsibilities in the community.

SYMPTOMATOLOGY OF MENTAL ILLNESS

- **symptoms affecting consciousness**
  - **Confusion**; disturbance of orientation to place ,person and time
  - **Stupor** ;lack of reaction to surroundings

Cont..

- Lack of awareness of the surroundings
- No response to stimuli
- Not aware of what is going on in the environment

Cont..

- **Delirium** ; bewildered , restless . Dream like change in consciousness
  - Impaired reality testing
  - Anxiety , confusion ,disorientation , restless
  - Hallucinations experience

Cont..

- **Depersonalization**; disturbances in the way one experiences the self
- **Derealization** ; disturbance in the way one experiences one's physical environment

Cont..

- These two are common in one experiencing psychological stress due to bereavement , severe illness, loss of income or livelihood

Symptoms affecting mood

- **Mood** is the sustained and pervasive inward feeling
- **Affect** is the somatic and behavioral changes
  - Outward expression of the emotion

    emotion is the feeling or response to sensory input from the external enviroment or mental images

Cont..

- Mood is influenced by patient's experiences and expectations as well as presence of the disease
- **Depression**; a pathological feeing of sadness
  - Disease process of the mind.

Cont..

- An illness or a symptom common in depressive illness
- One perceives loss in life , person, property or status

Cont..

- **Elation** ; Air of confidence and enjoyment
- Associated with increased motor activity
- One is cheerful and has happy mood
- **Inappropriate (incongruous**);mood is not in harmony with patient ideas.

Cont..

- **Flat (blunt)-**does not show any feelings
- **Apathy**-blunted emotion associated with loss of energy and drive

Symptoms affecting motor activity

- Externally observable behaviors
  - Impulses
  - Motivations
  - Drive instincts
  - Wishes

Cont..

Hyperactivity-restless, aggressive and active

common in maniacs and substance abuse

Cont..

- **Waxy flexibility**; patient maintains body position in a particular posture for sometime
  - Common in catatonic schizophrenia
  - **Negativism** ; frequent opposition to suggestions and instructions
  - **Mannerism ;** habitual involuntary movement and attitudes

Cont..

- **Echopraxia**; imitation of one person's movement by another
- **Compulsion**; uncontrollable urge to perform and act repeatedly

  -Common in obsessive-compulsive neurosis

Symptoms affecting thinking process

- **Flight of ideas** ; rapid thought manifested by verbalization
  - Shifting of ideas which tend to be connected to one another
  - Common in maniacs

Cont..

Neologisms ; new words created by a patient or a normal word used to mean a different thing

Common in schizophrenia

Cont…

- **Word salad**; several words put together with no clear meaning
- **Thought block** ;sudden interruption in the stream of thought before an idea or thought is completed

Cont..

- **Incoherence** ;thinking that does not flow logically
- **Echolalia** ;psychopathological repetition of words or phrases said by another person

cont

- **Thought broadcasting** ;patient feels that he/she shares hi/her ideas with other people.
  - One may hear his/her ideas loudly from a radio, telephone
  - Common in Schizophrenia

Symptoms affecting thought content

- **Delusion** ;false belief firmly held by patient and cannot be removed by argument or logic
  - It is out of keeping in educational or cultural background.

Cont..

- **Grandeur** ; exaggerated self importance , ability , wealth ,association with important people
- **Persecution** ;one feels he/she is being persecuted by some people

Cont..

- **Worthlessness** ;feeling unworthy ,remorse , one can easily commit suicide
  - Common in depressive illness

Cont..

- **Nihilistic** ; patient believes that he does not exist , extreme pessimism
- **Hypochodriac** ;belief in ones illness contrary to all medical evidence

Cont..

- **Religious** ;special relation with God
- **Jealousy** ;doubt about the fidelity of the sexual partner

Symptoms affecting sensory perception

- **Hallucinations** ;false sensory perception not associated with real external stimuli
- Affects five common senses
  - Visual ,tactile
  - Olfactory , Gustaltory
  - Auditory

Cont..

- **Illusions** ;false sensory perception with external stimuli

  _Misinterpretation of objects

  –One sees a snake instead of a stick

  –Common in organic psychosis

PSYCHIATRIC ADMISSION AND DISCHARGE

Based on the mental Health Act cap 248

Objectives

- At the end of this section you will be able to:
  - Apply the knowledge acquired on legal aspects in admission and discharge of psychiatric patients
  - Demonstrate skills in history taking
  - Demonstrate skills in mental status examination

MENTAL HEALTH LEGISLATION

- Mental health Act of the laws of Kenya CAP 248 1989
- Revised in 1991
- **Guiding principles**
  - Regulates mental health care environment in the best interest of the ;

Cont..

- Patient/client/customer/user

- Safeguards the human rights of the patient as guaranteed
- Under international human right instruments

Cont..

- Regional charters as well as national constitutions and legislations
- Sets obligations of mental health care providers

Cont..

- Regulates;
  - Access to mental health care
  - Provision of mental health care
  - Treatment for voluntary and involuntary patients
  - Statutory treatment through criminal justice systems

Cont..

- Regulates the manner in which the property of mentally sick is dealt with by the courts of law.

OBJECTIVES OF MENTAL HEALTH ACT

- **Demystify** mental illness
- **Destigmatise** mental illness
- **Descriminate** mental illness
- **Decentralize** mental health services
- **Make mental illness a community issue**

SCHEDULE FORMS FOR ADMISSION

- **M.OH.613**; application for voluntary admission
- **M.O.H.614**; application for involuntary admission
- **M.O.H.615** ;application recommendation for involuntary treatment (doctor)

Cont..

- **M.O.H.616;** an order directing into care of relative or friend
- **M.O.H.637**; application for voluntary admission (under 16 years)
- **M.O.H.638**; application for emergency admission

Cont..

- **M.O.H.639;** report on death or departure
- **M.O.H.640**; warrant of removal of involuntary patients to other countries
- **M.O.H.641**; application to extend the stay of a foreign patient in the institution

Mental Health Act (Cap 248)

- **Mental Health Act (Cap 248)**
- The Mental Health Act is an act of parliament meant to amend and consolidate the law relating to the care of persons who are suffering from mental disorders, or mental sub-normality with mental disorder, for the custody of these persons, management of their properties, management and control of a mental hospital and for custodial purposes.

Cont…..

- The act provides for the procedures to be followed for reception into a mental hospital.
- It also stipulates that no person shall be received or detained for treatment in a mental hospital, unless they have been received and detained under this act or under criminal procedure code.

Sections of the Mental Health Act

Sections of the Mental Health Act

Part V - Voluntary Patients (voluntary admission)

- Any person who has attained the apparent age of sixteen years, decrees to voluntarily submit themselves to treatment for mental disorder, and who makes to the ‘person in charge’ a written application in duplicate in the form prescribed, may be perceived as a voluntary patient into a mental hospital

Cont…..

- The person fills in a form **MOH 613**, in duplicate
- Any person who has not attained the apparent age of sixteen years and whose parent or guardian desires to submit them for treatment for mental disorder, may if the guardian or parent makes to the ‘person in charge’ of a mental institution, a written application in duplicate in the prescribed forms, be perceived as a voluntary patient. In such cases forms **MOH 637** in duplicate should be filled and signed by the guardian or the parent.

Part VI - Involuntary Patients (Involuntary admission)

- **Part VI - Involuntary Patients (Involuntary admission)**
- Involuntary patients are those who are incapable of expressing themselves as willing or unwilling to receive treatment. They require the forms **MOH 614** to be filled in duplicate by the husband, wife or relative of the patient, indicating the reasons why they are applying for admission.

Cont…..

- The patient is admitted for a period of not more than six months. The ‘person in charge’ can prolong this period by six more months provided the total period does not exceed twelve months.
- An **MOH 615** form should be filled by the doctor indicating why he thinks that the patient can benefit from the treatment.

Cont…..

- Both the **MOH 614** and **MOH 615** forms must reach the hospital within 14 days of the date they were signed, otherwise they become invalid.

Part VII – Emergency Admissions

- **Part VII – Emergency Admissions**
- A police officer, chief or assistant chief can arrest any person(s) who is found to be dangerous to him/herself or others, and take them to a mental hospital for treatment within 24 hours or a reasonable time. The patient should be reviewed after 72 hours and can be discharged if found to be of sound mind.

Cont…..

- If found to be of unsound mind, the patient may be admitted for treatment as an involuntary patient.
- For the purposes of admission, the form **MOH 638** must be filled in by the police officer or an administrative officer.

Part VIII - Admission and Discharge of Members of the Armed Forces

- **Part VIII - Admission and Discharge of Members of the Armed Forces**
- Any member of the armed forces may be admitted into a mental hospital for observation, if a medical officer of the armed forces, by letter addressed to the 'person in charge', and certifies that:

i. The member of the armed forces has been examined within a period of 48 hours before issuing the letter

ii. For reasons recorded in the letter, the member of the armed forces is a proper person to be admitted to a mental hospital for observation and treatment

Cont…..

- A member of the armed forces may be admitted for an initial period of 28 days from the date of admission, that period may be extended if, at or before the end of 28 days.
- Two medical practitioners, one of whom shall be a psychiatrist, may recommend the extension after re-examining the patient.

Cont…..

- The said patient can be discharged if two medical practitioners, one of whom is a psychiatrist, by a letter addressed to the ‘person in charge’, certifying that they have examined the member of the armed forces within a period of 72 hours before issuing the letter.

Part IX - Admission of a Patient from Foreign Countries

Part IX - Admission of a Patient from Foreign Countries

- According to this section of the act:

i. No person suffering from mental disorder shall be admitted into a mental hospital in Kenya from any state outside Kenya except under Part IX of M.H.A.

Cont…..

ii. Where it is necessary to admit a person suffering from mental disorder from any foreign country into any mental hospital in Kenya for observation, the government or other relevant authority in that country shall apply in writing to the mental health board to approve the admission, no mental hospital shall receive a person suffering mental disorders from a foreign country without the board's written approval.

Cont…..

- On the admission of a person into a mental hospital under this section, the ‘person in charge’ shall within 72 hours or such longer period as the board may approve

i. Examine the person or cause the person to be examined to determine the extent of the mental disorder and the nature of treatment and

ii. Within that period forward to the Board a report on the findings, together with the warrant or other document from the foreign country concerned accompanying the patient/person.

Cont…..

- A person shall not be detained in a mental hospital under this section for a period longer than two months from the date of admission to the mental hospital unless the board, on application in the prescribed forms by the ‘person in charge’, approves.

PART X-DISCHARGE AND TRANSFER OF PATIENTS

- **21.** The person in charge of a mental hospital
- May by order in writing and upon
- The recommendation of the medical
- Practitioner in charge of any person admitted to the mental hospital

Cont..

- Order the discharge of any person’s
- Admitted to the hospital and that
- Person thereupon be discharged as having
- Recovered from mental disorder :

Cont..

- Provided that-

  i. An order shall not be made under this section for a person who is detained under the Criminal Procedure Code ; And

  (ii) this section shall not prejudice The Board's powers under section 15

Cont..

- (1) if any relative or friend of a person
- Admitted into any mental hospital
- Under this Act desires to take the person
- Into custody and care , he may apply to

Cont..

- The person in charge , who may subject
- To subsection (2), order that the person

Cont..

- Be delivered into custody and care
- Of the relative or friend upon
- Such terms and conditions to be compiled
- With by the relative or friend.

Cont..

- (2) In the exercise of his powers under
- Subsection (1) the person in charge shall
- Consult with medical practitioner in charge

Cont..

- Of the person’s treatment in the
- Mental hospital and the Board or
- The relevant district mental health council

Cont..

- Which is performing the Boards functions
- Under section 7 (1):
- (2) An insurance company which makes
- Any insurance policy which expressly

Cont..

- Excludes or puts restrictions on the
- Treatment of any person suffering
- from mental shall be guilty of an offence

PART X1V-OFFENCES

- **47**. Any person who , not being a medical
- Practitioner ,knowingly and wilfully signs
- A medical certificate for the purposes
- Of any of the provision of this Act
- Shall be guilty of an offence

Cont..

- **48.** Any medical practitioner who knowingly,
- Wilfully or recklessly , certifies anything
- In a certificate made under this Act
- Which he knows to be untrue
- Shall be guilty of an offence

Cont..

- **49**. Any person who wilfully assists the escape
- Of any person suffering from mental
- Disorder being conveyed to from, or
- Who harbours any person suffering
- From mental disorder whom he knows
- Has escaped from a mental hospital

Cont..

- Shall be guilty of an offence
- **50.**Any person in charge of, or any person employed at , a mental hospital who through
- Wilfully neglect or connivance permits any patient in a mental hospital
- To leave such hospital other than

Cont..

- Under this Act or any other law for the
- Time being in force shall be guilty of an offence
- **51.** Any person in charge of , or any person employed at , a mental hospital who
- Strikes , ill treats , abuses or wilfully

Cont..

- Neglects any patient in the mental hospital
- Shall be guilty shall be guilty of an offence;
- But nothing in this section shall be deemed to make an offence for the person in charge of,

Cont..

- But nothing in this section shall be deemed
- to make it an offence for the person in charge of
- Or any person employed at , a mental hospital
- To take steps he considers necessary in the

Cont..

- Patient to prevent the patient from causing physical injury to himself or to others
- **52.** Any person who ,without the content
- Of the person in charge gives , sells or
- barters any articles or commodity of any

Cont..

- Of any kind to any patient in a mental hospital
- Whether inside or outside the grounds of
- Mental hospital, shall be guilty of an offence

Cont..

- **53**. Any person who is guilty of an offence
- Under this Act, or who contravenes any of the provisions of this act shall , where no

Cont..

- Other penalty is expressly provided ,
- be liable on conviction to a fine not
- exceeding ten thousand shillings
- Or to exceeding twelve months or to both.

Kenya board of mental health

- The act also provides for the establishment of a board, that is, the Kenya board of mental health for the purposes of administering this act.
- The members of the Kenya board of mental health include:

- Chairman, who can be the Director of Medical Services (DMS) or the Deputy Director of Medical Services and is appointed by the cabinet secretary
- Psychiatrist (medical practitioner) appointed by the cabinet secretary

- One clinical officer with training and experience in mental health care appointed by the cabinet secretary .
- One psychiatric nurse with experience in mental health care, appointed by the cabinet secretary.
- The commissioner of social services or their nominee appointed by the cabinet secretary.

- Director of education or their nominee appointed by the cabinet secretary.
- A representative from each province in Kenya, being resident in the provinces, appointed by the cabinet secretary.

- The board may co-opt any person whose skills, knowledge or experience may be useful.
- It may also establish a committee for better function and regulate its own procedure.

Functions of Kenya board of mental health

- To coordinate mental health activities in Kenya.
- To advise the government on the state of mental health and mental health care facilities in Kenya.

Ct…

- To inspect mental health care hospitals to ensure that they meet the prescribed standards.
- To approve the establishment of mental health care hospitals.
- To assist when necessary in the administration of mental health hospitals.

Ct…

- To receive and investigate any matters referred to it by a patient or relative of a patient concerning the treatment of the patient at a mental health hospital and, where necessary, to take or recommend to the minister any remedial action.
- To advise the government on the care of the persons suffering from mental sub-normality without mental disorder.

Ct…

- To initiate and organise community or family based programmes for the care of persons suffering from mental disorder, and to perform such other functions as may be placed upon it by this act or under the law.

Criminal procedure code (Cap 75)

- **Cognizable offence”** means an offence for which a police officer may, in accordance with the First Schedule or under any law for the time being in force, arrest without warrant;
-

Ct..

- “Drug related offence” means any specified in Part V of the Dangerous Drugs Act (Cap. 245) and includes the possession, manufacture, distribution or receipt of any drug of any quantity whatsoever;

Ct…

- “Non-cognizable offence” means an offence for which a police officer may not arrest without warrant

Powers of the court

- 4. Offences under Penal Code
- Subject to this Code, an offence under the Penal Code (Cap. 63) may be tried by the High Court, or by a subordinate court by which the offence is shown in the fifth column of the First Schedule to this Code to be triable.

Ct…

- 5. Offences under other laws
- (1) An offence under any law other than the Penal Code (Cap. 63) shall, when a court is mentioned in that behalf in that law, be tried by that court.
- (2) When no court is so mentioned, it may, subject to this Code, be tried by the High Court, or by a subordinate court by which the offence is shown in the fifth column of the First Schedule to this Code to be triable.

Ct…

- 6. Sentences which High Court may pass

  The High Court may pass any sentence authorized by law.

Ct..

- 7. Sentences which subordinate courts may pass
- (1) A subordinate court of the first class held by—
- (a) a chief magistrate, senior principal magistrate, principal magistrate or senior resident magistrate may pass any sentence authorized by law for any offence triable by that court;
- (b) a resident magistrate may pass any sentence authorized by law for an offence under section 278, 308(1) or 322 of the Penal Code or under the Sexual Offences Act, 2006.

Ct…

- (2) Subject to subsection (1), a subordinate court of the first class may pass the following sentences in cases where they are authorized by law—
- (a) imprisonment for a term not exceeding seven years;
- (b) a fine not exceeding twenty thousand shillings;

Ct…

- (3) A subordinate court of the second class may pass the following sentences in cases where they are authorized by law—
- (a) imprisonment for a term not exceeding two years;
- (b) a fine not exceeding ten thousand shillings;

Further reading

- Download a copy of the Cap 75 of Kenya.

Self assessment

- Highlight five functions of the Kenya mental health board.
- Outline five offences under the mental health act.
- Explain voluntary mode of admission.

ASSESSMENT OF A PSYCHIATRIC PATIENT

HISTORY TAKING IN PSYCHIATRY

- HISTORY TAKING IN PSYCHIATRY
  i. Personal data/demographic data
  ii. Chief complains
  iii. History of presenting illness
  iv. Past medical and psychiatric history
  v. Family history
  vi. Personal development history: delivery, milestones, infancy and early childhood
  vii. Social history
  viii. Sexual history

Mental Status Examination

Mental Status Examination

- **Appearance**
- Dress, grooming, hygiene, cosmetics, apparent age, posture, facial expression.
- **Behaviour/activity**
- Hyperactivity or hyperactivity, rigid, relaxed, restless, or agitated motor movements, gait and coordination, facial grimacing, gestures, mannerisms, passive, combative, bizarre.

Cont…..

- **Attitude**
- Interactions with interviewer: - Cooperative, resistive, friendly, hostile
- **Speech-**
- Quantity: - poverty of speech, poverty of content, volume.
- Quality: - articulate, congruent, monotonous, talkative, repetitious, spontaneous, circumstantial, confabulation, tangential and pressured
- Rate:-slowed, rapid

- **Mood and affect**
- Mood (Intensity depth duration):- sad, fearful, depressed, angry, anxious, ambivalent, happy, ecstatic, grandiose.
- Affect (Intensity depth duration) :- appropriate, apathetic, constricted, blunted, flat, labile, euphoric,
- **Perception**
- Hallucination, illusions, depersonalization, derealization, distortions
- **Thoughts**
- Form and content-logical vs. illogical, loose associations, flight of ideas, autistic, blocking., broadcasting, neologisms, word salad, obsessions, ruminations, delusions, abstract vs. concrete

- **Sensorium and Cognition**
- Level of consciousness, orientation, attention span, , recent and remote memory, concentration, , ability to comprehend and process information, intelligence
- **Judgment**
- Ability to assess and evaluate situations makes rational decisions, understand consequence of behaviour, and take responsibly for actions
- **Insight**
- Ability to perceive and understand the cause and nature of own and other's situation
- **Reliability**
- Interviewer's impression that individual reported information accurately and completely

Cont…..

- **Psychosocial Criteria**
- Internal:-Psychiatric or medical illness, perceived loss such as loss of self concept, /self-esteem
- External:-Actual loss, e.g. death of loved ones, diverse, lack of support systems, job or financial loss, retirement of dysfunctional family system
- **Coping skills**
- Adaptation to internal and external stressors, use of functional, adaptive coping mechanisms, and techniques, management of activities of daily living

Cont…..

- **Relationships**
- Attainment and maintenance of satisfying, interpersonal relationships congruent with developmental stages, including sexual relationship as appropriate for age and status
- **Cultural**
- Ability to adapt and conform to present norms, rules, ethics.

- **Spiritual (Value-belief)**
- Presence of self-satisfying value-belief system that the individual regards as right, desirable, worthwhile, and comforting
- **Occupational**
- Engagement is useful, rewarding activity, congruent with developmental stages and societal standards (work, school and recreation)

Summary of MSE

- All you need to do is:

ACTMAD

- Appearance (Appearance, behaviour and attitude)
- Cognition (thought process and content, memory, judgment, concentration, insight, perceptions)
- Mood, Manner and Affect
- Activities (feeding, sleep, sex, physical exercise, speech)
- Disabilities/Drugs

- OR be

ASEPTIC

- Appearance and Behaviour
- Speech
- Emotion (Mood and Affect)
- Perception (Hallucination and Illusion)
- Thought content and process
- Insight and Judgment
- Cognition

Physical examination

- Importance?
- Do a general examination (Head to Toe exam)
  - Brainstorm
- Remember to take vital signs

summary

- **summary**
- Voluntary admission
- Involuntary admission
- Emergency admission
- Armed forces admission
- Foreigners admission
- History taking
- MSE

QUESTIONS

THANK YOU

CORE CONCEPTS ON THERAPEUTIC COMMUNICATION

THERAPEUTIC COMMUNICATION

- An interpersonal interaction between the nurse and client during which the nurse focuses on the client's specific needs to promote an effective exchange of information
- Skilled use of therapeutic communication techniques helps the nurse understand and empathize with the client's experience

GOALS OF THERAPEUTIC COMMUNICATION

- Establish a therapeutic nurse-client relationship

- Identify the most important client concern at the moment (the client-centered goal)

Ct…

- Assess the client’s perception of the problem as it unfolded.

-This includes detailed actions (behaviors and messages) of the people involved and the client’s thoughts and feelings about the situation, others, and self

- Facilitate the client’s expression of emotions

GOALS OF THERAPEUTIC COMMUNICATION

- Teach the client and family necessary self-care techniques
- Recognize the client's needs
- Implement interventions designed to address the client's needs

Ct…

- Guide the client toward identifying a plan of action to a satisfying and socially acceptable resolution.

THERAPEUTIC COMMUNICATION TECHNIQUES

THERAPEUTIC COMMUNICATION TECHNIQUES

- Accepting
- Broad Openings
- Consensual validation
- Encouraging Comparison
- Encouraging Description of Perceptions
- Encouraging Expression
- Exploring
- Focusing
- Formulating a Plan of Action
- General Leads
- Giving Information
- Giving Recognition
- Making Observations
- Offering self

THERAPEUTIC COMMUNICATION TECHNIQUES

- Placing Event in Time or Sequence
- Presenting Reality
- Reflecting
- Restating
- Seeking Information
- Silence
- Suggesting Collaboration
- Summarizing
- Translating into Feelings
- Verbalizing the Implied
- Voicing Doubt

THERAPEUTIC COMMUNICATION TECHNIQUES: ACCEPTING

- Definition
  - Indicating reception
- Examples
  - "Yes"
  - "I follow what you said"
  - Nodding

Accepting ct…

- Rationale
- An accepting response indicates the nurse has heard and followed the train of thought.
- It does not indicate agreement but is nonjudgmental.
- Facial expression, tone of voice, and so forth also must convey acceptance or the words will lose their meaning

THERAPEUTIC COMMUNICATION TECHNIQUES: BROAD OPENINGS

- Definition
  - Allowing the client to take the initiative in introducing the topic
- Examples
  - "Is there something you'd like to talk about?"
  - "Where would you like me to begin?"

Ct…

- Rationale
- Broad openings make explicit that the client has the lead in the interaction.
- For the client who is hesitant about talking, broad openings may stimulate him or her to take the initiative

THERAPEUTIC COMMUNICATION TECHNIQUES: CONSENSUAL VALIDATION

- Definition
  - Searching for mutual understanding, for accord in the meaning of the words.
- Examples
  - "Tell me whether my understanding of it agrees with yours."
  - "Are you using this word to convey that . ."

Ct…

- Rationale
- For verbal communication to be meaningful, it is essential that the words being used should have the same meaning for all participants.
- Sometimes words, phrases, or slang terms have different meanings and can be easily misunderstood.

THERAPEUTIC COMMUNICATION TECHNIQUES: ENCOURAGING COMPARISON

- Definition
  - Helping the client to understand by looking at similarities and differences.
- Examples
  - “Was it something like. . . ?”
  - “Have you had similar experiences?”

Ct…

- Rationale
- Comparing ideas, experiences, or relationships brings out many recurring themes.
- The client benefits from making these comparisons because he or she might recall past coping strategies that were effective or remember the he or she has survived a similar situation

THERAPEUTIC COMMUNICATION TECHNIQUES: ENCOURAGING DESCRIPTION OF PERCEPTIONS

- Definition
  - Asking client to verbalize what he or she perceives.
- Examples
  - "Tell me when you feel anxious"
  - "What is happening?"
  - "What does the voice seem to be saying?"

Ct…

- Rationale
- To understand the client, the nurse must see things from his or her perspective.
- Encouraging the client to describe ideas fully may relieve the tension the client is feeling, and he or she might be less likely to take action on ideas that are harmful or frightening.

THERAPEUTIC COMMUNICATION TECHNIQUES: ENCOURAGING EXPRESSION

- Definition
  - Asking client to appraise the quality of his or her experience.
- Examples
  - "What are your feelings in regard to. . ?"
  - "Does this contribute to your distress?"

Ct…..

- Rationale
- The nurse asks the client to consider people and events in light of his or her own values.
- Doing so encourages the client to make his or her own appraisal rather than accepting the opinion of others.

THERAPEUTIC COMMUNICATION TECHNIQUES: EXPLORING

- Definition
  - Delving further into a subject or idea.
- Examples
  - "Tell me more about that."
  - "Would you describe it more fully?"
  - "What kind of work?"

Ct….

- Rationale
- When clients deal with topics superficially, exploring can help them examine the issue more fully.
- Any problem or concern can be better understood if explored in depth.
- If the client expresses an unwillingness to explore a subject, however, the nurse must respect his or her wishes.

THERAPEUTIC COMMUNICATION TECHNIQUES: FOCUSING

- Definition
  - Concentrating on a single point.
- Examples
  - "This point seems looking at more closely."
  - "Of all the concerns you have mentioned, which is most troublesome?"

Ct…

- Rationale
- The nurse encourages the client to concentrate his or her energies on a single point, which may prevent a multitude of factors or problems from overwhelming the client.
- It is also a useful technique when a client jumps from one topic to another.

THERAPEUTIC COMMUNICATION TECHNIQUES: FORMULATING A PLAN OF ACTION

- Definition
  - Asking the client to consider kinds of behavior likely to be appropriate in future situations.
- Examples
  - "What could you do to let your anger out harmlessly?"
  - "Next time this comes up, what might you do to handle it?"

Ct…

- Rationale
- It may be helpful for the client to plan in advance what he or she might do in future similar situations.
- Making definite plans increases the likelihood that the client will cope more effectively in a similar situation

THERAPEUTIC COMMUNICATION TECHNIQUES: GENERAL LEADS

- Definition
  - Giving encouragement to continue.
- Examples
  - "Go on."
  - "And then?"
  - "Tell me about it."

- Rationale
- General leads indicate that the nurse is listening and following what the client is saying without taking away the initiative for the interaction.
- They also encourage the client to continue if he or she is hesitant or uncomfortable about the topic.

THERAPEUTIC COMMUNICATION TECHNIQUES: GIVING INFORMATION

- Definition
  - Making available the facts that the client needs.
- Examples
  - “My name is. . .”
  - “Visiting hours are. . .”
  - “My purpose in being here is. . .”

- Rationale
- Informing the client of facts increases his or her knowledge about a topic or lets the client know what to expect.
- The nurse is functioning as a resource person
- Giving information also builds trust with the client.

THERAPEUTIC COMMUNICATION TECHNIQUES: GIVING RECOGNITION

- Definition
  - Acknowledging, indicating awareness.
- Examples
  - "Good Morning Ms. A. . ."
  - "You've finished your list of things to do."
  - "I notice that you've combed your hair."

Ct….

- Rationale
- Greeting the client by name, indicating awareness of change, or noting efforts the client has made all show that the nurse recognizes the client as a person, as an individual.
- Such recognition does not carry the notion of value, that is, of being “good” or “bad”.

THERAPEUTIC COMMUNICATION TECHNIQUES: MAKING OBSERVATIONS

- Definition
  - Verbalizing what the nurse perceives.
- Examples
  - “You appear tense.”
  - “Are you uncomfortable when . . ?”
  - “I notice that you are biting your lip.”

Ct…

- Rationale
- Sometimes clients cannot verbalize or make themselves understood.
- Or the client may not be ready to talk.

THERAPEUTIC COMMUNICATION TECHNIQUES: OFFERING SELF

- Definition
  - Making oneself available.
- Examples
  - "I will sit with you awhile."
  - "I will stay here with you."
  - "I am interested in what you think."

Ct…

- Rationale
- The nurse can offer his or her presence, interest, and desire to understand.
- It is important that this offer is unconditional, that is, the client does not have to respond verbally to get the nurse’s attention.

THERAPEUTIC COMMUNICATION TECHNIQUES: PLACING EVENT IN TIME SEQUENCE

- Definition
  - Clarifying the relationship of events in time.
- Examples
  - “What seemed to lead up to. . ?”
  - “Was this before or after?”
  - “When did this happen?”

Ct…

- Rationale
- Putting events in proper sequence helps both the nurse and client to see them in perspective.
- The client may gain insight into cause-and-effect behavior and consequences, or perhaps some things are not related.
- The nurse may gain information about recurrent patterns or themes in the client’s behavior relationship.

THERAPEUTIC COMMUNICATION TECHNIQUES: PRESENTING REALITY

- Definition
  - Offering for consideration that which is real.
- Examples
  - "I see no one else in the room."
  - "That sound was a car backfiring."
  - "Your mother is not here. I am a nurse."

Ct…

- Rationale
- When it is obvious that a client is misinterpreting reality, the nurse can indicate what is real.
- The nurse does this by calmly and quietly expressing the nurse's perceptions of the facts not by way of arguing with the client or belittling his or her experience.
- The intent is to indicate an alternative line of thought for the client to consider, not to "convince" the client that he or she is wrong.

THERAPEUTIC COMMUNICATION TECHNIQUES: REFLECTING

- Definition
  - Directing client actions, thoughts, and feelings back to the client.
- Examples
  - Client: "Do you think I should tell the doctor?"
  - Nurse: "Do you think you should?"
  - Client: "My brother spends all my money and then has the nerve to ask for more."
  - Nurse: "This causes you to feel angry?"

Ct….

- Rationale
- Reflection encourages the client to recognize and accept his or her own feelings.
- The nurse indicates that the client’s point of view has value, and that the client has the right to have opinions, make decisions, and think independently

THERAPEUTIC COMMUNICATION TECHNIQUES: RESTATING

- Definition
  - Repeating the main idea expressed.
- Examples
  - Client: “I can’t sleep. I stay awake all night.”
  - Nurse: “You have difficulty sleeping.”
  - Client: “I am really mad. I am really upset.”
  - Nurse: “You’re really mad and upset.”

Ct…..

- Rationale
- The nurse repeats what the client has said in approximately or nearly the same words the client has used.
- This restatement lets the client know that he or she communicated the idea effectively.
- This encourages the client to continue
- Or if the client has been misunderstood, he or she can clarify his or her thoughts.

THERAPEUTIC COMMUNICATION TECHNIQUES: SEEKING INFORMATION

- Definition
  - Seeking to make clear that which is not meaningful or that which is vague.
- Examples
  - "I am not sure that I follow."
  - "Have I heard you correctly?"

Ct….

- Rationale
- The nurse should seek clarification throughout interactions with clients.
- Doing so can help the nurse to avoid making assumptions that understanding has occurred when it has not.
- It helps the client to articulate thoughts, feelings, and ideas more clearly.

THERAPEUTIC COMMUNICATION TECHNIQUES: SILENCE

- Definition
  - Absence of verbal communication, which provides time for the client to put thoughts or feelings into words, regain composure, or continue talking.
- Examples
  - Nurse says nothing but continues to maintain eye contact and conveys interest

Ct….

- Rationale
- Silence often encourages the client to verbalize provided that it is interested and expectant.
- Silence gives the client time to organize thoughts, direct the topic of interaction, or focus on issues that are most important.
- Much nonverbal behavior takes place during silence, and the nurse needs to be, aware of the client and his or her own nonverbal behavior.

THERAPEUTIC COMMUNICATION TECHNIQUES: SUGGESTING COLLABORATION

- Definition
  - Offering to share, to strive, to work with the client for his or her benefit.
- Examples
  - "Perhaps you and I can discuss and discover the triggers for your anxiety."
  - "Let's go to your room and I will help you find what you are looking for."

Ct…..

- Rationale
- The nurse seeks to offer a relationship in which the client can identify problems in living with others, grow emotionally, and improve the ability to form satisfactory relationships.
- The nurse offers to do things with, rather than for, the client

THERAPEUTIC COMMUNICATION TECHNIQUES: SUMMARIZING

- Definition
  - Organizing and summing up that which has gone before.
- Examples
  - “Have I got this straight?”
  - “You’ve said that. .”
  - “During the past hour, you and I have discussed..”

Ct….

- Rationale
- Summarization seeks to bring out the important points of the discussion and to increase the awareness and understanding of both participants.
- It omits the irrelevant and organizes the pertinent aspects of the interaction.
- It allows both client and nurse to depart with the same ideas and provides a sense of closure at the completion of each discussion.

THERAPEUTIC COMMUNICATION TECHNIQUES: TRANSLATING INTO FEELINGS

- Definition
  - Seeking to verbalize client's feelings that he or she expresses only indirectly.
- Examples
  - Client: "I am dead."
  - Nurse: "Are you suggesting that you feel lifeless?"
  - Client: "I am way out in the ocean."
  - Nurse: "You seem to feel lonely or deserted."

Ct….

- Rationale
- Often the client says, when taken literally, seems meaningless or far removed from reality.
- To understand, the nurse must concentrate on what the client might be feeling to express himself or herself this way.

THERAPEUTIC COMMUNICATION TECHNIQUES: VERBALIZING THE IMPLIED

- Definition
  - Voicing what the client has hinted at or suggested.
- Examples
  - Client: “I can’t talk to you or anyone. It is a waste of time.”
  - Nurse: “Do you feel that no one understands?”

Ct…

- Rationale
- Putting into words what the client has implied or said indirectly tends to make the discussion less obscure.
- The nurse should be as direct as possible without being unfeelingly blunt or obtuse.
- The client may have difficulty communicating directly
- The nurse should take care to express only what is fairly obvious; otherwise the nurse may be jumping to conclusions or interpreting the client’s communication

THERAPEUTIC COMMUNICATION TECHNIQUES: VOICING DOUBT

- Definition
  - Expressing uncertainty about the reality of the client's perceptions.
- Examples
  - "Isn't that unusual?"
  - "Really?"
  - "That is hard to believe."

THERAPEUTIC COMMUNICATION TECHNIQUES: VOICING DOUBT

- Rationale
  - Another means of responding to distortions of reality is to express doubt.
  - Such expression permits the client to become aware that others do not necessarily perceive events in the same way or draw the same conclusions.

Ct…

- This does not mean the client will alter his or her point of view, but at least the nurse will encourage the client to reconsider or reevaluate what has happened.
- The nurse neither agreed nor disagreed; however, he or she has not let the misperceptions and distortions pass without comment.

NON-THERAPEUTIC COMMUNICATION TECHNIQUES

NON-THERAPEUTIC COMMUNICATION TECHNIQUES

- These responses cut off communication and make it more difficult for the interaction to continue

- It takes practice for the nurse to avoid making these typical comments

NON-THERAPEUTIC COMMUNICATION TECHNIQUES

- Advising
- Agreeing
- Belittling Feelings Expressed
- Challenging
- Defending
- Disagreeing
- Disapproving
- Giving Approval
- Giving Literal Responses
- Indicting the Existence of an External Source
- Interpreting
- Introducing an Unrelated Topic
- Making Stereotyped Comments

NON-THERAPEUTIC COMMUNICATION TECHNIQUES

- Probing
- Reassuring
- Rejecting
- Requesting an Explanation
- Testing
- Using Denial

NON-THERAPEUTIC COMMUNICATION TECHNIQUES: ADVISING

- Definition
  - Telling the client what to do.
- Examples
  - "I think you should."
  - "Why don't you?"

ct,….

- Rationale
- Giving advice implies that only the nurse knows what is best for the client.

NON-THERAPEUTIC COMMUNICATION TECHNIQUES: AGREEING

- Definition
  - Indicating accord with the client.
- Examples
  - "That is right."
  - "I agree."

Ct….

- Rationale
- Approval indicates the client is “right” rather than “wrong.”
- This gives the client the impression that he or she is “right” because of agreement with the nurse.
- Opinions and conclusions should be exclusively the client’s
- When the nurse agrees with the client, there is no opportunity for the client to change his

NON-THERAPEUTIC COMMUNICATION TECHNIQUES:

BELITTLING FEELINGS EXPRESSED

- Definition
  - Misjudging the degree of the client's discomfort.
- Examples
  - Client: "I have nothing to live for. . . I wish I was dead"
  - Nurse: "Everybody gets down in the dumps." OR "I have felt that way myself."

Ct….

- Rationale
- When the nurse tries to equate the intense and overwhelming feelings the client has expressed to “everybody” or to the nurse’s own feelings, the nurse implies that the discomfort is temporary, mild, self-limiting, or not very important.
- The client is focused on this or her own worries and feelings; hearing the problems or feelings of others is not helpful.

NON-THERAPEUTIC COMMUNICATION TECHNIQUES: CHALLENGING

- Definition
  - Demanding proof from the client.
- Examples
  - "But how can you be the President of the United States?"
  - "If you are dead, why is your heart beating?"

Ct….

- Rationale
- Often the nurse believes that if he or she can challenge the client to prove unrealistic ideas, the client will realize there is no “proof” and then will recognize reality.
- Actually challenging causes the client to defend the delusions or misperceptions more strongly than before.

NON-THERAPEUTIC COMMUNICATION TECHNIQUES: DEFENDING

- Definition
  - Attempting to protect someone or something from verbal attack.
- Examples
  - "This hospital has a fine reputation."
  - "I am sure your doctor has your best interests in mind."

Ct….

- Rationale
- Defending what the client has criticized implies that he or she has no right to express impressions, opinions, or feelings.
- Testing the client that his or her criticism is unjust or unfounded does not change the client’s feelings but only serves to block further communication

NON-THERAPEUTIC COMMUNICATION TECHNIQUES: DISAGREEING

- Definition
  - Opposing the client's ideas.
- Examples
  - "That is wrong."
  - "I definitely disagree with. . ."
  - "I do not believe that. . ."

Ct…

- Rationale
- Disagreeing implies the client is “wrong”
- Consequently the client feels defensive about his or her point of view or ideas.

NON-THERAPEUTIC COMMUNICATION TECHNIQUES:
GIVING APPROVAL

- Definition
  - Sanctioning the client's behavior or ideas.
- Examples
  - "That is good."
  - "I am glad that. . "

Ct….

- Rationale
- Saying what the client thinks or feels if “good” implies that the opposite is “bad”.
- Approval then, tends to limit the client’s freedom to think, speak, or act in a certain way.
- This can lead to the client’s acting in a particular way just to please the nurse.

NON-THERAPEUTIC COMMUNICATION

TECHNIQUES:

GIVING LITERAL RESPONSES

- Definition
  - Responding to a figurative comment as though it were a statement of fact.
- Examples
  - Client: "They are looking in my head with a television camera."
  - Nurse: "Try not to watch television." OR "What channel?"

Ct…..

- Rationale
- Often the client is at a loss to describe his or her feelings, so such comments are the best he or she can do.
- Usually it is helpful for the nurse to focus on the client’s feelings in response to such statements.

NON-THERAPEUTIC COMMUNICATION TECHNIQUES:

INDICATING THE EXISTENCE OF AN EXTERNAL SOURCE

- Definition
  - Attributing the source of thoughts, feelings, and behavior to others or to outside influences.
- Examples
  - “What makes you say that?”
  - “What made you do that?”
  - “Who told you that you were a prophet?”

NON-THERAPEUTIC COMMUNICATION TECHNIQUES:

INDICATING THE EXISTENCE OF AN EXTERNAL SOURCE

- Rationale
  - The nurse can ask, "What happened?" or "What events led you to draw such a conclusion?"
  - But to question "What made you think that?" implies that the client was made or compelled to think in a certain way.
  - Usually the nurse does not intend to suggest that the source is external but that is often what the client thinks.

NON-THERAPEUTIC COMMUNICATION TECHNIQUES: INTERPRETING

- Definition
  - Asking to make conscious that which is unconscious; telling the client the meaning of his or her experience.
- Examples
  - "What you really mean is. . ."
  - "Unconsciously you are saying. . ."

Ct…

- Rationale
- The client’s thoughts and feelings are his or her own, not to be interpreted by the nurse or for hidden meaning.
- Only the client can identify or confirm the presence of feelings.

NON-THERAPEUTIC COMMUNICATION TECHNIQUES:
INTRODUCING AN UNRELATED TOPIC

- Definition
  - Changing the subject.
- Examples
  - Client: “I would like to die.”
  - Nurse “Did you have visitors last night?”

Ct….

- Rationale
- The nurse takes the initiative for the interaction away from the client.
- This usually happens because the nurse is uncomfortable, does not know how to respond, or has a topic he or she would rather discuss.

NON-THERAPEUTIC COMMUNICATION TECHNIQUES:
MAKING STEREOTYPED COMMENTS

- Definition
  - Offering meaningless cliches or trite comments.
- Examples
  - "It is for your own good."
  - "Just keep your chin up"
  - "Just have a positive attitude and you will be better in no time."

Ct….

- Rationale
- Social conversation contains many cliches and much meaningless chit-chat.
- Such comments are of no value in the nurse-client relationship.
- Any automatic responses will lack the nurse‘s consideration or thoughtfulness.

NON-THERAPEUTIC COMMUNICATION TECHNIQUES: PROBING

- Definition
  - Persistent questioning of the client.
- Examples
  - "Now tell me about this problem. You know I have to find out."
  - "Tell me your psychiatric history."

Ct….

- Rationale
- Probing tends to make the client feel used or invaded.
- Clients have the right not to talk about issues or concerns if they choose.
- Pushing and probing by the nurse will not encourage the client to talk.

NON-THERAPEUTIC COMMUNICATION TECHNIQUES: REASSURING

- Definition
  - Indicating there is no reason for anxiety or other feelings of discomfort.
- Examples:
  - "I would not worry about that."
  - "Everything would be alright."
  - "You are coming along just fine."

Ct….

- Rationale
- Attempts to dispel the client’s anxiety by implying that there is not sufficient reason for concern completely devalues the client’s feelings.
- Vague reassurances without accompanying facts are meaningless to the client.

NON-THERAPEUTIC COMMUNICATION TECHNIQUES: REJECTING

- Definition
  - Refusing to consider or showing contempt for the client's ideas or behaviors.
- Examples
  - "Let us not discuss. . ."
  - "I do not want to hear about. . ."

Ct….

- Rationale
- When the nurse rejects any topic, he or she closes it off from exploration.
- In turn, the client will feel personally rejected along with his or her ideas.

NON-THERAPEUTIC COMMUNICATION TECHNIQUES:

REQUESTING AN EXPLANATION

- Definition
  - Asking the client to provide reasons for thoughts, feelings, behaviors, events.
- Examples
  - "Why do you think that?"
  - "Why do you feel that way?"

Ct…

- Rationale
- There is a difference between asking the client to describe what is occurring or has taken place and asking him to explain why. Usually a “why” question is intimidating.
- In addition, the client is unlikely to know “why” and may become defensive trying to explain himself or herself.

NON-THERAPEUTIC COMMUNICATION TECHNIQUES: TESTING

- Definition
  - Appraising the client's degree of insight.
- Examples
  - "Do you know what kind of hospital this is?"
  - "Do you still have the idea that. . ?"

Ct…..

- Rationale
- These types of questions force the client to try to recognize his or her problems.
- The client’s acknowledgement that he or she does not know these things may meet the nurse’s needs but is not helpful for the client

NON-THERAPEUTIC COMMUNICATION TECHNIQUES: USING DENIAL

- Definition
  - Refusing to admit that a problem exists.
- Examples
  - Client: “I am nothing.”
  - Nurse: “Of course you are something. Everybody is something.”
  - Client: “I am dead.”
  - Nurse: “Do not be silly.”

Ct….

- Rationale
- The nurse denies the client’s feelings or the seriousness of the situation by dismissing his or her comments without attempting to discover the feelings or meaning behind them.

NON-VERBAL COMMUNICATION

- This is transmitted with or without verbal communication.
- It is essential that the nurse become aware of her own non-verbal communication in addition to becoming skillful in identifying the client's non-verbal communication.

Ct….

- Non-verbal communication provides clues about the validity of the spoken words and congruency with the client’s behavior.
- The phrase “Actions speak louder than words” is generally accurate.

NON-VERBAL COMMUNICATION

- A list of ways in which non-verbal communication is conveyed to others follows:
  - Tone of voice
  - Voice inflection
  - Facial Expression
  - Silence
  - Gestures
  - Mannerism
  - Posture

NON-VERBAL COMMUNICATION

- List of ways in which non-verbal communication is conveyed to others:
  - Eye contact
  - Rate of speech
  - A “hurry up” attitude
  - An “I couldn’t care less” attitude
  - Physical appearance
  - Touch
  - Space

WHAT TO REMEMBER IN THERAPEUTIC COMMUNICATION

- Be empathetic and not just sympathetic!

EMPATHY

- Is the ability of the nurse to perceive the meanings and feelings of the client and to communicate that understanding to the client.
- It is considered one of the essential skills a nurse must develop

Ct….

- Being able to put himself on the client’s shoes does not mean that the nurse has had the same exact experiences as the client
- Nevertheless, by listening and sensing the importance of the situation to the client, the nurse can imagine the client’s feelings about the experience

EMPATHY

- Both the client and the nurse give a "gift of self" when empathy occurs – the client by feeling safe enough to share feelings, and the nurse by listening closely enough to understand.
- Empathy has been shown to positively influence client outcomes

Ct…

- Clients tend to feel better about themselves when the nurse is empathetic

EXAMPLE OF EMPATHY

- Client: “I am so confused! My son just visited and wants to know where the safety deposit box key is.”
- Nurse: “You’re confused because your son asked for he safety deposit key.” (Using reflection)

Ct…

- Nurse: “Are you confused about the purpose of your son’s visit?” (Using clarification)
- Note that from these empathetic moments, a bond can be established to serve as the foundation for the nurse-client relationship

SYMPATHY

- Feelings of concern or compassion one shows for another.
- By expressing sympathy, the nurse may project his or her personal concerns onto the client, thus inhibiting the client's expression of feelings

EXAMPLE OF SYMPATHY

- Client: “I am so confused! My son just visited and wants to know where the safety deposit box key is.”
- Nurse: “I know how confusing sons can be. My son confuses me, too, and I know how bad that makes you feel.”

Ct….

- Note that the nurse’s feelings of sadness or even pity could influence the relationship and hinder the nurse’s abilities to focus on the client’s needs.
- Sympathy often shifts the emphasis on the nurse’s feelings, hindering the nurse’s ability to view the client’s needs more objectively.

NURSE – PATIENT RELATIONSHIP

NURSE-PATIENT RELATIONSHIP

- Series of interactions between the nurse and the patient in which the nurse assists the patient to attain positive behavioral change

CHARACTERISTICS OF THE NURSE-PATIENT RELATIONSHIP

- Goal-directed
- Focused on the needs of the patient
- Planned
- Time-limited
- Professional

BASIC ELEMENTS OF THE NURSE-PATIENT RELATIONSHIP

- **T**rust
- **R**apport
- **U**nconditional positive regard
- **S**etting limits
- **T**herapeutic communication

PHASES OF THE NURSE-PATIENT RELATIONSHIP

- Pre-orientation phase
- Orientation phase
- Working phase
- Termination phase

PRE-ORIENTATION PHASE

- Begins when the nurse is assigned to a patient
- Phase of Nurse-Patient Relationship in which the patient is excluded as an actual participant
- Nurse feels certain degree of anxiety

PRE-ORIENTATION PHASE

- Includes all of what the nurse thinks and does before interacting with the patient
- Tasks include data gathering, planning for the first interaction
- Major task is to develop self-awareness

ORIENTATION PHASE

- Begins when the nurse and the patient interacts for the first time
- Parameters of the relationship are to be laid
- Nurse begins to know about the patient

Ct…..

- Tasks include establishing rapport, developing trust, assessment (and formulation of a nursing diagnosis).

- Major task is to develop a mutually acceptable contract

WORKING PHASE

- It is highly individualized
- More structured than the orientation phase
- The longest and most productive phase of the nurse-patient relationship

Ct…

- Limit-setting is employed
- Tasks include planning and implementation
- Major task is identification and resolution of the patient’s problems

TERMINATION PHASE

- It is a gradual weaning process
- It is a mutual agreement
- It involves feelings of anxiety, fear and loss
- It should be recognized in the orientation phase

Ct….

- Tasks include evaluation
- Major task is to assist patient to review what has been learned and to transfer his learning to his relationship with others

TERMINATION PHASE

- How to terminate?
  - Gradually decrease interaction time
  - Focus on future oriented topics
  - Encourage expression of feelings
  - Make the necessary referral

PROBLEMS AFFECTING THE NURSE-PATIENT RELATIONSHIP - TRANSFERENCE

- Occurs when the client displaces onto the nurse attitudes and feelings that the client originally experience in other relationships

- These patterns are automatic and unconscious

PROBLEMS AFFECTING THE NURSE-PATIENT RELATIONSHIP - TRANSFERENCE

- Example:
  - An adolescent female client working with a nurse who is about the same age as the teen's parents might react to the nurse like she reacts to her parents. She might experience intense feelings of rebellion or make sarcastic remarks.

PROBLEMS AFFECTING THE NURSE-PATIENT RELATIONSHIP - COUNTERTRANFERENCE

- Occurs when the nurse displaces onto the client attitudes or feelings from his or her past.

- Example:
  - A female nurse who has teenage children and who is experiencing extreme frustration with an adolescent client may respond by adopting a parental or chastising tone.

- The nurse is countertransfering her own attitudes and feelings toward her children onto the client.

PROBLEMS AFFECTING THE NURSE-PATIENT RELATIONSHIP – RESISTANCE

- Development of ambivalent feelings toward self-exploration

PRINCIPLES OF CARE IN PSYCHIATRIC SETTINGS

PRINCIPLES OF CARE IN PSYCHIATRIC SETTINGS

- The nurse views the patient as a holistic human being with interdependent and interrelated needs.

PRINCIPLES OF CARE IN PSYCHIATRIC SETTINGS

- The nurse accepts the patient as a unique human being with inherent value and worth exactly as he is.

PRINCIPLES OF CARE IN PSYCHIATRIC SETTINGS

- The nurse should focus on the patient's strengths and assets and not on his weakness and liabilities

PRINCIPLES OF CARE IN PSYCHIATRIC SETTINGS

- The nurse views the patient's behavior non-judgmentally, while assisting the patient to learn more adaptive ways of coping

PRINCIPLES OF CARE IN PSYCHIATRIC SETTINGS

- The nurse should explore the patient' behavior for the need it is designed to meet and the message it is communicating.

PRINCIPLES OF CARE IN PSYCHIATRIC SETTINGS

- The nurse has the potential for establishing a nurse-patient relationship with most if not all patients.

PRINCIPLES OF CARE IN PSYCHIATRIC SETTINGS

- The quality of the nurse-patient relationship determines the degree of change that can occur in the patient’s behavior.

LEVELS OF INTERVENTIONS IN PSYCHIATRIC NURSING

PRIMARY LEVEL OF PREVENTION

- Interventions aimed at the promotion of mental health and lowering the rate of cases by altering the stressors.

- Examples:
  - Health education
  - Information dissemination
  - Counseling

SECONDARY LEVEL OF PREVENTION

- Interventions that limit the severity of a disorder.

- Has two components:
  - Case finding
  - Prompt treatment

- Examples:
  - Crisis intervention
  - Administration of medications

TERTIARY LEVEL OF PREVENTION

- Interventions aimed at reducing the disability after a disorder.

- Has two components:
  - Prevention of complication
  - Active program of rehabilitation

- Examples:
  - Alcoholics Anonymous
  - Occupational therapy

CHARACTERISTICS OF A PSYCHIATRIC NURSE

CHARACTERISTICS OF A PSYCHIATRIC NURSE

- Empathy
  - The ability to see beyond outward behavior and sense accurately another person's inner experiencing.
- Genuineness / Congruence
  - Ability to use therapeutic tools appropriately
- Unconditional Positive Regard
  - Respect

ROLES OF THE NURSE IN PSYCHIATRIC SETTINGS

ROLES OF THE NURSE IN PSYCHIATRIC SETTINGS

- Ward Manager
  - Creates a therapeutic environment.
- Socializing Agent
  - Assists the patient to feel comfortable with others
- Counselor
  - Listens to the patient’s verbalizations

ROLES OF THE NURSE IN PSYCHIATRIC SETTINGS

- Parent Surrogate
  - Assists the patient in the performance of activities of daily living.
- Patient Advocate
  - Enables the patient and his relatives to know their rights and responsibilities
- Teacher
  - Assists the patient to learn more adaptive ways of coping

ROLES OF THE NURSE IN PSYCHIATRIC SETTINGS

- Technician
  - Facilitates the performance of nursing procedures
- Therapist
  - Explores the patient's needs, problems and concerns through varied therapeutic means
- Reality Base
  - Enables the patient to distinguish objective reality and subjective reality

ROLES OF THE NURSE IN PSYCHIATRIC SETTINGS

- Healthy Role Model
  - Acts as a symbol of health by serving as an example of healthful living.

Self assessment

- What are the qualities of a psychiatric nurse?
- State four roles of a psychiatric nurse

TREATMENT MODALITIES IN PSYCHIATRY

MANAGEMENT AND CARE OF COMMON PSYCHIATRIC CONDITIONS

Objectives

- At the end of this section you will be able to describe treatments under the following headings:
- Drugs and other physical treatments
- Psychological treatments

Drugs and Other Physical Treatments

- There are various types of drugs and other

physical treatments used to treat patients

suffering from mental health illness.These can be grouped together under the following categories:

- • Antipsychotic Medication
- • Antidepressant
- • Anxiolytics or Anti-anxiety Drugs
- • Antiparkinsonian Drugs
- • Electroconvulsive Therapy (ECT)

Antipsychotic Medication

- Antipsychotic drugs are also called major tranquillisers or neuroleptics used in the treatment of psychoses like schizophrenia, bipolar disorders (manic phase) and alcohol withdrawal disorder.

Examples of antipsychotics

- Chlorpromazine
- Haloperidol
- Flupenthixol
- Fluphenazine

Generic Name	Trade Name	Daily Doses (range)
Low Potency Drugs		
Chlorpromazine	Largactil	300-1000mgs
Sulpride	Domatil,Sulparex	200-2400mg
Thioridazine	Melleril	50-800mg
High Potency Drugs		
Haloperidol	Haldol,Serenace	1-20mg
Thiothixene	Navane	6-60mgs
Zoxapine	Loxitane	60-250mgs
Molindone	Lidone	50-400mgs
Flupenthixol	Depixol	6-18mg
Fluphenazine	Moditen	2.5-20mg
Trifluoperazine	Stelazine	5-30mg
Zuclopenthixol	Clopixol	20-150mg
Pimozide	Orap	2-10mg
Depot Injections		
Fluphenazine decanoate	Modecate	12.5-100mg(IM 2 Weekly)
Zuclopenthixol Acetate	Clopixol Acuphase	50-150mg every 2-3 days
Haloperidol decanoate	Haldol decanoate	50-300mg (IM 4 weekly)

Quiz

- Differentiate typical and atypical antipsychotics.

Mechanisms of Action

- The drugs are thought to work by blocking dopamine receptors causing a decrease in psychotic symptoms. The drug is metabolised in the liver and excreted by the kidneys.

For one to get the desired effects, one must maintain the patient on the lowest dose possible and initial therapy should be on divided doses so that the patient can be monitored.

- Gertrude and MacFarland (1986) have identified the following expected responses to the treatment:
- Initially the patient is drowsy and cooperative within hours to a week.
- The patient becomes more sociable and less withdrawn for the next two months.
-

- The thought disorder generally disappears in six weeks or more.
- Improvement is generally noted in

hallucinations, acute delusions, sleeping habits, appetite, tension,combativeness, hostility, negativism and personal grooming.

Side Effects

- There are several side effects that may be experienced by patients. These include drowsiness and orthostatic hypotension, especially after im injections.

The patient mayalso experience extra pyramidal symptoms like:

- Dystonia, that is, spasms of muscles of face, neck, back, eye, arms and legs.
- Oculogyric crisis, presenting as fixed upward gaze from spasm of oculomotor muscles.

- Torticollis, that is, pulling of the head to the side from spasm of cervical muscles.
- Opisthotonus, which refers to the hyperextension of the back from spasm of back muscles.
- Akathisia or continuous motorrestlessness.
- Akinesia or lack of body movement especially arms.
- Pseudoparkinsonism, which presents with a shuffling gait, mask-like facial expression, tremor, rigidity and akinesia.

- Tardive dyskinesia, that is, a wormlike movement of the tongue, frequent blinking, and involuntary movement of tongue, lips and jaw.
- They may experience convulsive seizures or allergic or toxic effects (some of which are rare and serious).
- These include:
- • Aggranulosis
- • Oral monoliasis
- • Dermatitis
- • Jaundice

- endocrine or metabolic effects like weight gain or decreased libido, impotence, impaired ejaculation in males.
- Decreased thermoregulatory ability and as a result might complain of being too cold or too hot.

Mgt of side effects

- Adjust dose of antipsychotics
- Give antiparkinsonian drugs like artane.

- **Contraindications**

Comatose, glaucoma, acute myocardial infarction

Antidepressants

- These drugs are used to treat affective disorders.

Mechanisms of Action

- They act by increasing epinephrine and serotonin. Both of them are metabolised in the liver and excreted in theurine.

Examples

- Tricyclic- amitriptyline, imipramine
- Tetracyclic- maprotiline
- Mono aine oxidase inhibitors- isocarboxacid, phenelzine
- SSRI- fluoxetine, paroxetine, citalopram

Major Groups	Generic Name	Trade Name	Daily Dosage (range)
Tricyclic antidepressants	Amitriptyline	Elavil (laroxyl)	75-300mg
	Imipramine	Tofranil	100-300mg
Tetra cyclic anti-depressants	Maprotiline	Ludiomil	75-300mg
Monoamine Oxidase Inhibitors	Isocarboxacid	Marplan	10-60mg
	Phenelzine	Nardil	45-90mg
Selective Serotonin Reuptake Inhibitors	Fluoxetine	Prozac	20mg
	Citalopram	Cipramil	20-60mg
	Paroxetine	Seroxat	10-50mg

- Dosage may be given in divided doses, but total dose can be given at bed time due to sedative effects.
- Medication should be continued for 6 mths after pt is free from depression.

S.E

- Drowsiness in early stages,which later subsides.
- Hypotension may be experienced initially,leadingto feeling of fainting.
- Withdrawal symptoms may be

experienced if the drug is not gradually stopped.

- mild anticholinergic effects

from tricyclic and monoamine oxidase inhibitors, dry mouth, constipation, blurred vision, Tachycardia, nausea, oedema, hypotension and urinary retention.

- Side effects that are specific to tricyclics are:
- Allergic reactions manifested as skin rash and jaundice.
- Tachycardia.
- Tremors.
- Long term treatment may depress bone marrow, predispose to sore throat and aching, and fever.

- specific side effects of monoamineoxidase inhibitors include:
- Liver damage that is rare but fatal.
- Precipitation of manic episodes.
- Hypertension crisis characterised by severe headache palpitation, neck stiffness, nausea, vomiting, increased Bp, chest pain and collapse. It occurs 30 minutes to 24 hours after eating food containing tyramine. These foods include cheese, wine, beer, sour cream, liver, chocolate, bananas, avocadoes, soy sauce, and beans.

Mgt of S. E

- The main form of treatment when side effects occur is to discontinue the drug and then give regitine to lower the blood pressure.

Contraindications

- The use of antidepressants is contraindicated when the patient suffers from glaucoma, agitated states, urinary retention, cardiac disorders and seizure disorders.

Anxiolytics or Anti-anxiety Drugs

- These drugs, when given to a patient having generalised anxiety disorder, are able to provide relief.
- They are mainly recommended for acute anxietystates, which may present with palpitations,sweating, trembling, shortness of breath, chestpain, nausea, dizziness, a feeling of unreality,fear of losing control or dying, chills ornumbness.

- Examples of these drugs include buspirone, a novel anxiolytic, and benzodiazepines like diazepam and lorazepam.

Effects

- The main effects include sedation, muscle relaxation and elevation of seizure threshold.

Side Effects

- Side effects include dizziness, headache, nervousness, insomnia, light headedness, dry mouth, nausea, vomiting, abdominal and gastric distress and diarrhoea.
- When high doses of medication are used for more than four months, the patient is likely to develop drug dependence or withdrawal syndrome.

- **Contraindications**
- Benzodiazepines should not be used together with other central nervous system depressants. They should be given with caution to patients who are elderly, depressed or suicidal and those with a history of substance abuse.

Antiparkinsonian Drugs

- These are drugs given to counteract the side effects of major tranquillisers.
- An example of such a drug is benztropine (cogentin) whose initial dose is 0.1-1mg daily, the maintenance dose is 0.5-6mg daily divided into two or four times.
- **Side Effects**
- Side effects include dry mouth, nausea,
- constipation, urinary retention, blurred vision,
- disorientation and confusion.

MOOD STABILIZING DRUGS

MOOD STABILIZING DRUGS

- These are used for the following:
  - To treat bipolar affective disorder by stabilizing the client's mood
  - To avoid or minimize the highs and lows that characterize bipolar illness
  - To treat the acute phases of mania

MOOD STABILIZING DRUGS

- Lithium is the most established mood stabilizer

- Some anticonvulsant drugs are effective mood stabilizers such as:
  - Carbamazepine (Tegretol)
  - Valproic Acid (Depakene, Depakote)

MOOD STABILIZING DRUGS

- Other anticonvulsants, such as gabapentin (Neurontin) and lamotrigine (Lamictal), are being used on a trial basis for mood stabilization

- Occasionally, clonazepam (Klonopin), an anti-anxiety agent, is also used to treat acute mania.

MECHANISM OF ACTION OF MOOD STABILIZING DRUGS

- Lithium normalizes the reuptake of certain neurotransmitters, such as:
  - Serotonin
  - Norepinephrine
  - Acetylcholine
  - Dopamine

- Lithium also reduces the release of norepinephrine through competition with calcium

- Lithium produces its effects intracellularly rather than within neuronal synapses.

MECHANISM OF ACTION OF MOOD STABILIZING DRUGS

- Valproic Acid is known to increase levels of the inhibitory neurotransmitter GABA.

- Both anticonvulsants, Valproic Acid and Carbamazepine, are thought to stabilize mood by inhibiting the kindling process – the snowball-like effect seen when minor seizure activity seems to build up into more frequent and severe seizures.

MECHANISM OF ACTION OF MOOD STABILIZING DRUGS

- In seizure management, anticonvulsants raise the level of the threshold to prevent these minor seizures.

- It is suspected that this same kindling process may also occur in the development of full-blown mania, with stimulation by more frequent minor episodes.

- This explain why anticonvulsants are effective in the treatment and prevention of mania as well.

DOSAGE OF MOOD STABILIZING DRUGS - LITHIUM

- Lithium is available in tablets, capsules, liquid and a sustained release form but NO PARENTERAL FORMS ARE AVAILABLE.

- Daily dosages generally range from 900 mg to 3,600 mg.

- More importantly, the serum Lithium level should be about 1.0 mEq/L

DOSAGE OF MOOD STABILIZING DRUGS - LITHIUM

- Serum Lithium levels of less than 0.5 mEq/L are rarely therapeutic, and levels of more than 1.5 mEq/L are usually considered toxic.

- The Lithium level should be monitored every 2 to 3 days while the therapeutic dosage is being determined, then weekly.

- When the client's condition is stable, the level may need to be checked once a month or less frequently.

DOSAGE OF MOOD STABILIZING DRUGS - ANTICONVULSANTS

- Carbamazepine is available in liquid, tablet, and chewable forms. Dosages usually range from 800 to 1,200 mg / day and the extreme dosage is 200 to 2,000 mg / day.

- Valproic acid is available in liquid, tablet, and capsule forms and as sprinkles, with dosages raging from 1,000 to 1,500 mg / day and the extreme dosage is 750 to 3,000 mg / day

- Serum drug levels, obtained 12 hours after the last dose of the medication, are monitored for therapeutic levels of both anticonvulsants.

SIDE EFFECTS OF MOOD STABILIZING DRUGS - LITHIUM

- Common side effects of Lithium therapy include:
  - Mild nausea (take medication with food) or diarrhea
  - Anorexia
  - Fine hand tremors (use propranolol – a beta blocker)
  - Polydipsia
  - Polyuria
  - Metallic taste in the mouth
  - Fatigue or lethargy

SIDE EFFECTS OF MOOD STABILIZING DRUGS - LITHIUM

- Weight gain and acne are side effects that occur later in lithium therapy and both are distressing for clients.
  - These are difficult to manage or minimize and frequently lead to noncompliance

TOXIC EFFECTS OF MOOD STABILIZING DRUGS - LITHIUM

- These include:
  - Severe diarrhea
  - Vomiting
  - Drowsiness
  - Muscle weakness
  - Lack of coordination

- Untreated, these symptoms worsen and can lead to renal failure, coma and death.

DRUG ALERT!!! - LITHIUM

- When toxic signs occur, the drug should be discontinued immediately.

- If Lithium levels exceed 3.0 mEq/day, dialysis may be indicated.

SIDE EFFECTS OF MOOD STABILIZING DRUGS - ANTICONVULSANTS

- Side effects of carbamazepine and valproic acid include:
  - Drowsiness
  - Sedation
  - Dry mouth
  - Blurred vision

- Carbamazepine may also cause rashes and othostatic hypotension.

- Valproic Acid may cause weight gain, alopecia and hand tremors.

CLIENT TEACHING REGARDING MEDICATION MANAGEMENT: MOOD STABILIZING DRUGS

- Have serum levels monitored periodically to ensure therapeutic levels of the medication.
- Take the medication with food to minimize nausea.
- For the fine hand tremors, ask the physician to prescribe a beta-blocker such as propranolol (Inderal).

CLIENT TEACHING REGARDING MEDICATION MANAGEMENT: MOOD STABILIZING DRUGS

- To help minimize weight gain, get a balanced diet and get regular exercise. Expect some weight gain.

- Minimize side effects of sedation and drowsiness from anticonvulsant medications by taking larger doses at bedtime and smaller doses during the day.

CLIENT TEACHING REGARDING MEDICATION MANAGEMENT: MOOD STABILIZING DRUGS

- Use calorie-free beverages and sugar-free candy to relieve dry mouth. Avoid calorie-laden beverages, because they do not relieve dry mouth and stimulate more weight gain.

- If you are taking lithium, keep water intake in a normal range and avoid heavy sweating, because this decreases serum lithium levels rapidly.

STIMULANTS

STIMULANT DRUGS

- Stimulant drugs, specifically amphetamines, were first used in the treatment of psychiatric disorders in the 1930s for their pronounced effects of CNS stimulation.

- Today, the primary use is for attention deficit / hyperactivity disorder (ADHD) in children and adolescents, residual attention deficit disorder in adults, and narcolepsy (attacks of unwanted but irresistible daytime sleepiness that disrupt a person's life).

STIMULANT DRUGS

- The primary drugs used to treat ADHD are the CNS stimulants:
  - methylphenidate (Ritalin)
  - pemoline (Cylert)
  - dextroamphetamine (Dexedrine)

MECHANISM OF ACTION OF STIMULANT DRUGS

- Amphetamines and methylphenidate are often termed indirectly acting amines because they act by causing release of the neurotransmitters (norepinephrine, dopamine, and serotonin) from presynaptic nerve terminals, as opposed to having direct agonist effects on the postsynaptic receptors.

- They also block the reuptake of these neurotransmitters.

DOSAGES OF STIMULANT DRUGS

- For the treatment of narcolepsy in adults, both dextroamphetamine (Dexedrine) and methylphenydate (Ritalin) are given in divided doses totaling 20 – 200 mg/day.

- The higher doses may be needed because adults with narcolepsy develop tolerance to the stimulants, requiring more medication to sustain improvement.

DOSAGES OF STIMULANT DRUGS

- The dosages used to treat ADHD in children vary widely depending on:
  - the physician
  - the age, weight and behavior of the child
  - the tolerance of the family for the child's behavior

DOSAGES OF STIMULANT DRUGS

GENERIC (TRADE) NAME	DOSAGE
Methylphenidate (Ritalin)	Adults: 20-200 mg/day, orally, in divided doses Children: 10-60 mg/day orally, in 2-4 divided doses
Dextroamphetamine (Dexedrine)	Adults: 20-200 mg/day, orally, in divided doses Children: 5-40 mg/day orally, in 2-3 divided doses
Pemoline (Cylert)	Children: 37.5-112.5 mg/day orally, given once a day in the morning

SIDE EFFECTS OF STIMULANT DRUGS

- The most common side effects of stimulants are:
  - Anorexia
  - Weight loss
  - Nausea
  - Irritability

- Caffeine, sugar, and chocolate should be avoided because they may worsen these symptoms.

SIDE EFFECTS OF STIMULANT DRUGS

- Less common side effects include:
  - Dizziness
  - Dry mouth
  - Blurred vision
  - Palpitations

SIDE EFFECTS OF STIMULANT DRUGS

- The most common long-term problem with stimulants is the growth and weight suppression that occurs in some children.

- This can usually be prevented by taking "drug holidays" on weekends, holidays, or during summer vacation, which helps to restore normal eating and growth patterns

CLIENT AND FAMILY TEACHING FOR MEDICATION MANAGEMENT: STIMULANT DRUGS

- Never leave the supply of medication in a place the child can reach to avoid overdose or taking additional medication

- Take the medication at meal time to minimize nausea and anorexia.

CLIENT AND FAMILY TEACHING FOR MEDICATION MANAGEMENT: STIMULANT DRUGS

- Monitor the child's weight and height because growth suppression can be a long-term consequence of stimulant therapy.
  - Not giving the drugs during weekends during the summer can help resume normal growth patterns.

- Try a dosage schedule that provides a dose of medication before beginning routine tasks of concentration such as nightly homework.

CLIENT AND FAMILY TEACHING FOR MEDICATION MANAGEMENT: STIMULANT DRUGS

- Avoid beverages containing caffeine. Limit intake of chocolate, sugar, or any other substance that increases the child's activity level.

- Alleviate dry mouth with calorie-free beverages or sugar-free candy.

- Consult often with the school nurse or other person responsible for giving medications at school.
  - Medications should be given in a manner that is not intrusive, nor should it draw undue attention to the child.

SENSITIZING DRUGS

DISULFIRAM (ANTABUSE)

- Disulfiram is a sensitizing agent that causes an adverse reaction when mixed with alcohol in the body.

- This agent's only use is as a deterrent to drinking alcohol in persons receiving treatment for alcoholism.

ADVERSE REACTION WHEN DISULFIRAM (ANTABUSE) MIXES WITH ALCOHOL

- Five to ten minutes after someone who is taking disulfiram ingests alcohol, symptoms begin to appear:
  - Facial and body flushing from vasodilation
  - A throbbing headache
  - Sweating
  - Dry mouth
  - Nausea and vomiting
  - Dizziness and weakness
- In severe cases, severe hypotension, confusion and even death.
- Symptoms progress rapidly and last from 30 minutes to 2 hours.

MECHANISM OF ACTION OF DISULFIRAM (ANTABUSE)

- Disulfiram inhibits the enzyme aldehyde dehydrogenase, which is involved in the metabolism of ethanol.
- Acetaldehyde levels are then increased from 5 to 10 times higher than normal, resulting in the disulfiram-alcohol reaction

SIDE EFFECTS OF DISULFIRAM (ANTABUSE)

- Side effects of taking Disulfiram include:
  - Fatigue
  - Drowsiness
  - Halitosis
  - Tremor
  - Impotence

- It can interfere with the metabolism of other drugs the client is taking such as: phenytoin (Dilantin), isoniazid (INH), warfarin (Coumadin), barbiturates, and long-acting benzodiazepines such as diazepam and chlordiazepoxide.

CLIENT EDUCATION FOR PATIENTS TAKING DISULFIRAM (ANTABUSE)

- Many common products contain alcohol, such as:
  - Shaving cream
  - Aftershave lotion
  - Cologne
  - Deodorant
  - Over-the-counter medications such as cough preparations
- When used by the client taking disulfiram, these products can produce the same reaction as drinking alcohol
- The client must read product labels carefully and select items that are alcohol-free

Electroconvulsive therapy

- Def : this is the induction of a grand mal seizure/generalized seizure through the application of electrical current to the brain.
- The stimulus is applied through electrodes that are placed either bilaterally in the frontotemporal region or unilaterally on the same side as the dominant hand.

- Typical range of application might be 70-125 volts of electrical current for 0.7-1.5 seconds.the duration of the resulting seizure is 25-90 seconds.
- Most patients require an average of 6-10 treatments. They can be done upto 3 times per week.
- Maximum of 8-12 treatments.

Mechanism of action

- Electrical stimulation results in significant increases in the circulating levels of neurotransmitters. i.e dopamine, norepinephrine , serotonin.
- ECT may also result in increase in glutamate and gamma-amino butyric acid.

Indications

- Suicidal patients
- Agitated
- Delusional Catatonic Acute mania or manic episodes of affective disorder
- Major depression
- Schizophrenia
- Other conditions-Obsessive compulsive disorders, personality disorders, neurosis

Contra-indications

- The only absolute contraindication for ECT is raised intracranial pressure from the brain tumor, recent cardiovascular accident or other cerebral vascular lesions.
- NB- ECT is associated with a physiological rise in cerebral spinal fluid pressure during the treatment, resulting in ICP that could lead to brain stem herniation.

Others but not absolute contra-indications

- Myocardial infarction
- Cerebral vascular accident
- Aortic or cerebral aneurysm
- Severe underlying hypertension
- Congestive heart failure
- Severe osteoporosis
- Acute and chronic pulmonary disorders
- High risk or complicated pregnancy

Pre -ECT

- Investigations are done i.e. chest/spine X-ray, lab investigations (complete blood count, urinalysis), Electrocardiogram (ECG),Electro encephalogram (EEG).
- Informed consent is obtained from the relatives.
- Patient is straved for 6 hours.
- Removal of all metallic objects like rings, bracelets.

- Nurse assesses the patients mood and level of interaction with others, evidence of suicidal ideations/plan, level of anxiety/fear, thought & communication patterns, baseline short term and long-term memory, patient and family knowledge of indications for , side effects and potential risks of ECT, current and past medications, baseline vital signs and history of allergies, patients ability to carry out activities of daily living.

Procedure

- 30 minute before the procedure administer pre-medication i.e atropine 0.6 mg IM to dry the secretions and counteract the effects of vagal stimulation induced by ECT.
- Nurse alleys anxiety and any fears.
- Patient is placed in supine position.
- Anaesthesiologist administers sodium thiopental (short acting anaesthetic).

- Administration of muscle relaxant (succynylcholine) IV to prevent severe muscle contractions during the seizures, thereby reducing the possibility of fracture and dislocated bones.
- Succinylcholine paralyzes respiratory muscles, hence the patient is given oxygen prior to and during the treatment .
- Airway is placed or tongue depressor in the patients mouth and proper positioning to allow for airway patency.
- Electrodes are placed bilaterally/unilateraaly on the temporal region to deliver electrical stimulation.

Role of the nurse during the procedure

- Ensure the airway is patent and provide sunctioning if needed.
- Assist the anaesthesiologist in oxygenation.
- Observe readouts on machines monitoring vital signs and cardiac functioning.
- Provide support to patients arms and legs during the seizure.
- Observe and record type & amount of movement produced during the seizure.

- NB
- Oxygen is continued until spontaneous respirations return.
- Most pt awaken after 10-15 minutes and are confused and disoriented.
- Some patients will sleep for 1-2 hours following the treatment.

Post ECT

- **Role of the nurse**
- Monitor vital signs every 15 minutes for the first 1 hour.
- Position the client in the side lying position with head on the side to prevent aspiration.
- Orient the client to place and time.
- Describe what has occurred.

- Provide assurance that memory loss they may be experiencing is temporary.
- Allow the pt to verbalize fears and anxiety related to receiving ECT.
- Stay with the patient until he/she is fully awake, oriented and able to perform self care activities without assistance.
- Provide pt with a highly structure schedule of routine activities to minimize confusion.

Side effects of ECT

- Most common s/E of ECT are :-Temporary memory loss and confusion.
- Other side effects include:- anxiety and apprehension, burns from poor contact with the electrodes, loose and broken teeth.

Risks associated with ECT

- Mortality due to cardiovascular complications e.g acute MI
- Permanent memory loss
- Brain damage

Psychological treatments

- Psychotherapy- individual ,group therapy, family therapy, couple therapy
- Behavioral therapy- cognitive behavioral therapy
- Activity therapy- occupational therapy ,recreation therapy, dance therapy.
- Rehabilitation

- Milieu therapy
- Aversion therapy
- Flooding therapy
- Systematic desensitization

Self assessment

- Describe the role of the nurse during an ECT session.
- What are the extrapyramidal side effects of antipsychotic drugs?

Rehabilitation

Psychiatric rehabilitation

- Rehabilitation is the process of enabling the individual to return to his highest possible level of functioning. It is an important component of the community mental health program, and is undertaken at the level of tertiary prevention.

Indications

- The following disorders are indicated commonly for rehabilitation:
- Chronic schizophrenia
- Chronic organic mental disorders
- Mental retardation
- Alcohol and drug dependence

Goals of psychosocial rehabilitation

- No matter what form psychosocial services take, core goals include helping people feel:
- **Hopeful:** People may be left feeling demoralized as a result of their condition. Rehabilitation focuses on helping clients feel hopeful about the future

- **Empowered:** Each individual needs to feel that they are able to set their own goals and have the power and autonomy to pursue those aims.

- **Skilled:** Rehabilitation helps teach people skills to help them manage their condition and live the life they want to live. This includes living skills, work skills, social skills, and others.

- **Supported:** Mental health professionals should offer support and help clients build relationships and social connections in their community.

Principles of psychosocial rehabilitation

- All people have potential that can be developed.
- People have a right to self-determination.
- The emphasis is on the individual's strengths rather than their symptoms.

Ct..

- Each person's needs are different.
- Professional services should be committed and take place in as normalized an environment as possible.

Ct…

- There is a focus on a social model of care (as opposed to a medical model).
- It is centered on the present rather than fixated on the past.

Who Can Benefit From Psychosocial Rehabilitation?

- People who need help restoring their full functioning after treatment
- Those who are disabled and need ongoing assistance in multiple life domains

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- Individuals who, while functional, feel that they need a boost of support and assistance
- People who lack the supportive environment and resources they need to achieve full functioning

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- Individuals with chronic and severe psychiatric conditions can benefit from PSR services.
- Rehabilitation can help these individuals learn basic skills that will allow them to function and cope with their condition.

- People with intellectual and cognitive disabilities can benefit from learning life, social, and self-care skills.

Psychiatric Rehabilitation Approaches

- **a. Psychoeducation:** Includes diagnosing the problem, telling the person what to expect regarding illness and discussing treatment alternatives.

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- **b. Working with families:** Encouraging family members to get involved in treatment and rehabilitation programs.

- **c. Group therapy:** Positive aspects of group therapy include an opportunity for ongoing contact with others, validation of their perceptions, sharing their views about problems and problem solving abilities.

Group therapy session

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- **d. Social skills training :**It involves teaching specific living skills that the patient is expected to have in order to survive in the community

Rehabilitation Team

- Professionals contributing to psychiatric rehabilitation include, psychiatrist, clinical psychologist, psychiatric social worker, psychiatric nurse, occupational therapist, recreational therapist, counselor and other mental health paraprofessionals.

Steps in Psychiatric Rehabilitation

- Psychiatric rehabilitation begins with a comprehensive medical psychiatric diagnosis and functional assessment. These are key elements in identifying impairments and disabilities.

Reduction of impairments

- Rehabilitation interventions with psychiatric patients require reduction or elimination of the symptoms and cognitive impairments that interfere with social and vocational performance. These impairments are reduced and eliminated for the greater part by various psychotropic agents.

Remediation of disabilities through skill training

- Skill training is used to remediate disabilities in social, family and vocational functioning.
- Patients generally require training in self-care skills, interpersonal skills, vocational and employment pursuits, recreational and leisure skills.

Remediating disabilities through supportive interventions

- When restoration of social and vocational functioning through skills training is limited by continuing deficits, rehabilitation strategies aim at helping the individuals compensate for handicap by learning skills in living and working environments, adjusting the individual and family expectations to a level of functioning that is realistically attainable.

Remediation of handicaps

- In addition to clinical rehabilitation interventions, the disabled persons can be helped to overcome their handicaps through social rehabilitation interventions, e.g. community support programs.

Types of Rehabilitation

- Social Rehabilitation
- Vocational Rehabilitation
- Educational Rehabilitation
- Occupational Rehabilitation

Social Rehabilitation

- The client has to feel a sense of responsibility and adapt healthy roles and modifies the deviant behavior with normal behavior. The preventive and curative measures will be adopted by the individual client to lead satisfactory and useful life in social environment. Opportunities have to be provided to the client to maintain and reestablish social adequacy

Vocational Rehabilitation

- It is a process which enables persons with functional, psychological, developmental cognitive, and emotional disabilities or impairments or health disabilities to overcome barriers to accessing, maintaining, or returning to employment or other useful occupation.

Educational Rehabilitation

- Educating or giving training for a person with a psychiatric disorder in specific areas is essential whereby the client meets the goals of treatment and rehabilitation. For example, explaining the family about client's disease condition and its prognosis, treatment alternatives; fears and myths associated with it; giving proper explanation and reality goals

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- Remedial education related to vocational service in supportive environment is provided. Encourage the clients who have the capabilities to undergo higher education and attainment of vocational skills.

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- Measures are taken to enhance the person's acceptance of his illness; encourage the client and family members to actively participate in care related activities and adopt coping strategies to overcome the problems occurring related to disease process and to live more productively within the community.

Occupational Rehabilitation

- It will be carried out to achieve job oriented skills and pleasure seeking activities, whereby the client can obtain independent way of living and will try to overcome the problems related to illness.

Role of a Nurse in Psychiatric Rehabilitation

- **1. Assessment of the Individual:**
- The nurse should assess the individual in the areas of symptoms present, motivation, strengths, interpersonal skills, self-esteem, activities of daily living and drug compliance.

- **2. Assessment of Family:**
- Components of family assessment: Family structure including developmental stages, roles, responsibilities, norms and values.
- Family attitudes towards the mentally ill member.

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- Emotional climate of the family.
- Social support available to the family.
- Past family experiences with mental health services.
- The family's understanding of the patient's problems and the plan of care.

- **3. Assessment of Community:**
- It includes assessment of community agencies that provide services to people who have mental illnesses assessment of attitudes of the people towards the mentally ill, etc.

- **Planning and Implementation**
- Planning and implementation in rehabilitative psychiatric nursing focuses on fostering independence by maximizing personal strengths. The nurse and the patient must work together to find ways for the patient to overcome any remaining impaired areas of functioning.

Individual Interventions

- **Hospital rehabilitation (Inpatient rehabilitation):** This involves therapeutic community, recreational therapy, social skills training and training in basic living skills.

- **Community rehabilitation:** Providing care in community settings (Homes, residential care settingsfoster homes etc.)

Family Interventions

Health education to family members regarding the disease process, available resources, communication skills and problem solving techniques. Motivating the family members to provide proper care to the patient.

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- Group therapy and support to family members through self-help groups; nurses are in a favorable position to help families cope with stress and adapt to changes in the family structure.

Community Interventions

- There are several ways that nurses can intervene in the community tertiary prevention programs. Among these are health education to the public, training to school teachers, village leaders and paraprofessionals in the rehabilitation of mentally ill people.

Evaluation

- Evaluation of psychiatric rehabilitation services usually takes place at the level of impact on the patient, family and the effectiveness of the community service system.

Barrier in Rehabilitation

- To find wholesome environment, adequate housing for the client to live comfortably.
- To identify job resources and to find suitable job.

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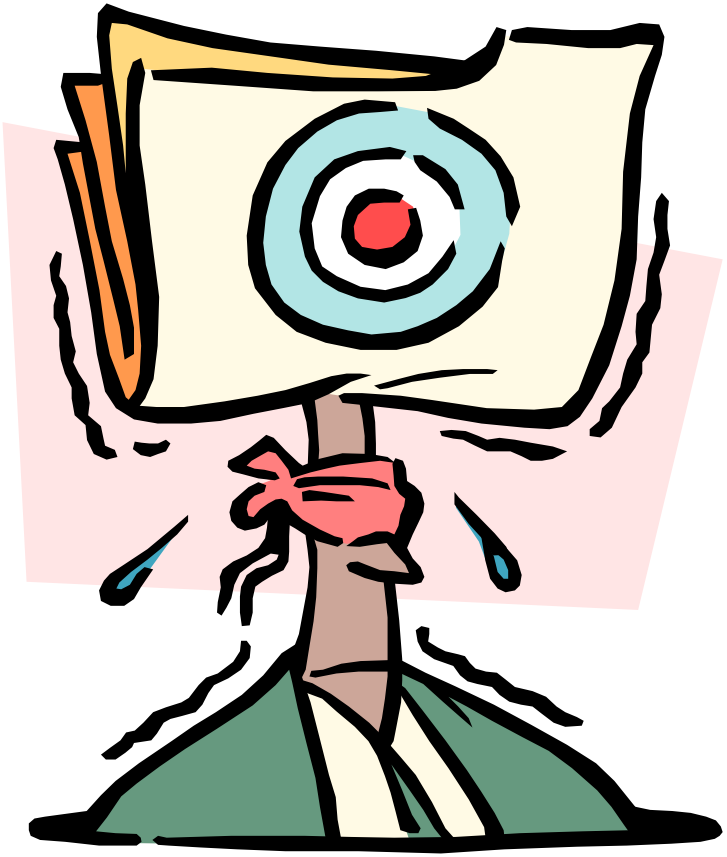
- Unable to participate actively, occupationally and socially in the life.
- The attitude of family members will be affecting the client’s performance related to instrumental and expressive functions of family.

Question time

???

Thank you



CLASSIFICATION OF MENTAL DISORDERS

- **Classification of Mental Disorders**

- There are two major classifications of mental disorders used internationally.
- These are:
. International Classification of Diseases (ICD)
. Diagnostic and Statistical Manual (DSM)
- The ICD is the WHO system of classification, currently in its 10th edition, commonly referred to as ICD 10.
- The DSM classification is the American Psychiatric Association system, currently in the 4th edition and commonly referred to as DSM IV

ICD10 is classified in the following manner.

- F0 Organic, including symptomatic mental disorders.
- F1 Mental and behaviour disorders due to psychoactive substance use.
- F2 Schizophrenia, schizotypal and delusional disorders.
- F3 Mood (affective) disorders.
- F4 Neurotic, stress related and somatoform disorders.
- F5 Behavioral syndromes associated with physiological disturbances and physical factors.
- F6 Disorders of adult personality and behaviour.
- F7 Mental retardation.
- F8 Disorders of psychological development.
- F9 Behavioral and emotional disorders with onset usually occurring in childhood or adolescence.

DSM- IV

- DSM-IV-TR (text revision, 2000) consists of five axis / domains on which disorders are assessed.
- Axis 1-clinical disorders (all mental disorders except personality disorders and mental retardation)
- Axis II- Personality disorders & MR
- Axis III- General medical conditions (must be connected to mental disorders)

- Axis IV- Psychosocial and environmental problems(limited social support network).
- Axis V- Global assessment of functioning.

DSM IV mental disorders classification

1. Disorders usually first diagnosed in infancy, childhood or adolescence.
2. Delirium, Dementia, Amnestic and other cognitive disorders.
3. Mental disorders due to general medical condition.
4. Substance related disorders
5. Schizophrenia and other psychotic disorders
6. Mood disorders
7. Anxiety disorders
8. Somatoform disorders
9. Personality disorders

9. Factitious disorders
10. Dissociative disorders
11. Sexual & gender identity disorders
12. Eating disorders
13. Sleep disorders
14. Impulse and control disorders not otherwise classified.
15. Adjustment disorders

DSM-5 CLASSIFICATION

- This classification was developed in 2013.
- It has discarded the multi-axial system of diagnosis e.g Axis 1,Axis II etc..

Neurodevelopmental disorders

- Intellectual disabilities- Intellectual disability developmental disorder, Global developmental delay
- Communication disorders- language disorder, speech sound disorder, childhood onset fluency disorder (stuttering), social (pragmatic) communication disorder

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- Autistic spectrum disorder
- Attention deficit/hyperactivity disorder
- Specific learning disorder
- Motor disorder- developmental coordination disorder, stereotypic movement disorder, Tic disorders, persistent (chronic) motor or vocal disorder, provisional tic disorder
- Other neurodevelopmental disorders

Schizophrenia spectrum and other psychotic disorders

- Schizotypal (personality) disorder
- Delusional disorder
- Brief psychotic disorder
- Schizophreniform
- Schizophrenia

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- Schizo affective disorder
- Substance/medication induced psychotic disorder
- Psychotic disorder due to another medical condition
- Catatonia

Bipolar and related disorders

- Bipolar 1 disorder
- Bipolar 11 disorder
- Cyclothymic disorder
- Substance/ medication induced bipolar and related disorder
- Bipolar and related disorder due to another medical condition

Depressive disorders

- Disruptive mood dysregulation disorder
- Major depressive disorder
- Persistent depressive disorder (dysthymia)
- Premenstrual dysphoric disorder
- Substance/ medication induced depressive disorder
- Depressive disorder due to another medical condition

Anxiety disorders

- Separation anxiety disorder
- Selective mutism
- Specific phobia
- Social anxiety disorder
- Panic disorder
- Panic attack
- Agoraphobia

- Generalized anxiety disorder
- Substance/medication induced anxiety disorder
- Anxiety disorder due to another medical condition
- Other specified anxiety disorder
- Unspecified anxiety disorder

Obsessive compulsive and related disorders

- Obsessive compulsive disorder
- Body dysmorphic disorder
- Hoarding disorder
- Trichotillomania/ hair pulling disorder
- Excoriation/skin picking disorder
- Substance/medication induced obsessive compulsive related disorder

- Obsessive compulsive and related disorder due to another medical condition
- Other specified obsessive-compulsive and related disorder
- Unspecified obsessive compulsive and related disorder

Trauma and stressor related disorders

- Reactive attachment disorder
- Disinhibited social engagement disorder
- Post traumatic stress disorder
- Acute stress disorder
- Adjustment disorders
- Other specified trauma and stressor related disorders
- Unspecified trauma and stressor related disorder

Dissociative disorders

- Dissociative identity disorder
- Dissociative amnesia
- Depersonalization/derealization disorder
- Other specified dissociative disorder
- Unspecified dissociative disorder

Somatic symptom and related disorders

- Somatic symptom disorder
- Illness anxiety disorder
- Conversion disorder/ functional neurological symptom disorder
- Psychological factors affecting other medical conditions
- Factitious disorder

Feeding and eating disorders

- Pica
- Rumination disorder
- Avoidant/restrictive food intake disorder
- Anorexia nervosa
- Bulimia nervosa
- Binge eating disorder

Elimination disorders

- Enuresis
- Encoporesis
- Other specified elimination disorders
- Unspecified elimination disorder

Sleep wake disorders

- Insomnia disorder
- Hypersomnolence disorder
- Narcolepsy

- **Breathing related sleep disorders;**
- Obstructive sleep apnea hypopnes
- Central sleep apnea
- Sleep related hypoventilation
- Circadian rhythm sleep-wake disorders

- **Parasomnias ;**
- Non- rapid eye movement sleep arousal disorders- sleep walking, sleep terrors
- Nightmare disorder
- Rapid eye movement sleep behavior disorder
- Restless legs syndrome
- Substance/medication induced sleep disorder
- Other specified hypersomnolence disorder

Sexual dysfunctions

- Delayed ejaculation
- Erectile disorder
- Female orgasmic disorder
- Female sexual interest/arousal disorder
- Genito-pelvic pain/penetration disorder
- Male hypoactive sexual desire disorder
- Premature ejaculation
- Substance /medication induced sexual dysfunction

Gender dysphoria

- Gender dysphoria
- Other specified gender dysphoria
- Unspecified gender dysphoria

Disruptive, impulse-control and conduct disorders

- Oppositional defiant disorder
- Intermittent explosive disorder
- Conduct disorder
- Antisocial personality disorder
- Pyromania
- Kleptomania

Substance related and addictive disorders

- **Substance related disorders**
- Substance use disorders
- Substance induced disorders- substance intoxication and withdrawal

- **Alcohol related disorders**
- Alcohol use disorder
- Alcohol intoxication
- Alcohol withdrawal

- **Caffeine related disorders;**
- Caffeine intoxication
- Caffeine withdrawal
- Other caffeine induced disorders

- **Cannabis related disorders;**
- Cannabis use disorder
- Cannabis intoxication
- Cannabis withdrawal
- Other cannabis induced disorders

- Hallucinogen related disorders
- Inhalant related disorders
- Opiod related disorders
- Sedative, hypnotic or anxiety related disorders
- Stimulant related disorders
- Tobbaco related disorders
- Non substance related disorders- gambling disorder

Neurocognitive disorders

- **Delirium**
- **Major and mild neurocognitive disorders**
  (Refer to the DSM 5 for further classification)

Personality disorders

- Cluster A personality disorders- paranoid, schizoid and schizotypal
- Cluster B personality disorders- antisocial, borderline, histrionic and narcissistic
- Cluster C personality disorders- avoidant, dependent, obsessive compulsive personality disorders

Paraphilic disorders

- Voyeuristic disorder
- Exhibitionistic disorder
- Frotteuristic disorder
- Sexual masochism disorder
- Sexual sadism disorder
- Pedophilic disorder
- Fetishistic disorder
- Transvetic disorder

Other mental disorders

- Other specified mental disorder due to another medical condition
- Unspecified mental disorder due to another medical condition
- Other specified mental disorder
- Unspecified mental disorder

Self assessment

- What is the difference between DSM 1V classification and DSM 5 classification of mental disorders?

SCHIZOPHRENIA SPECTRUM & OTHER PSYCHOTIC CONDITIONS

- **Schizophrenia**

- Schizophrenia is a group of mental disorders that presents with varied symptoms of disordered thinking and bizarre social behavior. It affects approximately 1% of the total population, including adolescents and young adults. There is increased incidence in the lower social class. Patients with this condition occupy 50% of beds in mental hospitals.

Eugen Bleur's fundamental symptoms

- Ambivalence- marked inability to decide for or against/coexistence of two opposing feelings like love & hate.
- Autism-withdrawal into self
- Affect disturbances- like inappropriate affect
- Association disturbances like loosening associations and thought disorders.

Schineider’s first rank symptoms

- Hallucinations- voices speaking out.
- Thought alleniation phenomenon like thought withdrawal, thought insertion, thought diffussion or broadcast.
- Passivity phenomena-made feelings, made impulses, delusional perceptions.

Positive symptoms of schizophrenia

- Are those symptoms that most individuals do not normally experience but are present in people with schizophrenia.Respond well to medications.
- Delusions
- Hallucinations
- Thought disorders & speech

Negative symptoms

- Are deficits of normal emotional responses or of other thought processes. Respond less well to medication.Include;
- Flat or blunted affect & emotion.
- Poverty of speech/alogia.
- Anhedonia/ inability to experience pleasure.
- Asociality/lack of desire to form relationships.
- Avolition/lack of motivation.
- These contribute to poor quality of life, functional disability and burden on others than positive symptoms.

Risk factors

- Family hx of schizophrenia.

  Children who are ;
- Highly individualistic in their thought processes.
- Overly independent and obedient.
- Shy, withdrawn and loners.
- Unmanagable, prone to destructive aggressive behaviour.
- Sensitive to separation.

Causes

- Genetic
- Environment-drug use, hostile environ, unemployment.
- Substance abuse-cocaine, cannabis.
- Factors like infections, stress can lead to schizophrenia later in life.

- PHASES

- In the acute phase, the patient will experience:
. Auditory and visual hallucinations.
. Illusions.
. Delusions of persecution, grandeur or impending distraction.
. Autistic thinking characterised by being highly personal.
. Speech characterised by incoherence, echolalia, neologisms and unwillingness to speak.
. Inappropriate behaviour, for example, grimacing, negativism, suggestibility and poor personal hygiene.
. Blunting ambivalence and inappropriateness of affect.

- The residual phase. At this stage, the patient usually exhibits symptoms of flat or blunt affect, rambling speech, poor hygiene and grooming and distortion of some perceptual experiences may persist.

- The chronic/relapse phase, the patient may point to feelings of boredom and apathy, impulsive suppression of feelings and psychotic disorganisation with increasing perceptual and cognitive dysfunction, loss of identity and loss of self-control.

- NB: Currently in DSM 5 schizophrenia does not have types, however DSM IV had 5 types of schizophrenia.

Types

- **Catatonic Schizophrenia**
- This is a disorder characterised by a stuporous state in which the person is mute, negative or complains in response to a request.
- The individual may be immobile, display waxy flexibility and may retain urine and faeces.
- It may alternatively be characterised by a highly excited state in which the person is abusive, aggressive, hyperactive or agitated (catatonic excitement).

- **Disorganized /hebephrenic Schizophrenia**
- This is a disorder characterized by incoherence, foolishness and regressive behavior.
- Disorganized behavior.
- Inappropriate affect-full of joy while expressing bad news.
- Poor grooming & very unkempt
- Incontinent of both urine & feaces
- Delusions & hallucinations are present.
- Decorate themselves in extra ordinary ways.

- **Paranoid Schizophrenia**
- This is a disorder characterised by delusions of persecution or grandeur.
- Pt is tense, suspicious, guarded, reserved and aggressive.
- Auditory hallucinations.

Residual schizophrenia

- Xterized by ;
- Poverty of speech
- Neglect themselves..
- Decline in interpersonal relationships.
- Residual odd beliefs & hallucinations.

- **Undifferentiated Schizophrenia**
- This disorder is characterised by a variety of symptoms found in several of the types of schizophrenia

- In the current DSM 5 schizopherenia does not have the different types.

mgt

- Short hospitalisation to assist with the problem of being a danger to themselves or others, of deviant behaviour that is abrasive to the family and community and monitoring effects of drugs or other therapies.
- Outpatient treatment to provide aftercare, maintenance therapy, social support programs and medical clinics.
- Rehabilitation services to provide opportunities to increase skills in living such as vocational rehabilitation

- Pharmaco-therapy
- Administer antipsychotics to assist in alleviating symptoms .
- Fluphenazine decanoate 25-50mg IM every 2-3 weeks.
- Haloperidol 100-250 mg IM every 4 weeks

- Individual psychotherapy where the therapist has one on one counselling of the client.
- Group psychotherapy to assist patient in daily problems by exploring relationships, sources of anxiety and coping techniques. In group therapy support is offered on how to cope with problems encountered in everyday life.

- Family therapy to assist in defining the problem as one concerning the entire family rather than concerning the patient alone, to improve communication, to clarify roles within the family, to manage stress and to assist in problem solving.

Health education

- Teach patient and family various ways of obtaining help in improving work, educational and social skills.
- Stress the need for re-hospitalisation if the need arises.
- Teach the patient how to take medication and how to tolerate and/or adapt to side effects of the drugs.

- Patient and relatives should be taught how to identify stress early and then use problem solving skills.
- Family members should be taught how to supervise the patient's medication and how to respond to disturbing behaviour.
- Provide information on the nature of schizophrenia and the treatments available.
- Stress the importance of not using illicit drugs and alcohol.

- Milieu therapy which involves restructuring of the clients environment . This is because most schizophrenic patients tend to have suicidal ideations. Try and remove any harmful objects in the clients environment.

- Cognitive behavioral therapy

Nursing diagnosis

- Aggression.
- Impaired communication.
- Suspiciousness.
- Sensory/perceptual alteration: visual, auditory.
- Self-care deficit: feeding/hygiene, dressing/grooming, toileting.
- Ineffective coping.
- Suicidal tendencies.
- Alteration of thought process.
- Disturbance in self-concept, that is, disturbance in self-esteem, role performance and personal identity.

Psychosis

- The word "psychosis" is used to describe conditions that affect the mind, in which there has been some loss of contact with reality.
- The terms "early psychosis" or "first episode psychosis" mean that an individual is experiencing psychosis for the first time.

- Hallucinations, delusions (false beliefs), paranoia and disorganized thoughts and speech are symptoms of psychosis.
- These symptoms can seem so real that often the person does not realize that they are experiencing psychosis. Psychosis also affects feelings and behaviour.

Phases of a psychotic episode

- There are three phases to psychosis
- ***Prodromal Phase.***
- This is the period before the psychosis becomes more obvious. There are often changes in feelings, thoughts, perceptions and behaviours.
- Prodromal symptoms vary from person to person and some people may not experience a prodrome. The duration of this phase is quite variable, although it usually spans several months.

- ***Acute Phase.***
- This is the stage when the typical psychotic symptoms emerge.
- It is also the stage that is easiest to recognize and diagnose. Therefore the acute phase is when most people begin receiving treatment.

- ***Recovery.***
- Some of the symptoms that are apparent in the Acute Phase may linger in Recovery.
- With appropriate treatments, the great majority of people successfully recover from their first episode of psychosis.

- ***Signs & symptoms***

Early signs(prodromal phase)

- social withdrawal
- reduced concentration, attention
- depressed mood
- sleep disturbance
- anxiety
- suspiciousness
- skipping school or work
- irritability

Definite Psychotic Symptoms

- It is in the Acute Phase that typical psychotic symptoms emerge. These are the symptoms that are hard to miss.
- These symptoms are frequently separated into "positive" and "negative" categories.

- *Positive symptoms* are referred to as "positive" because they are viewed as an excess or distortion of the person's normal functioning.

- Some of the positive symptoms include:

- *Delusions (Fixed false beliefs)*

  Such as:
- being followed or monitored

- being plotted against

- having special abilities or "powers"
- certain songs or comments are specifically directed toward oneself or communicating a hidden message
- being controlled by forces or other individuals
- having one's thoughts broadcast so others can hear them

- *Hallucinations*
  Hallucinations involve seeing, hearing, feeling, smelling or tasting something that is not actually there.

- *Disorganized Speech or Behaviour* The speech of individuals with psychosis may be disorganized in such a way that the person moves quickly from one topic to the next, or to the point where the person's speech may not be understandable.

- The behaviour of individuals with psychosis also may be disorganized. The person may have difficulties performing activities of daily living (e.g., cooking, self-care) or display inappropriate behaviours or responses (e.g., laughing while describing a personal tragedy).

- *Negative symptoms* reflect a decrease in, or loss of, normal functions.
- These symptoms are often less evident than positive symptoms and require careful assessment.

Some examples of negative symptoms include:

- little display of emotions
- not speaking very much
- difficulties in thinking or coming up with ideas
- decreased ability to initiate tasks
- lowered levels of motivation or drive

- It is also common for other symptoms or problems to occur along with the psychotic symptoms.

  Some examples of other problems include:
- depression
- anxiety
- suicidal thoughts or behaviours
- substance abuse
- difficulties functioning
- sleep disturbance

- *Recovery Stage*
- In the recovery stage, the acute symptoms mentioned above will lessen and start to fade; however, some symptoms may linger.
- Even after the psychosis has responded to treatment, problems such as depression, anxiety, decreased self esteem, social problems and difficulties with work or school may occur.

causes psychosis

- Psychosis occurs in a variety of mental and physical disorders; therefore, it likely has multiple causes. Biology, stress and drug use are three of the most common theories.

- *Biology*
- *Neurotransmitters.* There is strong evidence that some psychoses involve a dysfunction in neurotransmitters in the brain.-dopamine

- *Genetics.* Individuals whose close relatives experience psychosis are themselves at increased risk.

- *Brain Changes.*
- Changes have been found in the brains of some individuals with schizophrenia, which appear to have been present since birth or early childhood.
- Possible causes of the changes include: genetic transmission, abnormal neurodevelopment and pregnancy or birth complications (e.g. exposure of mother to a virus during the second trimester of pregnancy).

- *Stress*
- Some psychoses appear to occur primarily in response to stress. Stresses can be such things as significant life events (e.g. death of a loved one, moving to a new city, etc.)

- *Drugs*
- Psychosis can be induced by drugs or can be drug assisted. For example, it appears that amphetamines can cause a psychotic episode, while other drugs, including marijuana, can increase a person's natural vulnerability to psychosis resulting in a psychotic episode.

Other causes

- Neurological causes E.g.CVA, brain tumors, brain abscess.
- Infections-Acute meningitis, malaria, HIV.
- Metabolic causes- Hypoglycaemia.
- Psychiatric causes- schizophrenia, depression, manic episode.

Types of psychosis

- Brief reactive psychosis- short lived psychosis triggered by stress.
- Puerperal psychosis
- Substance induced psychosis

Assignment – Read on the other types of psychosis.

Nursing diagnosis

- Altered thought processes related to delusional thinking.
- Altered sensory perception related to panic anxiety.
- Social isolation related to inability to trust.
- Self care deficit rlt to difficulties in performing ADLs.
- Altered sleep patterns rlt to hallucinations.
- Ineffective coping patterns.
- Impaired verbal communication.

investigations

- Hb
- BS for mps
- Culture & sensitivity
- Blood sugars
- Serum urea & electrolyte levels
- CSF examination
- X-ray skull
- Head CT scan
- MRI head

Management

- Identify and manage the cause.
- Do various investigations-HB, blood sugars, BS for MPs, UECs ,Head CT & MRI.
- 1 st line treatment is antipsychotics-IM/oral Chlorpromazine 100-200mg IM stat then 12 hourly OR Haloperidol 5-10mg 12 hrly .
- Monitor Bp before each dose & withhold if systolic BP is 90 or below..

- Sleep state should be monitored to ensure 6-7 hrs of sleep.
- Incase of extra pyramidal side effects like dystonia occurs one hour-48 hrs after starting treatment, give antiparkinsonian drugs-artane(0.5-6 mg daily in 2 to 4 divided doses)
- Close monitoring of pt to avoid danger to self or others.
- Cognitive behavioral therapy.

- Family therapy.
- ECT/Shock treatment to relieve the underlying symptoms of psychosis due to depression.
- Individual therapy suitable for patients with prominent delusions.
- Monitor for side effects of drugs.
- Providing hygiene care.
- Promoting home and community based care.

DELUSIONAL DISORDER

DELUSIONS

- Disturbances in the content rather than the form of thought
- Fixed false beliefs about one's environment or event occurring in it

Types of delusions

- Persecutory or Paranoid Delusions
- Grandiose Delusions
- Religious Delusions
- Somatic Delusions
- Referential Delusions or Ideas of Reference

PERSECUTORY / PARANOID DELUSIONS

- Involves the client's belief that "others" are planning to harm the client or are spying, following, ridiculing, or belittling the client in some way.
- Sometimes the client cannot define who these "others" are.

- Examples:
  - The client may think that food has been poisoned or that rooms are bugged with listening devices
  - Sometimes the "persecutor" is the government or other powerful organization
  - Occasionally, specific individuals, even family members may be named as the "persecutor"

GRANDIOSE DELUSIONS

- Are characterized by the client's claim to association with famous people or celebrities, or the client's belief that he or she is famous or capable of great feats

- Examples:
  - The client may claim to be engaged to a famous movie star or related to some public figure such as claiming to be the daughter of the President of the Philippines
  - May claim he or she has found a cure for cancer

RELIGIOUS DELUSIONS

- Often center around the second coming of Christ or another significant religious figure or prophet
- These religious delusions appear suddenly as part of the client's psychosis and are not part of his or her religious faith or that of others.

- Examples:
  - Client claims to be the Messiah or some prophet sent from God
  - Believes that God communicated directly to him or her
  - He or she has a “special” religious mission in life or special religious powers

SOMATIC DELUSIONS

- Are generally vague and unrealistic beliefs about the client's health or bodily functions
- Factual information or diagnostic testing does not change these beliefs

- Examples
  - A male client may say that he is pregnant
  - A client may report decaying intestines or worms in the brain

REFERENTIAL DELUSIONS / IDEAS OF REFERENCE

- Involve the client's belief that television broadcasts, music, or newspaper articles have special meaning for him or her

- Examples:
  - The client may report that the president was speaking directly to him on a news broadcast or that special messages are sent through newspaper articles

CATATONIA

Definition

- Catatonia is a neuropsychiatric condition that affects both behavior and motor function, and results in unresponsiveness in someone who otherwise appears to be awake.

- Although often associated with schizophrenia and other affective disorders, catatonia may be a result of, or due to, any number of psychotic disorders, mood disorders or general medical conditions.

- Catatonia is sometimes referred to as catatonic syndrome, because there is not just one identifying sign or symptom associated with this condition or symptoms that appear separately from one another, but rather a collection of several symptoms that appear together at the same time.

Symptoms

- According to the DSM-5, at least three out of twelve symptoms must be present for a diagnosis of catatonia. These symptoms include:
- Stupor (oblivious inability to move or respond to stimuli), catalepsy (rigid body posture)
- Mutism (little to no verbal communication)

- Waxy flexibility (body remains in whatever position it is placed by another)
- Negativism (lack of verbal response)
- Posturing (holding a posture or position that goes against gravity)
- Mannerisms (extreme or odd movements and mannerisms)

- Stereotypy (frequent repetitive movements for no reason)
- Agitation (for no reason), grimacing (distorted facial expressions)
- Echolalia (repeating others’ words)
- Echopraxia (repeating others’ movements)

- Other common symptoms include rigidity and automatic obedience.
- When catatonia is associated with schizophrenia, stupor may continue for long periods.

Causes

- Brain disorders such as neurodegenerative disease and encephalitis.
- Severe vitamin B12 deficiency
- Infection
- Exposure to toxins,

- Conditions such as schizophrenia and other psychotic disorders, autism, extreme trauma, and mood disorders are associated with catatonia.

Management

- Treatment depends on the underlying cause of the disorder.

Drugs

- Benzodiazepines (tranquilizers)
- Antidepressants
- Muscle relaxers
- Antipsychotic medications.
- Electroconvulsive therapy
- NMDA antagonists (anesthetic pain medications) are used in some cases.

Self assessment

- State five characteristics of catatonia.
- List four positive and four negative symptoms of schizophrenia.
- What are the treatment modalities used in management of a psychotic patient?

AFFECTIVE DISORDERS

AFFECTIVE DISORDERS/MOOD DISORDERS

- A group of mental disorders that present mainly with symptoms of mood disturbance with associated changes in thinking and behaviour.
- Classification of mood disorders.
- Manic episode
- Depressive episode
- Bipolar mood disorder

MANIA

- A state of abnormally elevated or irritable mood, arousal or energy levels.
- Its usually the opposite of depression.
- A manic episode is not a disorder in and of itself, but a type of bipolar disorder.
- A manic episode is characterized by period of time where an elevated, expansive or notably irritable mood is present, lasting for atleast 1 week.

- The feelings must be sufficiently severe to cause difficulty or impairment in occupational, social, educational or other important functioning.

Stages of mania

- Stage 1- Euphoria. Mild elevation of mood. Usually seen in hypomania.
- Stage 2- Elation. Moderate elevation of mood .Feeling of confidence and enjoyment.
- Stage 3- Exaltation. Severe elevation of mood. Delusions of grandeur.
- Stage 4- Ecstasy. Very severe elevation of mood. Seen in delirious & stupors mania.

- 3 or more of the following symptoms must be present.

I. Inflated self esteem/ grandiosity.
II. Decreased need for sleep.
III. More talkative than usual/ pressure of speech.
IV. Flight of ideas
V. Attention is easily drawn to irrelevant items.
VI. Increase in goal directed activity(either socially, at work, school or sexually)
VII. Excessive involvement in pleasurable activities that have a high potential for painful consequences.

Other symptoms

- Appearance- often reflects the prevailing mood. Brightly colored clothes.
- Increased appetite. Food may be eaten greedily.
- Irritability when stopped from doing what they want.
- Psychomotor activity- overactive. Often start activities but leave them unfinished & start new ones.

- Speech- rapid & copious.
- Grandiose delusions.
- Hallucination can occur in severe cases.

MGT

- Anti-psychotics which helps to treat hallucinations & delusions.Haloperidol 1.5-4mg upto three times a day.
- Olanzepine 15mg OD.
- Mood stabilizers –Lithium to control acute attacks and stabilize mood.Dose 0.4-1.2g daily as a single dose or in two divided doses.Anticonvulsants can also be used like Na-valproate in pts unresponsive to lithium.750mg daily in 2-3 divided doses.
- Tegretol 200mg BD.

- Benzodiazepines- diazepam is helpful in the initial stages of RX until lithium achieves full effect.Shouldnt be used for a long time to avoid dependence.
- Psychotherapy-Self help coping strategies & healthy lifestyle choices.
- ECT- severe mania when pt does not tolerate or respond to lithium or other drugs.

- Nurse should monitor the pts vital signs & mental status when attempting to control manic symptoms with neuroleptics.

DEPRESSION

- Definition - Is a medical illness that causes persistent feelings of sadness & loss of interest.

Types/levels of depression

- Are 4 levels of depression:-
- Transient depression
- Mild
- Moderate
- Severe/major depression
- Transient depression-xterized by sadness, downhearted, crying, tiredness.

- Mild depression: Triggered by an external event and the experience follows normal grief reaction. Lasts less than 2 weeks.
- Moderate depression: Persists over time. The person experiences a sense of change and often seeks help.

- Major/severe depression- symptoms include;- sadness, apathy, feelings of worthlessness, self blame, thoughts of suicide, weight loss, lessened interest in sex, sleeplessness, reduction in activity.

CAUSES OF DEPRESSION

- Loss: e.g. relationship, loved one, major body part or function.
- Major life events related to disruption of life patterns/ conflict.
- Role changes e.g. marriage, divorce, parenting or employment status.
- Disruption of coping skills/stress.
- Physical changes due to medical illness or debilitating condition or drug induced.
- Disturbance in neurotransmitter system.
- Genetic factors.

Causes ct…

- Physical, sexual and emotional abuse.
- Hormonal changes- especially during menopause.

Signs and symptoms of depression

- Feelings of sadness & unhappiness.
- Irritability & frustration even over small matters.
- Loss of interest or pleasure in normal activities.
- Reduced sex drive.
- Insomnia or excessive sleeping.
- Changes in appetite-decreased/increased.

Ct signs & symptoms

- Angry outbursts.
- Slowed speaking, thinking or body movements.
- Indecisiveness, distractibility, and decreased concentration.
- Fatigue, tiredness & loss of energy.
- Feelings of worthlessness.
- Frequent thoughts of death or suicide.

- Crying spells for no apparent reason.
- Unexplained physical problems-backpain, headache.

MANAGEMENT

- Pharmacotherapy: SSRI's, TCA's, Monoamine oxidase inhibitors.
- Electroconvulsive therapy.
- Psychotherapy: psychological counselling is essential with focus on causes of depression.It helps clients to cope better with stressful life events.
- Encourage client to discuss losses and changes in life situation.
- Assist in developing short term goals.
- Encourage daily exercise.

- Engage client in group activities.
- Promote adequate nutrition: weigh patient weekly to check for weight /gain.
- Ensure adequate sleep and rest.
- Encourage patient to stay out of bed during the day.
- Behavior therapy: Rewarding acceptable behavior and providing social skills.
- Maintenance of social support by family members.
- Physical care: encourage and supervise personal hygiene.
- Prevention of suicide.

- Vagus nerve stimulation- Rx using electrical impulses with a surgically implanted pulse generator to affect the mood of the brain.It helps in chronic treatment of resistant depression.

- Trans cranial magnetic stimulation- The use of powerful magnetic field to alter the brain activity. A large electromagnetic coil is held against your scalp near your forehead to produce an electric current in the brain. Its an option for pple not responding to antidepressants.

NURSING DIAGNOSIS FOR A DEPRESSED PATIENT.

- Risk for suicide related to feelings of worthlessness, depressed mood or anger turned towards self.
- Low self-esteem related to hypersensitivity to criticism/pessimistic look evidenced by self negating verbalization.
- Social isolation/impaired social interaction related to dysfunctional interaction with others as evidenced by withdrawal.

- Disturbed thought processes related to impaired problem solving ability/difficulty in concentrating.
- Imbalanced nutrition less than body requirement related to loss of appetite as evidenced by weight loss and general body weakness.

- Self-care deficit (hygiene, grooming) related to self-neglect as evidenced by uncombed hair& offensive body odour.

BIPOLAR DISORDERS

- Definition- It is a disorder in which there are alternating periods of depression and mania.
- Bipolar mood disorder is further classified into :-
- Bipolar 1
- Bipolar 11 disorders

- Bipolar 1- xterized by episodes of severe mania and severe depression.
- Bipolar 11- xterized by episodes of hypomania and severe depression.

- Bipolar disorder ,manic episodes-symptoms include: hyperactivity, speech pressure, hyper sexuality, delusions, abusive and sleeplessness.
- Bipolar disorder, depressive episodes-symptoms are the same as those of major depression.

- The current episode in bipolar mood disorder is specified as one of the following:-
- Hypomanic
- Manic without psychotic symptoms
- Manic with psychotic symptoms
- Mild or moderate depression
- Severe depression, without psychotic symptoms
- Severe depression with psychotic symptoms
- Mixed or
- In remission

mgt

- Hospitalization- used in acute mania and in depression where there is evidence of poor judgement, weight loss and lack of emotional support.
- Drug therapy with antidepressants or antimanic drugs.
- Antidepressants-tricyclics (imipramine, start at dose of 25-50mg each night and build to 100-150 mg) for 7-10days or until patient is stable.

- Antimanic drugs;-
- If pt displays excitement or disruptive behavior, antipsychotic medication is given-haloperidol 1-5-4mg tds.
- In acute agitation, benzodiazepines may be used together with antipsychotics-diazepam 5-10mg up to 4 times a day.
- Lithium helps relieve mania and depression. Also prevents episodes from recurring. Levels of lithium should be measured frequently to avoid toxicity.

- ECT used in severe mania and depression.
- Psychotherapy.
- Behavioral therapy and focuses on rewarding acceptable behaviour and providing social skills.
- Family therapy and social support.
- Suicide prevention esp for depressed patient.
- Vigorous exercise to improve depression state.

- Heath education on the condition and ways of management, stress coping skills.
- Assist pt and family in learning ways to manage stress.
- Monitoring side effects of drugs.

Self assessment

- Mr. Zame is admitted in the psychiatry ward with a diagnosis of Bipolar 11 disorder.
- Differentiate bipolar 1 and bipolar 2 disorder.
- Explain how you would conduct an MSE on Mr. Zame.
- Describe the management of Mr. Zame.

NEUROSES

OBJECTIVES

- By the end of the session(s), you should be able to:
  - Define neuroses
  - Describe types of neuroses
  - Explain forms/types of anxiety disorders
  - Explain forms/types of somatoform disorders
  - Explain types of dissociative disorders
  - Explain the management of neuroses

DEFINITIONS

- Neuroses are psychiatric disturbances, characterized by excessive anxiety that is expressed directly or altered through defense mechanisms.
- It appears as a symptom, such as an obsession, a compulsion, a phobia, or a sexual dysfunction

Common X-tics of neurosis

- The following are common characteristics of people with neuroses:
  - They are aware that they are experiencing distress.
  - They are aware that their behaviors are maladaptive.
  - They are unaware of any possible psychological causes of the distress.
  - They feel helpless to change their situation.
  - They experience no loss of contact with reality.

Types of neuroses

- The following disorders are examples of psychoneurotic responses to severe anxiety as they appear in the *DSM-IV-TR:*
  - **Anxiety disorders:** Disorders in which the characteristic features are symptoms of anxiety and avoidance behavior
  - **Somatoform disorders:** Disorders in which the characteristic features are physical symptoms for which there is no demonstrable organic pathology.

Cont……

- **Dissociative disorders:** Disorders in which the characteristic feature is a disruption in the usually integrated functions of consciousness, memory, identity, or perception of the environment

ANXIETY DISORDERS

- ANXIETY
- Feeling of apprehension accompanied by sympathetic nervous system arousal e.g. sweating, ↑ HR, ↑RR
- Abnormal fear out of proportion to any external stimulus
- A Psychophysiologic response to a perception of internal or external danger.
- May be spontaneous

TYPES (DSM IV-TR)

- (Panic Attack)
- (Agoraphobia)
- Panic Disorder
- Specific Phobia
- Social Phobia
- Obsessive-compulsive Disorder
- Post-traumatic Stress Disorder
- Acute Stress Disorder
- General Anxiety Disorder

…types

- Anxiety Disorder due to a General Medical condition
- Substance Induced Anxiety Disorder

OTHER TYPES (AD NOS)

- Situational Anxiety
- Anxiety about Death
- Anxiety about Mutilation, Loss of Prowess, or Loss of Attractiveness
- Anxiety about Loss of Self-esteem
- Separation Anxiety
- Stranger Anxiety

. . . Other Types (AD NOS)

- Anxiety about Loss of Control
- Anxiety about Dependency
- Anxiety about Intimacy
- Anxiety about being Punished
- Signal Anxiety
- Etc

PANIC ATTACK

- Abrupt & discrete period of intense fear or discomfort x-terised by
  - Palpitations
  - Sweating
  - Trembling
  - Shortness of breath
  - Feeling of choking
  - Chest discomfort
  - Paresthesias
  - Nausea/abdo. Distress
  - Feeling dizzy
  - Derealization
  - Depersonalization
  - Fear of going crazy
  - Fear of dying
  - Chills or hot flushes

AGORAPHOBIA

- Anxiety about being in places or situations from which escape might be difficult (or embarrassing) or which help may not be available in the event of having an unexpected or situationally predisposed Panic Attack or panic like symptoms.

. . . agoraphobia

- Fears involve situations like:
  - Being outside the home
  - Being on a bridge
  - Traveling in a bus
  - Being in a crowd / standing in a line
- The situations are avoided or else endured with marked distress or anxiety about having a panic attack, or require a companion

PANIC DISORDER

- Recurrent unexpected Panic Attacks
- Persistent concern about having additional attacks
- Worry about the implications of the attack or its consequences
- Significant change in behaviour related to the attacks
- Presence **//** Absence of Agoraphobia

SPECIFIC PHOBIA

- Marked & persistent fear that is excessive or unreasonable, cued by the presence or anticipation of a specific object or situation.
- Exposure to the phobic stimulus almost invariably invokes an immediate anxiety response, which may take the form of a situationally bound/predisposed Panic Attack. (Children = crying, freezing, tantrums, or clinging)
- Adults recognizes that the fear is excessive/unreasonable.
- The phobic situation is avoided or else is endured with intense anxiety or distress
- Distresses / Interferes significantly with functioning

Types of Specific Phobia

- Animal
  - Arachnophobia
  - Zoophobia
  - Ailurophobia
- Natural Environment
  - Acrophobia
  - Aquaphobia
  - Aqua/Hydrophobia
  - Nyctophobia
  - Astraphobia
  - Mysophobia
- Blood-Injection-injury
  - Haematophobia
  - Algophobia
- Situational
  - Claustrophobia
  - Monophobia
  - Ocholophobia
  - (Public transport)
- Others
  - Thanatophobia, HIV/AIDS, Venophobia, Xenophobia, triskaidekaphobia

SOCIAL PHOBIA

- A marked & persistent fear of social or performance situation(s) in which the person is exposed to unfamiliar pple or to possible scrutiny by others.
- Fears that will act in a way that will be humiliating/embarrassing
- Exposure to the feared social situation almost invariably provokes anxiety, which may take the form of a Panic Attack.

. . . Social Phobia

- Person recognizes that the fear is excessive / unreasonable
- The feared situation(s) is avoided or else endured with intense anxiety / distress
- Distress/interfere with functioning
- E.g. Initiating /maintaining conversations; Participating in small groups; Dating; Speaking to authority figures; attending parties.

OBSESSIVE-COMPULSIVE DISORDER

- Obsessions:
  - Recurrent & persistent *thoughts, impulses, or images* that are experienced, at some time during the disturbance, as intrusive & inappropriate & that cause marked anxiety / distress
  - The thoughts are not simply excessive worries about real life problems
  - Person attempts to ignore/suppress/neutralize them with other thought/action
  - Person recognizes that the thoughts are a product of own mind (not imposed from without)

. . . OCD

- Compulsions:
  - Repetitive *behaviours* or mental acts that the person feels driven to perform in response to an obsession, or according to the rules that must be applied rigidly
  - The acts are aimed at preventing or reducing distress or preventing some dreaded event or situation, <u>but</u> the acts either are not concerned in a realistic way with what they are designed to neutralize / prevent / are clearly excessive

. . . OCD

- Person recognizes that the obsessions and compulsions are excessive / unreasonable (but some may not have the insight)
- Significantly interferes with functioning

Examples of OCD

- Washers
- Checkers

PTSD

- Post Traumatic Stress Disorder – exposed to traumatic *event* in which;
  - one experienced, witnessed, or was confronted with an event(s) that involved actual/threatened death/serious injury, or a threat to the physical integrity of self/others
  - one's response involved intense fear, helplessness or horror.

. . . PTSD

- The traumatic event is persistently *re-experienced* as;
  - Recurrent & intrusive distressing recollections of event
  - Recurrent distressing dreams of the event
  - Acting or feeling as if the event were recurring
  - Intense psychological distress at exposure to internal/external cues that symbolize/resemble an aspect of the event
  - Physiological reactivity on exposure to same

. . . PTSD

- Persistent *avoidance* of stimuli associated with the trauma & numbing of general responsiveness by:
  - Efforts to avoid thoughts/feelings/conversations associated with the trauma
  - Efforts to avoid activities/places/pple that arouse recollections of the trauma
  - Inability to recall important aspects of the trauma

. . PTSD

- Markedly diminished interest or participation in significant activities
- Feeling of detachment or estrangement from others
- Restricted range of affect
- Sense of foreshortened future

- Persistent symptoms of increased arousal –
  - Insomnia, irritability, hypervigilance, diff. Conc., exaggerated startle response
- Comes 1 month + after the event

ACUTE STRESS DISORDER

- ("PTSD" of $^{2}/_{7} - {}^{4}/_{52}$)
- Exposed to a traumatic event in which
  - Experienced/witnessed/confronted with an event(s) that involved actual or threatened death or serious injury, or threat to the physical integrity of self/others
  - Response involved intense fear, helplessness or horror

. . . ASD

- Either while / after experiencing the event, dissociative symptoms dev e.g.
  - Subjective feeling of numbing, detachment, or absence of emotional responsiveness
  - Reduction of awareness of surroundings
  - Derealization
  - Depersonalization
  - Dissociative amnesia

. . . ASD

- The traumatic event is persistently re-experienced in as recurrent images/ thoughts/dreams/illusions/flashbacks, sense of reliving the experience, distress on exposure to reminders of the event
- Avoidance+ of stmuli that arouse recollections of the trauma
- Anxiety+/ arousal
- **Lasts 2 days – 4 weeks after event**

GENERAL ANXIETY DISORDER

- Uncontrollable Anxiety++ / worry+ about event(s)/activities
- X-terised by Restlessness, fatigue, irritability, concentration, muscle tension, insomnia
- The signs must have lasted for more than 6 months
- R/o other AD/etc
- S+S causes significant Distress/functioning
- No drug/medical condition

AD due to (A Gen. Med. Cond.)

- The symptoms of this disorder are judged to be the direct physiological consequence of a general medical condition. Symptoms may include prominent generalized anxiety symptoms, panic attacks, or obsessions or compulsions
- R/o other Disorders, delirium etc
- Specify
  - With General Anxiety
  - With Panic Attacks
  - With obsessive-compulsive symptoms

SUBSTANCE INDUCED AD

- X-terised by prominent anxiety symptoms that are judged to be due to the direct physiological effects of a substance (i.e., a drug of abuse, a medication, or toxin exposure)
- The symptoms may occur during substance intoxication or withdrawal and may involve prominent anxiety, panic attacks, phobias, or obsessions or compulsions
- R/o other disorders / delirium / etc

MANAGEMENT - Psychotherapy

- Individual psychotherapy - clients discusses his/her difficulties or fears with a therapist
- Supportive therapy
  - Support adaptive defenses, Reality Testing, Advice, Adaptive Behaviour

Cognitive therapy

- Try to change ones perception of difficulty/fear.
- Achieved using Psychodynamic Therapy
  - Interpretation, confrontation, clarification
- Clients become aware of fears
- Done in sessions

Behaviour therapy

- Systematic desensitization – gradual exposure of client to phobic stimuli either in real or imagined situation. Relaxation is required.
- Implosion therapy (therapy) - Client imagines situation or participates in real life situations that s/he finds extremely frightening for a prolonged period of time. Relaxation is not required.
- Adjunctive techniques e.g. Relaxation, Hypnosis, Biofeedback,

Group/Family therapy

- Good in PTSD
- Clients share experiences with other clients or relatives
- They come to know they are not alone

MANAGEMENT - Pharmacotherapy

- Benzodiazepines e.g. Alprazolam, diazepam, Clonazepam, midazolam, chlordiazepoxide, lorazepam etc
- Antidepressants e.g. Tricyclic (clomipramine etc), SSRIs (Fluoxetine etc), MAOI (Phenelzine)
- Barbiturates e.g. Phenobarbital
- Antihistamines e.g. diphenhydramine,
- Antipsychotics e.g. haloperidol, risperidone
- β adrenergic blockers e.g. Propranolol, Atenolol
- Buspirone

PD & GAD drugs

- Anxiolytics (diazepam, alprezolam, clonazepam, buspirone)
- Antidepresants – (TCAs-imipramine, clomipramine SSRIs – paroxetine, fluoxetine)
- Beta blockers – propranolol
- Alpha 2 receptor agonists - clonidine

Phobias - Drugs

- Anxiolytics
- Antidepressants
  - TCAs
  - SSRIs
  - MAOI – phenelzine
- Beta-blockers

PTSD Mx - Pharmacologic

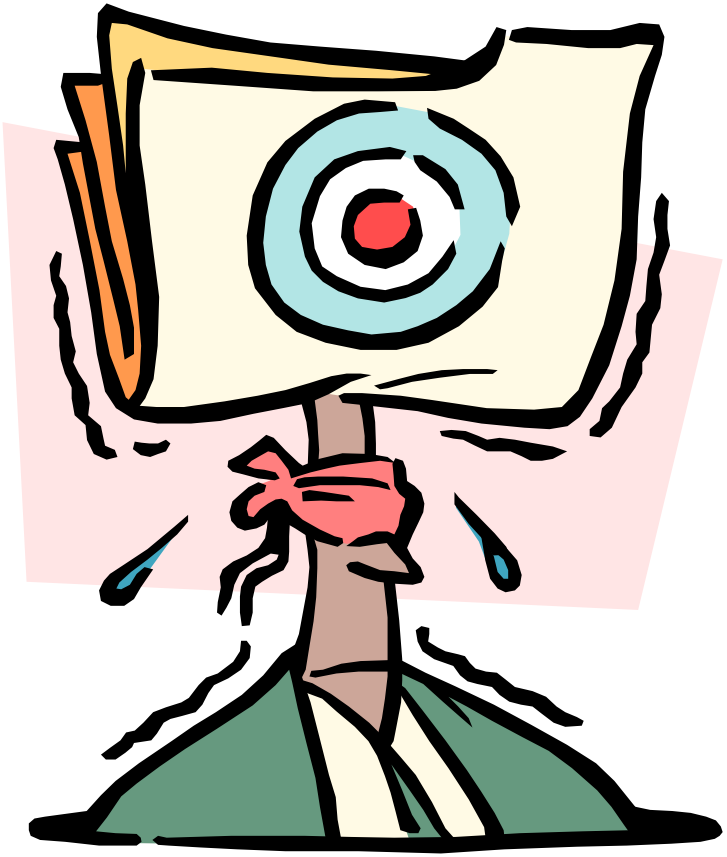
- Anxiolytics
- MAOIs
- SSRIs – first line treatment
- Betablockers
- Carbamazepine
- Valproic
- Lithium

CONCLUSION

- Anxiety disorders involve panic, fear++, apprehension . . .
- Presentation by ANS arousal
- Very common
- Several types
- Management includes specific Rx techniques and medications

WEWE IKO SWALI?

Thank you



ORGANIC DISORDERS

Definition

- Are disorders caused by physical illnesses affecting the brain.
- It represents a group of mental disorders that present with a variety of symptoms, especially a disturbance in cognition.

Include

- Delirium
- Dementia
- Amnesia

DEMENTIA

- This is a slow decline in cognitive functioning, causing multiple changes. Affects advanced ages above 65 years.
- It is characterized by gradual loss of memory & depletion of higher intellectual functioning.
- It is a syndrome where there is disturbance in memory, intellect, personality change, cognitive impairment(amnesia), apraxia (difficulty in complex task).

Clinical features

- Impairment of intellectual functions.
- Impairment in memory/recent memory.
- Deterioration in personality with lack of personal care.
- Impairment in judgement & impulse control.
- Impairment in abstract thinking
- Emotional lability.

- Catastrophic reaction when confronted with an assignment which is beyond the intellectual capacity.
- Thought abnormalities- delusions
- Urinary/ fecal incontinence may develop.
- Disorientation in time, place, and person
- Altered sleep- sleep day time & awake at night.

Presenting complaints

- Forgetfulness & decline in mental functioning.
- Families seek help coz of failing memory, change in personality or behaviour and disorientation.
- Being apathetic/ disinterested.
- Decline in every day functioning-dressing, washing, cooking.
- Decline in memory, thinking, judgement, orientation and language.

Causes/aetiological factors

- Alhzeimers disease
- Infections of the CNS- tertiary syphylis, T.B brain, fungal meningitis, HIV.
- Encephalopathy.
- Vascular dementia/multi-infarct.-caused by poor blood supply to the brain.
- Toxic dementias – drugs, carbon monoxide, benzodiazepines, anticonvulsants.

- Deficiency dementias-pernicious anaemia, folic acid deficiency, thiamine & pellagra.
- Neoplatic dementias- neoplasms & other space occupying lesions.
- Endocrine causes- thyroid, pituitary, adrenal dysfunction.
- Metabolic dementia-chronic hepatic or uremic encephalopathy.

- Traumatic dementias- head injury, chronic subdural haematoma.

Investigations

- Physical exam & history taking- decline in memory/forgetfulness & disorientation, impaired ADLs.
- Total blood count
- VDRL
- Thyroid function test
- CT scan/MRI brain- check lesions
- Brain biopsy
- Lumbar puncture

Nursing diagnosis

- Impaired verbal communication rltd to cerebral impairment as evidenced by altered memory, judgment.
- Bathing / hygiene self care deficit rltd to cognitive impairment as evidenced by inability to complete ADLs.
- Risk for injury rltd to cognitive impairment & wandering behaviour.

- Impaired social interaction rltd to cognitive impairment & difficulty recalling people.
- Risk for violence (self directed or directed to others) rltd to suspicion & inability to recognize people or places.

Management

- Preventive measures especially in vascular dementia eg. Diet, exercise.
- Treat the cause –DM, HTN, syphyllis, deficiencies like B12.
- Administer antipsychotics in very low doses to manage aggression & restlessness & delusions.
- Aspirin is given in low doses in vascular dementia inorder to slow deterioration.

- Give sedatives for abnormal sleep patterns in low doses.NB-Avoid benzodiazepines in elderly as they cause increased confusion.
- Monitor pts vital signs.
- Individual & group psychotherapy with focus on anxiety mgt & changes in lifestyle.
- Give educational & supportive psychotherapy & explain nature & course of the disease.

- Assist client to accept the extent of their disability.
- Give calendars for orientation, give note books for taking notes for memory problems.

Health education

- Reduction of stress & introduction of healthy habits in daily living.
- Acceptance of a spouse as they deteriorate.
- Developing ways of combating disorientation- appointment calendars, clocks, stable routines, setting places for important items.

DELIRIUM

- Delirium is a state of mental confusion that develops quickly and usually flactuates in intensity.
- It is a syndrome or group of symptoms caused by a disturbance in the normal functioning of the brain.

- A delirious patient has a reduced awareness of and responsiveness to the environment, which may be manifested as disorientation, incoherence and memory disturbance.
- Delirium is often marked by hallucinations, delusions and a dream like state.

Epidemiology

- It affects atleast 1 in 10 hospitalized patients, and is a common part of many terminal illnesses. Its common in the elderly the elderly than in the general population

Clinical features

- Diagnostic features include an acute onset, usually over hours or days, of confusion where the patient is disoriented and struggles to understand surroundings. This is evidenced by clouded thinking or awareness.
- This condition is often accompanied by:

. Poor memory.
. Agitation.
. Emotional upset.
. Loss of orientation/disorientation.

- Wandering/altered attention.
- Auditory hallucinations such as hearing voices.
- Visual hallucinations.
- Withdrawal from others.
- Illusions.
- Disturbed sleep (reversal of sleep patterns).
- Autonomic features, for example, sweating, tachycardia.

Risk factors

- Predisposing factors of delirium include :
- Older age, male sex, presence of dementia, depression, immobility, severity of physical illness and metabolic abnormalities.
- Precipitating factors include: narcotics, severe acute illness, hypernatremia, hypoxemia, shock, pain, a high number of hospital admissions.

Causes

- Acronym – I WATCH DEATH
- I- Infections like malaria, typhoid.
- W- Withdrawal(alcohol, sedatives).
- A –Acute metabolic conditions(hypoglycaemia)
- T- Trauma (head injury)
- C- CNS pathology(intracranial space occupying lesions, tumours)

- H -Hypoxia- 0f lungs and heart.
- D- Deficiency- Thiamine, Vit B 12 deficiency.
- E – Endocrine disorders(hyper/hypothyroidism)
- A- Acute vascular conditions like Acute ischaemic attacks.
- T- Toxins (Carbon monoxide poisoning, anticholinergics poisoning)
- H- Heavy metals:lead, mercury.

Pathology

- Underlying cause is lack of oxygen, glucose, amino acids, vitamin deficiency, synthesis and blockage of neurotransmitters, accumulation of toxins, and impaired cerebral blood flow.

Investigations

- Electro-encephalogram
- Neurotransmitter study.
- Brain imaging/CT scan of brain.

Management

- Identify the cause and treat the underlying cause.
- Example- 50ml of 50% dextrose IV for hypoglycemia, Oxygen for hypoxia, IV thiamine 100mg for thiamine deficiency.
- Symptomatic measures .Since many pts are agitated, emergency psychiatric treatment may be needed.
- Antipsychotics in low doses to control agitation, psychotic symptoms or aggression & should be continued for 3-5 days after the delirium has subsided.

- Insomnia use small doses of benzodiazepines (diazepam, lorazepam) incase of alcohol delirium or antipsychotics (haloperidol) in cases not secondary to alcohol, till recovery occurs to cause sedation.
- Nurse pt in a quiet room and well lit room to reduce disorientation.

- Monitor the progress and report adequately on :

❑ Delirium rating scale

❑ Delirium assessing scale

- Provide physical, sensory and environmental support.
- Physical support prevents accidents in the delirious patient.

- Provide adequate sensory stimulation. Too high or low stimulation disorientates them. Provide a friend or relative in their room, give familiar pictures, a clock, calendar.
- Regular orientation to person, place and time make the patient comfortable.
- If delirium is due to severe pain, give analgesics(opioids for both analgesics and sedation).

Nursing diagnosis

1. Risk for trauma related to impairment in cognitive & psychomotor function.

   Need to assist pt with ambulation, keep a dim light on at night and arrange furniture appropriately.

2. Disturbed thought processes rltd to cerebral degeneration as evidenced by disorientation, confusion, memory deficits .

Intervention- frequent orientation of client, placing clocks & calendars strategically and monitor side effects of drugs.

3.Self care deficit rltd to disorientation, confusion & memory deficits as evidenced by inability to fulfill activities of daily living.

Intervention- Provide assistance in self care activities like bathing & involve family members in care of patient.

AMNESIA

- Affects memory mainly.
- Marked by memory impairment & forgetfulness.
- Causes include :
- Head injury
- Alcohol blackout
- ECT effects
- Severe hypoxia
- Herpes encephalopathy

- Anterograde amnesia- impairment in the ability to learn new information.
- Retrograde amnesia- inability to recall previously remembered knowledge.
- Causes- medical conditions like hypoxia .

  -Substance abuse

Mgt

- Depends on the cause.
- Alcoholic blackout treatment with psychotherapy.
- Supportive prompts about date, time, patient location, reduced anxiety & disorientation.
- Explain problem to the patient.

Self assessment

- For the statements given below indicate true or false.
- 1. Individuals with anxiety in public places will be classified as having organic dissociative disorder……
- 2. Changes in the hormonal levels in the brain could be a contributing factor to the development of organic disorders…….
- 3. Individuals with organic disorders may have difficulties concentrating for long periods of time……..

Case scenario

- Mr.Wewe aged 35 years is admitted in the psychiatric ward with alcohol induced delirium.

a) Formulate five nursing diagnosis for Mr.Wewe.

b) Describe the use of delirium rating scale.

c) How would you manage Mr.Wewe till he becomes stable.

CHILD PSYCHIATRY

CLASSIFICATION IN CHILD PSYCHIATRY

- Mental retardation
- Specific development disorders
- Pervasive development disorders- autism
- Hyperkinetic disorders
- Conduct disorders
- Tic disorders- tourette’s disorder
- Ebyresis and encoporesis
- Speech disorders
- Habit disorders

AUTISTIC DISORDER

- Autism is a state of psychotic disorder with its onset in early infancy.
- Its characterized by a withdrawal of the child into self and into a fantasy world of his or her own creation.
- <u>Causes</u>
- Unsatisfactory mother child relationship.
- Genetic.
- Neurological complications like abnormality in brain structure.

Presenting symptoms

Impairment in communication and imaginative activity.

- Language delay or absence of language.
- Abnormal speech patterns & content.E.g echolalia, perseveration.
- Stereotypical or repetitive use of language.
- Lack of spontaneous imaginative play.
- Failure to imitate other’s activities /words.

Impairment in social interactions.

- Lack of interest in social activities.
- Limited eye-eye contact & facial expressions.
- Absent social smile.
- Indifference to affection & physical contact.
- Not able to share enjoyment, interest, or achievement with others.
- Failure to develop friendships or imaginative plays with peers.

Markedly restricted , stereotypical patterns of behaviour, interest and activities.

- Rigid adherence to routines & catastrophic reactions to changes in environment E.g moving furniture.
- Repetitive motor mannerisms E.g head banging, hand biting.
- Pre-occupation with repetitive activities like pouring water, twirling string.
- Persistent pre-occupation with parts of objects.

Other features

- Mental retardation- most autistic children have an IQ of less than 70.Present with mild to profound mental retardation.
- Autistic children enjoy music.
- Epilepsy is common in children with IQ of less than 50.
- Lack of attachment to parents & absence of separation anxiety.

Nursing diagnosis

- Risk for self mutilation related to neurological alterations.
- Impaired social interaction related to inability to trust; neurological alterations.
- Impaired verbal communication related to withdrawal into the self;inadequate sensory stimulation;neurological alterations.

- Disturbed personal identity related to inadequate sensory stimulation; neurological alterations.

Management

- Behavioral therapy.

❖Development of a regular routine with as few changes as possible.

❖Structuring the classroom materials and maintainance of acquired learning.

❖Positive reinforcements to teach self care skills.

❖Speech therapy & sign language teaching.

❖Behavioral techniques to encourage interpersonal relations.

- Psychotherapy.

❖ Parental counselling and supporting psychotherapy are very useful in allaying parental anxiety & guilt, & ensuring their active involvement in therapy.

- Pharmacotherapy.

❖Used to treat autism & associated epilepsy.

❖Haloperidol decreases the dopamine levels in the brain, hence decrease hyperactivity & behavioral symptoms.

❖Risperidone also antipsychotic can be used.

- Other drugs include : SSRIs, chorpromazine, imipramine, multivitamins.
- Antiepileptic medication is used for the treatment of generalized or other seizures.

Nursing mgt.

- Observation to ensure safety & to assist in understanding the individual child.
- Establishment of one-on –one relationships for support, & coordination of services.
- Teach the family about the therapeutic effects & side effects of drugs.
- Special parental support, teaching & reassurance of parents to avoid self blame.

- The nurse should focus on the mother’s significance to her child, enhancing her self esteem & discussing specific plans to deal with child.

ATTENTION DEFICIT HYPERACTIVITY DISORDER (ADHD)

- This problem is referred to as hyperactivity and is characterized by impulsivity(trait of acting without reflection & without thought to the consequences of the behaviour), excessive motor activity & difficulties in sustaining attention.
- It is the commonest psychological referral to mental health & pediatric facilities.
- Its common in boys than girls.
- Occurs before the age of 8 years.

Clinical features

- Excessive or exaggerated muscular activity like harphazard running or fidgeting.
- Difficulty in sustaining attention.
- Being highly distractible.
- Failure to follow instructions or respond to demands made on them.
- Low tolerance to frustration.

- Tend to be of low intelligence due to hyperactivity.IQ 7- 15 which is below average.
- They are socially intrusive and immature.
- Have high rate of driving offences when they attain adolescence.
- They don’t get along with their parents since they don’t obey them.

- Their negative behaviour tends to make their peers to view them negatively.
- They perform poorly in school with specific learning disabilities such as difficulties in reading or learning basic subjects in schools.

Causes

- Genetics- hyperactive parents get children with ADHD.
- Abnormal levels/deficits of neurotransmitters is associated with symptoms of inattention, hyperactivity, impulsivity, mood & aggression.- dopamine, norepinephrine, serotonin.
- Alterations in specific areas of the brain like – frontal lobes, basal ganglia, caudate nucleus, cerebellum.

- Intra-uterine exposure to toxic substances like alcohol.Causes fetal alcohol syndrome
- Perinatal influences like-prematurity, low birth weight, signs of fetal distress, prolonged labour, asphyxia.
- Environmental exposure to lead during childhood.
- Diet factors-food dyes/colourings, preservatives,artificial flavourings

Nursing diagnosis

- Risk for injury related to impulsive and accident- prone behaviour & inability to perceive self harm.
- Impaired social interaction related to intrusive & immature behaviour.
- Low self esteem related to dysfunctional family system & negative feedback.
- Non compliance to task expectations related to low frustration tolerance & short attention span.

Medical mgt

- CNS stimulants- dextroamphetamine are given to increase the attention span, control hyperactive behaviour & improve learning ability.
- Atomoxetine –Selective norepinephrine reuptake inhibitor.
- Bupropion-antidepressant

Nursing Management

- Ensure the client has a safe environment.Remove objects from immediate area on which client could injure self as a result of random, hyperactive movements.
- Identify deliberate behaviours that put the child at risk for injury.Institute consequences for repetition of this behaviour.

- Develop a trusting relationship with the child.
- Discuss with the client those behaviours that are and that are not acceptable.
- Give immediate positive feedback for acceptable behaviour-positive reinforcement.
- Provide an environment for task efforts that is free of distractions as possible .Client is highly distractible hence unable to perform in presence of even minimal stimulation.

- To reduce anorexia side effect of most drugs, the medication may be administered immediately after meals.
- The client should be weighed regularly at least weekly while on therapy with CNS stimulants becoz of potential for anorexia, weight loss & temporary interruption of growth & development.

- To prevent insomnia, the last dose should be administered atleast 6 hrs before bedtime.
- Nurse should be careful to monitor cardiovascular function while pt is on CNS stimulants to avoid sudden death incase of cardiovascular disease.
- Signs of liver damage should be observed while pt is on atomoxetine-itching, dark urine, right upper quardrant pain, yellow skin & eyes,sorethroat, fever, malaise.

Other management

- Behavioral therapy- positive reinforcement is encouraged.
- Family therapy

INTELLECTUAL DISABILITY

- Previously known as mental retardation in DSM IV. Mental retardation is defined as 'significantly sub-average general intellectual functioning, that is accompanied by significant limitations in adaptive functioning' in such areas like self-care, work and safety.
- For one to be diagnosed as having mental retardation, the problem should have started before the age of seventeen years.
- If it occurs thereafter, it is considered as dementia.

- Mental retardation occurs among children throughout the world.
- It tends to increase in severity with age starting at five to six and reaches the peak at aged 15.
- Mental retardation is often a burden to parents and a socio-economic burden to the community.

- Mildly retarded children are the majority and usually appear normal in early stages.
- With adequate resources for education, they are able to learn acceptable social and survival skills such that they lose the stigma of being identified as mentally retarded.

Levels of mental retardation:

- Borderline Mental Retardation: 71-84 IQ
- Mild Mental Retardation: 50-55 to 70 IQ
- Moderate Mental Retardation: 35-40 to 50-55 IQ
- Severe Mental Retardation: 20-25 to 35-40 IQ
- Profound Mental Retardation: below 20-25 IQ

- **Borderline Mental Retardation: 71-84 IQ**
- With early diagnosis, parental assistance and educational programs, they can adjust socially, master simple academic and occupational skills, and become self-reliant.

Mild Mental Retardation: 50-55 to 70 IQ

- They form the majority of those diagnosed as mentally retarded.
- They do not show any brain pathology and for this reason are capable of schooling. However, they require supervision since they are unable to foresee the consequences of their actions.
- Like borderline mental retardation, they can adjust socially, master simple academic and occupational skills, and become self-reliant.

- **Moderate Mental Retardation: 35-40 to 50-55 IQ**
- They are considered trainable, they can master some routine skills like cooking if provided with specialised training in such activities.
- They have the following characteristics:
- Low learning ability and not able to conceptualise.
- Physically they appear clumsy, with poor motor coordination and may suffer from bodily deformities.

- Though they appear unthreatening, they may turn out to be hostile and aggressive.
- They can acquire partial independence with early diagnosis, parental help and adequate opportunities for training.
- In rare occasions, moderately retarded people may be gifted with specialised skills like outstanding musical ability.

- **Severe Mental Retardation: 20-25 to 35-40 IQ**
- These individuals are sometimes referred to as 'dependent retarded'.
- They are observed to have severely retarded speech and motor development.
- Sensory and motor handicaps are also common among the severely mentally retarded.

- They are able to develop limited levels of personal hygiene and self-care skills that make them less dependent on others and although they are dependent on others for care, they can perform simple occupational tasks under supervision after specialised training.

- **Profound Mental Retardation: below 20-25 IQ**
- This condition is sometimes referred to as 'life support retarded'.
- Patients suffering from this condition are usually deficient in adaptive behaviour and unable to master any but the simplest tasks.
- Physical deformities, central nervous system pathology and retarded growth are not uncommon features.

- Other problems include convulsive seizures, mutism, deafness, low resistance to disease, poor health: all this combined make life expectancy quite short.
- The condition is easily diagnosed in infancy due to obvious physical deformities and grossly delayed development. They remain under custodial care throughout their lives.

Causes

- Genetic factors, often due to chromosomal influence and it runs in families, for example, Down's syndrome.
- Infections and toxic agents, especially during foetal development.
- It is also often seen in cases of prematurity and physical trauma. Premature babies, especially those with low birth weight, develop neurological disorders and often mental retardation.

- Ionising radiation. The radiation may act directly on the fertilised ovum or may produce mutations in the sex cells of either or both parents, which in turn may lead to defective offspring.
- Malnutrition and other biological factors also contribute to the condition.

Nursing diagnosis

- Risk for injury related to altered physical mobility or aggressive behaviour.
- Self care deficit related to altered physical mobility or lack of maturity.
- Impaired verbal communication related to developmental alteration.
- Impaired social interaction related to speech deficiencies or difficulty conforming to convectional social behaviour.

- Delayed growth & development related to isolation from significant others; hereditary factors.
- Anxiety (moderate to severe) related to hospitalization & absence of familiar surroundings.
- Ineffective coping related to inadequate coping skills secondary to developmental delay.

Management

Currently there is no cure, though with appropriate support & teaching most individuals can learn to do many things.

There are various homes for the mentally retarded people & schools.

Various medical complications are managed.E.g autistic children are given antipsychotics or mood stabilizers to help with their behaviour

- Behavioral therapy.
- Family therapy-counselling the family on ways of assisting the individual in self care & other activities.
- Healthy lifestyle is encouraged & avoidance of high risk behaviours.

Conduct disorder

- It is a serious behavioral and emotional disorder that can occur in children and teens.
- A child with this disorder may display a pattern of disruptive and violent behavior and have problems following rules.

- It is not uncommon for children and teens to have behavior-related problems at some time during their development.
- However, the behavior is considered to be a conduct disorder when it is long-lasting and when it violates the rights of others, goes against accepted norms of behavior and disrupts the child's or family's everyday life.

Causes

- • Biological: Some studies suggest that defects or injuries to certain areas of the brain can lead to behavior disorders.
- Conduct disorder has been linked to particular brain regions involved in regulating behavior, impulse control, and emotion. Conduct disorder symptoms may occur if nerve cell circuits along these brain regions do not work properly.

- Genetics: Many children and teens with conduct disorder have close family members with mental illnesses, including mood disorders, anxiety disorders, substance use disorders and personality disorders. This suggests that a vulnerability to conduct disorder may be at least partially inherited.

- Environmental: Factors such as a dysfunctional family life, childhood abuse, traumatic experiences, a family history of substance abuse, and inconsistent discipline by parents may contribute to the development of conduct disorder.

- Psychological: Some experts believe that conduct disorders can reflect problems with moral awareness (notably, lack of guilt and remorse) and deficits in cognitive processing.

- Social: Low socioeconomic status and not being accepted by their peers appear to be risk factors for the development of conduct disorder.

Symptoms

- Aggressive behavior: These are behaviors that threaten or cause physical harm and may include fighting, bullying, being cruel to others or animals, using weapons, and forcing another into sexual activity.
- Destructive behavior: This involves intentional destruction of property such as arson (deliberate fire-setting) and vandalism (harming another person's property).

Ct….

- Deceitful behavior: This may include repeated lying, shoplifting, or breaking into homes or cars in order to steal.
- Violation of rules: This involves going against accepted rules of society or engaging in behavior that is not appropriate for the person's age. These behaviors may include running away, skipping school, playing pranks, or being sexually active at a very young age.

- In addition, many children with conduct disorder are irritable, have low self-esteem, and tend to throw frequent temper tantrums. Some may abuse drugs and alcohol.
- Children with conduct disorder often are unable to appreciate how their behavior can hurt others and generally have little guilt or remorse about hurting others.

Diagnosis

- Medical and psychiatric history
- A physical exam
- Laboratory tests – blood tests
- Neuroimaging studies

- Individual Psychotherapy (a type of counseling) is aimed at helping the child learn to express and control anger in more appropriate ways.
- Cognitive-behavioral therapy aims to reshape the child's thinking (cognition) to improve problem solving skills, anger management, moral reasoning skills, and impulse control.

- Family therapy may be used to help improve family interactions and communication among family members.
- A specialized therapy technique called parent management training (PMT) teaches parents ways to positively alter their child's behavior in the home.

Enuresis

- Def; Bedwetting

  Causes

- Poor toilet training
- Delay in ability to hold urine
- Small bladders
- Poor sleep habits
- Problems in hormones that help regulate urination
- Family history of bedwetting

Management

- Positive reinforcement eg. Use of sticker charts for dry nights
- Night time alarm clocks
- Bladder training to help increase bladder size and childs ability to know when they have to urinate
- Decrease fluids and caffeine at night

Assignment

- Read and make notes on sleep disorders

SUICIDE

- It is the commonest cause of death in psychiatric patients.
- Suicide means deliberate self harm.
- Attempted suicide- unsuccessful suicidal act with non-fatal outcomes.
- Parasuicide-Is a self injury that appears to have clear intent to cause bodily harm or death.It is a risk factor to suicide.

- Suicidal gesture- An attempted suicide where the person never intends to die by suicide but sometimes some of those persons may accidentally die during the act.
- Epidemiology
- Suicide is among the top 13 causes of death according to WHO.

Causes

- Genetic- suicidal behaviour tends to run in families.
- Psychiatric disorders-are the major causes of suicide-depression , alcoholism, drug dependence, schizophrenia.
- Physical disorder- pts with incurable or painful physical disorders like cancer, HIV/AIDS.

- Psychosocial factors- marital difficulties, loss of loved ones by death.

Risk factors

- Age –greater than 40 years increases suicidal tendencies.
- Male sex- common in males than in females.
- Staying single.
- Previous suicidal attempts.
- Depression increases risk by 25 %.
- Family history of suicide.

- History of childhood abuse.
- Alcohol & drug dependence.
- Recent serious loss or major stressful life event-death or other loss.
- Untreatable physical illness.
- Unemployment.
- Low frustration tolerance,

Management

- SAD PERSONS scale is used to assess patient.
- S – Sex
- A – Age
- D- Depression
- P- Previous attempts
- E- Ethanol use
- R- Rational thinking loss

- S- Social support lacking
- O- Organized plan
- N- No spouse
- S- Sickness

Assess the lethality of the suicide plan – Evaluation of the suicide plan is important in determining the degree of suicide risk.

3 elements are considered

- Is there a specific plan with details?

✓ Jumping off a high place

✓ Using a gun

✓ Hanging

✓ Poisoning with Carbon monoxide

✓ Slashing ones waist

- How lethal is the proposed method?
- Is there access to the planned method?

Nursing diagnosis

- Risk for suicide related to various emotional states & situational circumstances(feelings of hopelessness, anger, frustration, abandonment, rejection),secondary to loss of job as evidenced by suicide attempt.
- Self care deficit related to frustration evidenced by poor grooming.

- Social isolation related to feelings of fear & shame as evidenced by avoidance of family members .
- Anxiety
- Altered nutrition
- Sleep pattern disturbance

mgt

- Pharmacological intervention:
- Antidepressants for pts with depressive disorders.
- Lithium incase pts have bipolar disorder, since it decreases suicide attempts.
- Antipsychotics- clozapine
- Antianxiety- to RX panic & insomnia- diazepam.

- ECT- decreases acute suicidal ideation.
- Encourage pt to join survivor support groups/networks.
- Teach pt on stress management & coping skills, communication skills after a suicide attempt.
- A focus on personal strengths & positive thoughts & emotions is essential.

- Counseling- should involve a NO- SUICIDE CONTRACT (No harm contract).This is a written contract in which the patient agrees not to harm himself, but to take an alternative action if feeling suicidal .E.g talk with staff.
- Spiritual care- Have pastor / chaplain instill hope concerning gift of life to the patient.

Case scenario

- Mr. Ojay aged 45 years has tried to commit suicide unsuccessfully. He seems very stressed since he was retrenched from work and he has 3 children at the university.
- What do you think might be the triggers to suicide?
- How would you manage Mr. Ojay?

CRISIS

CRISIS

- Def : Any situation in which the individual perceives a sudden loss of his or her ability to use effective problem solving & coping skills.

A number of events /circumstancesc can be considered as crisis:

- Lifethreatening situations
- Sexual assault
- Medical illness

- Mental illness
- Thoughts of suicide
- Loss of relationships-divorce, death of a loved one.

Crisis definition by Caplan

- Psychological disequilibrium in a person who confronts a hazardous circumstance that for him constitutes an important problem which he can for the time being neither escape nor solve with his customary problem solving resources.

Characteristics of crisis

- Crisis occurs in all individuals at one time or another & is not necessarily equated with psychopathology.
- Crises are precipitated by specific identifiable events.
- Crises are personal in nature.
- Crises are acute not chronic & will be resolved in one way or another within a brief period.

- A crisis situation contains the potential for psychological growth or deterioration.

Responses to crises

- Individuals in crises feel helpless to change.
- They don’t believe they have the resources to deal with the precipitating stressor.
- Levels of anxiety rise to the point that the individual becomes nonfunctional, thoughts become obsessional & all behaviour is aimed at relief of the anxiety being experienced.

- The feeling is overwhelming and may affect the individual physically & psychosocially.

Phases in the development of a crises.

PHASE 1

- ❖ The individual is exposed to the precipitating stressor.
- Anxiety increases.
- Previous problem solving techniques are employed.

PHASE 2

- ❖When previous problem solving techniques do not relieve the stressor, anxiety increases further.
- The individual begins to feel a great deal of discomfort at this point.
- Coping techniques that have worked in the past are attempted, only to create feelings of helplessness when they are not successful.
- Feelings of confusion & disorganization prevail.

PHASE 3

- All possible resources , both internal & external are called upon to resolve the problem & relieve discomfort.
- The individual may try to view the problem from a different perspective or even to overlook certain aspects of it.
- New problem solving techniques may be employed, and if effectual, resolution may occur at this phase, with the individual returning to higher, lower or previous level of premorbid functioning.

- PHASE 4
- If the problem is not solved & new coping skills are ineffective, anxiety can overwhelm the person & lead to serious personality disorganization, depression, confusion, violence against others or suicidal behaviour.

Types of crises

❖Emotional crises types

Baldwin (1978) identified 6 classes of emotional crises, which progress by degree of severity.

The type of crisis determines the method of intervention selected.

- Dispositional crises

An acute response to an external situational stressor.

Example:

Mary &Tom married 3 years ago with 1 year old daughter.

Tom has difficulties with his boss at work. This makes him to explode in anger at home & abusive to his wife, because dinner is not ready .He batters the wife & throws away the child. Wife moves to the emergency department in nearest hospital.

- Intervention
- Treat the physical wounds for the wife.
- Send her to a counsellor for guidance & counselling.

- Crises of anticipated life transitions.

- These are normal life cycle transitions that may be anticipated , but over which the individual may feel a lack of control.
- Example- marriage life with the presence of a new baby.

➢ Crises resulting from traumatic stress.

- These crises are precipitated by unexpected, external stresses over which the individual has little or no control, & from which he or she feels emotionally overwhelmed & defeated.
- Example :Gang rape when someone is on night shift or travelling at night
- Intervention-Talk to the client about the experience & ventilate feelings associated with it.
- Offer reassurance & support.

- Discuss stages of grief & how rape causes loss of self worth.
- Identify support systems that can help client resume normal activities.
- Explore new methods of coping with emotions arising from a situation with which she has had no previous experience.

➢ Maturational / developmental crises

- These types of crises occur in response to situations that trigger emotions related to unresolved conflicts in ones life.
- These crises are of internal origin & reflect underlying developmental issues that involve dependency, value conflicts, sexual identity & capacity for emotional intimacy.

➢ Crises reflecting psychopathology

- Emotional crises in which preexisting psychopathology has been instrumental in precipitating the crises or in which psychopathology significantly impairs or complicate adaptive resolution.
- Examples of psychopathology that may precipitate crisis.-Severe neuroses, schizophrenia.

➢ Psychiatric emergencies

- Crises situations in which general functioning has been severely impaired

& the individual rendered incompetent or unable to assume personal responsibility.

Examples of psychiatric emergencies:

Acute suicide

Drug overdoses

Acute psychosis

Uncontrollable anger

- Alcohol intoxication.
- Aggressive behaviour/violence

Other types of crises

❖Situational crises

- Arises from events that are extraordinary, externally rather than internal and often unanticipated.
- Events that precipitate situational crises include :change of job, death of a loved one, an abortion, change in financial status, divorce , sever physical &mental illneses.

❖Adventitious crises

- These are rare/ unexpected/ disastrous events that are unplanned & may be accidental.
- Can be manmade or natural.
- These crises results from;
- Natural disasters-flood, fire, earthquake.
- National disaster- acts of terrorism, war
- Crimes of violence- rape, assault, murder, bombing, child abuse.

CRISES INTERVENTION

- Refers to the methods used to offer immediate , short term help to individuals who experience an event that produces emotional, mental, physical & behavioral distress or problems.

Purposes of crises intervention

- It helps to reduce the intensity of an individuals emotional, mental, physical, & behavioral reactions to a crises.
- To help the individuals return to their levels of functioning before the crises /develop new coping skills.
- Aim to assist the individual in recovering from the crises & to prevent serious longterm problems from developing.

Approaches in crises intervention

- **Short-term Crisis Intervention Therapy**
- It is of brief duration and deals with immediate problems of emotional nature.
- The therapist provides as much help as the individual or family will accept.
- The therapist helps by clarifying the problem, suggests a plan of action, and provides reassurance and gives the needed information and support.

- **Immediate Crisis Intervention Services**
- Timing is critical in crisis intervention since it can reduce the emotional distress.
- A crisis counsellor provides objective emotional support and tries to provide long-term perspective to allow the victim to see that there is hope of survival.

- **Post Disaster Debriefing Sessions**
- The desire to 'unwind' in a psychologically safe environment and to share one's experience of the disaster is a universal need of people who have suffered through a traumatic situation.
- Victims are, therefore, encouraged to narrate their experiences with professionals and colleagues.
- This helps each victim to learn how different people react to a traumatic situation.

PHASES OF CRISES INTERVENTION

- Phase 1 – Assessment
- A nurse performs the following assessments :
- Ask the individual to describe the event that precipitated the crises.
- Determine when it occurred.
- Assess individuals physical & mental status.
- Assess coping methods used & results.

- Make diagnosis:
- Ineffective individual coping
- Anxiety
- Altered thought processes
- Potential for violence

- Phase 2- Planning
- Nurse selects appropriate nursing actions for the identified diagnosis.
- Phase 3 – Intervention
- Actions identified in phase 2 are implemented.

Nurse should focus on ;

- Use of reality oriented approach to solve crises.

- Remaining with the individual facing panic anxiety.
- Establish a rapid working relationship.
- Clarify problem the individual is facing.
- Help individual identify what precipitated crises.
- Guide the individual through problem solving.
- Identify external support systems to assist individual.

- Phase 4- Evaluation of crises resolution & anticipatory planning.
- To evaluate the outcome of crises intervention, a re-assessment is made to determine if the stated objectives were achieved.

Ask yourself these quiz.

- Have positive behavioural change occurred?

- Has the individual developed more adaptive coping strategies?
- Have they been effective?
- Has the individual grown from the experience by gaining insight?
- Does the individual believe that he/she could respond with healthy adaptation in future stressful situations to prevent crisis development?

Self assessment

- Why do most individuals undergo crisis?
- List atleast four psychiatric emergencies.
- What are the types of emotional crises?

PSYCHOSEXUAL DISORDERS

a) SEXUAL DYSFUNCTIONS

This is a disturbance in the desire, excitement, or orgasm phases of the sexual response cycle or pain during intercourse.

1. SEXUAL DESIRE DISORDERS

These are based on damage to biological sex drive, self esteem, acceptance of personal sexuality, sexual experiences and relationships

I. Hypoactive Sexual Desire Disorder

Persistently or recurrently deficient (or absent) sexual fantasies and desire for sexual activity.

May be caused by chronic stress, depression or a deteriorating relationship.

Common in both sexes.

ii. Sexual Aversion Disorder

Persistent or recurrent extreme aversion to, and avoidance of genital sexual contact with a sexual partner or by masturbation.

This person is disgusted in sexual situations.

2. Sexual arousal disorders

i. Female Sexual Arousal Disorder

Persistent or recurrent inability to attain, or to maintain an adequate lubrication and swelling response of sexual excitement until the completion of the sexual act.

May be lifelong or acquired.

May result from hormonal alterations in testosterone, estrogen, prolactin and thyroxin.

Use of antihistamines and anticholinergics.

ii. Male Erectile Disorder

Persistent or recurrent inability to attain, or to maintain an adequate erection to perform the sexual act.

Man with a lifelong erectile dysfunction has never been able to obtain an erection sufficient for intercourse.

It may also be acquired.

3. ORGASMIC DISORDERS

i. *Female Orgasmic Disorder*

Persistent or recurrent delay in, or absence of, orgasm following a normal sexual excitement phase.

It may be lifelong- never having achieved orgasm or acquired.

Physiological factors include fear of pregnancy, rejection or hostility towards men.

ii. Male Orgasmic Disorder

Persistent or recurrent delay in, or absence of, orgasm following a normal sexual act.

Lifelong- one has never been able to ejaculate during coitus.

Acquired orgasmic disorder develops after previously functioning.

Causes- interpersonal problems, substance abuse, some medications.

iii. Premature Ejaculation

Persistent or recurrent ejaculation with minimal sexual stimulation before, on, or shortly after penetration and before the person wishes it.

psychological factors - newly married, fear about performance, stressful relationships etc.

Physical factors- some men may be tactilely sensitive, responding more intensely to stimulation.

4. SEXUAL PAIN DISORDERS

i. *Dyspareunia*

Recurrent or persistent genital pain associated with sexual intercourse in either a male or a female.

Psychological factors include childhood abuse or rape, anxiety about sex.

This makes sex unpleasant.

ii. Vaginismus

This is an involuntary constriction response of the muscles that close the vagina.

This interferes with penile penetration.

It may also be elicited during a pelvic exam.

OTHER SEXUAL DISORDERS

- Substance induced sexual disorder

- Sexual disorder due to a general Medical condition.

- **Read**

SEXUAL DISORDERS

PARAPHILIAS

Definition

Paraphilias(sexual impulse disorder) are acts of sexual stimuli or acts that are deviations from normal sexual behaviors (what is expected by the society).

These are recurrent intense sexually arousing fantasies, sexual urges, or behaviors they involve nonhuman articles and nonconsenting patterns.

They must have been there for at least 6 months.

State 6 types of paraphilias

i. Exhibitionism

This is an illegal activity that involves the intentional display of the genitals in a public place.

100% occur in men exposing themselves to women.

Excitement results from the anticipation of the act, he ejaculates during or after the exposure.

he is aroused by observers response of shock and disgust.

ii. Fetishism:

It involves recurrent, intense, sexual urges involving the use of nonliving objects.

The sexual focus is on objects like female undergarments, shoes, stocking etc associated with the human body.

The fetish items are used during masturbation/ release sexual excitement.

Transvestic fetishism-cross dressing (womens clothing)

iii. Frotteurism

- This is recurrent preoccupation with intense sexual urges and behaviors common in men.
- Sexual excitement is derived from the actual touching or rubbing against a non consenting person using his penis.
- The frotteur does the act in crowded places.

iv. Pedophilia

Its characterized by recurrent, sexually arousing urges, or behaviors involving sexual activity with a prepubescent child or children.

The molester is at least 16 yrs.

Child molestations involve fondling or oral sex.

Others limit it to undressing the child and looking , exposing themselves, masturbating in their presence.

v. Sexual Masochism

- This is recurrent, intense, sexually arousing fantasies, sexual urges, or behaviors involving the act (real, not simulated) of being humiliated, beaten, bound, or otherwise made to suffer.

vi. Sexual Sadism

This involves the achievement of sexual satisfaction from the physical or physiological suffering (including humiliation) of the victim.

The sadist inflicts pain and suffering on a (usually) non/consenting person.

The pain of the victim is sexually exciting to the sadist, giving him sexual excitation.

Examples-restraint, beating, burning, rape, cutting, torture and even killing.

vii. Voyeurism:

This involving the act of observing an unsuspecting person who is naked, in the process of disrobing, or engaging in sexual activity.

Sexual excitement is achieved through the act of looking and no contact with the person is attempted.

Mastabating accompanies the “window peeping” but may occur later as the individual fantasizes about the voyeuristic act.

Paraphilia Not Otherwise Specified

i. Telephone and computer scatologia

Its characterized by obscene phone calling on an unsuspecting person or sending obscene messages or video images via email.

The conversation is accompanied by masturbation, which is often completed after the contact is interrupted.

Necrophilia

- Its an obsession with obtaining sexual gratification from cadavers.

iii. Partialism

Concentrate their sexual activity on one part of the body to the exclusion of all others. Examples include:

- *Cunnilingus*: oral contact with a woman's external genitals.
- *Fellatio* :oral contact with the penis.
- *Anilingus*: oral contact with the anus.

iv. Zoophilia,

- Animals, which may be trained to participate, are preferentially incorporated into arousal fantasies or sexual activities, including intercourse, masturbation, and oral-genital contact.

v. Coprophilia

Is sexual pleasure associated with the desire to defecate on a partner, to be defecated on, or to eat feces *(**coprophagia**)*.

A variant is the compulsive utterance of obscene words *(**coprolalia**)*.

These paraphilias are associated with fixation at the anal stage of psychosexual development.

vi. urophilia

- Its involves urinating on ones partner or being urinated on, as part of sexual stimulation.

viii. Hypoxyphilia

- is the desire to achieve an altered state of consciousness secondary to hypoxia while experiencing orgasm.
- Persons may use a drug (e.g., a volatile nitrite or nitrous oxide) to produce hypoxia.

READ ON THE MANAGEMENT OF PARAPHILIAS

Self assessment

- Define the following terms;
- Voyeurism
- Frouterism
- Exhibitionism

DISORDERS OF PERSONALITY

PERSONALITY

- Personality is those features of the individual which determine that individual's unique adjustment to the environment.
- Personality is "lifelong and persistent" and involves "enduring characteristics and attitudes" which influence the individual's ways of thinking, feeling and behaving.
- Personality can change over time.

Personality disorder

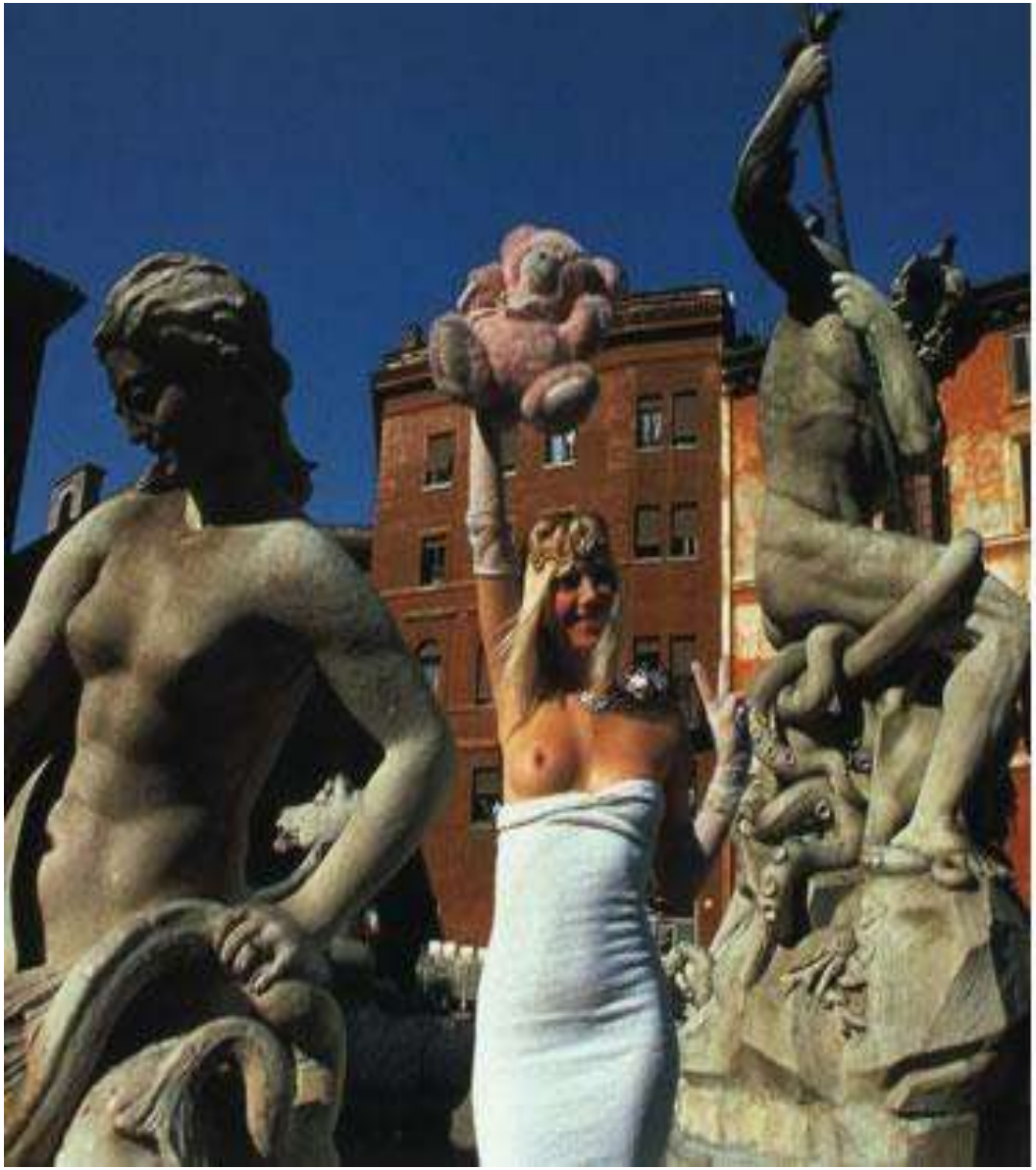
- Defn:

"A Personality disorder is an enduring pattern of inner experience and behaviour that deviates markedly from the expectations of the individual's culture, is pervasive and inflexible, has an onset in adolescence or early adulthood, is stable over time and leads to distress and impairment" (DSM-IV-TR).

Examples

- People with “neurotic disorders” (e.g. anxiety disorder) have “autoplastic defenses” i.e. they react to stress by changing their internal psychological process, and perceive their symptoms unacceptable, objectionable and needing to be changed.
- In contrast, People with personality disorders, have “alloplastic defences”i.e. they react to stress by attempting to change the external environment (rather than themselves), and perceive their symptoms to be acceptable, unobjectionable and not in need of change.

What are the striking differences from the two photos?



CLUSTER – A PERSONALITY DISORDERS (Odd/Eccentric)

Examples

- Paranoid personality disorder
- Schizotypal personality disorder
- Schizoid personality disorder

1. Paranoid Personality disorder

DSM – IV TR Diagnostic criteria:

A pervasive distrust and suspiciousness of others such that their motives are interpreted as negative without justification. (at least *four* of the following).

(i) Suspects others are exploiting, harming or deceiving him.

(ii) Doubts the loyalty or trustworthy of others.

(iii) Fears that information given to others will be used maliciously against him.

(v) Reads hidden demeaning or threatening meanings in what others are saying.

(v) Persistently bears grudges, i.e., is unforgiving of insults, injuries, or slights

(vi) Perceives attacks on his or her character or reputation that are not apparent to others and is quick to react angrily or to counterattack

(vii) Has recurrent suspicions, without justification, regarding fidelity of spouse .

- Does not occur exclusively during the course of schizophrenia, a mood disorder with psychotic features, or another psychotic disorder and is not due to the direct physiological effects of a general medical condition.

Treatment of PPD

(i) Psychotherapy.

(ii) Anxiety is treated with anxiolytics.

(iii) Use of antipsychotics.

Example of an Individual with PPD

A 57-year-old man living in a condominium complex constantly accuses his neighbors of plotting to avoid payment of their share of maintenance. He writes angry letters to other owners and has iriitiated several lawsuits. He lives alone and does not socialize.

2. Schizoid personality disorder

DSM –IV –TR Diagnostic Criteria

A pervasive pattern of detachment from social relationships and a restricted range of expression of emotions in interpersonal settings, beginning by early adulthood and present in a variety of contexts, as indicated by four (or more) of the following:

(i) Neither desires nor enjoys close relationships, including being part of a family.

(ii) Almost always chooses solitary activities.

(iv) Has little, if any, interest in having sexual experiences with another person.

(iv) Takes pleasure in few, if any, activities.

(v) Lacks close friends or confidants other than first-degree relatives

(vi) Appears indifferent to the praise or criticism of others

(vii) Shows emotional coldness, detachment, or flattened affectivity.

Treatment of Schizoid PD

(i) Individual psychotherapy e.g. Counselling.

(ii) Use of antipsychotics, antidepressants and psychostimulants.

Example of an Individual with Schizoid PD

A 24-year-old man lives alone and works nights as a security guard. He ignores invitations from coworkers to socialize and has no outside interests.

3. Schizotypal personality disorder

DSM – IV – TR Diagnostic Criteria

A. A pervasive pattern of social and interpersonal deficits marked by acute discomfort with, and reduced capacity for, close relationships as well as by cognitive or perceptual distortions and eccentricities of behavior, beginning by early adulthood and present in a variety of contexts, as indicated by five (or more) of the following:

(i) Ideas of reference (excluding delusions of reference).

(ii). Odd beliefs or magical thinking that influences behavior and is inconsistent with sub-cultural norms (e.g., superstitiousness).

(iii) Unusual perceptual experiences, including bodily illusions.

(iv) Odd thinking and speech (e.g., vague, circumstantial, metaphorical, over-elaborate, or stereotyped).

(v) Suspiciousness or paranoid ideation.

(iii) Inappropriate or constricted affect.

(iv) Behavior or appearance that is odd, eccentric, or peculiar.

(v) Lack of close friends or confidants other than first-degree relatives.

(ix) Excessive social anxiety that does not diminish with familiarity and tends to be associated with paranoid fears rather than negative judgments about self.

Treatment for Schizotypal PD

(i) Psychotherapy (treatment of choice).

(ii) Antidepressants and antipsychotics).

Example of an Individual with Schizotypal PD

A 30-year-old man is completely preoccupied with the study and the brewing of herbal teas. He associates many peculiar powers with such infusions and says that plants sometimes whisper to him. He spends all of his time alone, often taking solitary walks in the wilderness for days at a time, collecting plants for teas. He has no history of disorganized behavior.

CLUSTER B PD’s(Dramatic/ emotional/ labile/ erratic behaviour.

Examples:- Antisocial, Borderline, Histrionic, Narcissistic

1. Antisocial personality disorder

- A pervasive pattern of disregard for and violation of the rights of others occurring since age 15.
- DSM – IV - at least three of the following present:

- Failure to conform to social norms by repeatedly engaging in unlawful activity.
- Repeated lying or “conning” others for profit or pleasure.
- Failure to plan ahead.
- Irritability and aggressiveness as indicated by repeated physical fights.
- Failure to sustain consistent work or honour financial obligations.
- Destruction of property.
- Theft and violation of roles.

Treatment

- Inpatient self-help groups are useful.
- Psychotropic drugs can be used.

EXAMPLE

A 22-year-old man was recently arrested after he set his mother's house on fire. He has had numerous problems with the law, which started at an early age when he was sent to a juvenile detention center for his behavior at both home and school. He lacks remorse for setting the fire and expresses a desire that his mother would have died in the fire.

2. Borderline personality disorder

- A pervasive pattern of instability of interpersonal relationships, self image and effects, and marked impulsivity.
- DSM – IV criteria – at least 5 of the following present:

- Frantic efforts to avoid real or imagined abandonment.
- Unstable interpersonal relationships.
- Identity disturbance-Unstable self image
- Impulsivity (promiscuity, spending a lot, reckless driving etc.).
- Recurrent suicidal behaviour – self –mutilating behaviors, gestures or threats.
- Affective instability e.g. Irritability and anxiety.
- Chronic feelings of emptiness.
- Anger or difficulty controlling anger.
- Paranoid ideations or severe dissociative symptoms.

Treatment

- Psychotherapy.
- Pharmacotherapy.
- Suicidal management.

Self destructive behaviors

EXAMPLE

A 20-year-old nurse was recently admitted after reporting auditory hallucinations, which have occurred during the last few days. She reports marriage difficulties and believes her husband is to blame for the problem. She has several scars on her wrists and has a history of substance abuse.

3. **Histrionic personality disorder**

- A pervasive pattern of excessive emotionality and attention seeking behaviours.
- DSM – IV criteria – at least 5 of the following:-

– Not comfortable unless he is the centre of attention.
– Sexually seductive or provocative with others.
– Shallow and shifting emotional expressions.
– Use of physical appearance to attract attention.
– Excessive speech lacking details.
– Dramatic, theatrical and exaggerated emotions.
– Easily influenced by others and circumstances.
– Believes that all relationships intimate.

Treatment

- Insight – oriented psychotherapy.
- Anti-depressants.

EXAMPLE

A 30-year-old woman presents to the doctor's office dressed in a sexually seductive manner and insists that the doctor comment on her appearance. When the doctor refuses to do so, she becomes upset.

4. Narcissistic personality disorder

- A pervasive pattern of grandiosity (in fantasy and behavior), need for admiration and lack of empathy.
- DSM – IV Criteria (at least 5)

- Exaggerated sense of self importance.
- Preoccupation with fantasies of unlimited success, power, brilliance, beauty or ideal love.
- Believes he/she is “special” and should be understood or associated with high status people or institutions.
- Require excessive admiration.
- Have a sense of entitlement.
- Takes advantage of others to achieve his own ends.
- Lacks empathy.
- Is envious of others.
- Has arrogant behaviours.

Treatment

- Psychotherapy
- Antidepressants as adjunctive therapy.

EXAMPLE

A famous actor is outraged when a director questions his acting abilities during rehearsal for a play. The actor responds by walking off the stage and not returning to the stage unless the director apologizes publicly for her behavior.

CLUSTER C PERSONALITY DISODERS (Anxious Fearful, Neurotic, Introverted, Inhibited)

Examples: **avoidant, dependent and obsessive compulsive**

1. Avoidant personality disorder.

- A pervasive pattern of social inhibition, feelings of inadequacy, and hypersensitivity to negative evaluation.

- DSM – IV Criteria (at least four)

- Fears occupational activities with significant inter-personal contact due to fear of rejection or criticism.
- Unwilling to relate with people unless guaranteed.
- Restrains from intimate relationship due of fear of being ashamed or ridiculed.

- Inhibited in new interpersonal situations due to feelings of inadequacy.
- preoccupied with being criticised in social places.
- Low self esteem.
- Fear embarrassment.

Treatment

- Individual, group and behavioural psychotherapy.
- ß-Blockers to treat anxiety.
- Antidepressants and anxiolytics

2. **Dependent personality disorder**

- A pervasive and excessive need to be taken care of that leads to submissive and clinging behavior and fear of separation
- DSM – IV criteria (at least five).

- Difficulty in making decisions without excessive advice and reassurance.
- Need others to assume responsibility for major areas of his/her life.
- Difficulty in expressing disagreements.
- Desires support to an extent of volunteering to do unpleasant things.
- Sees self as helpless.
- Urgently seeks another source of care when a close relationship ends.
- Unrealistically preoccupied with fears of being left to care for self.

Treatment

- Insight oriented psychotherapy.
- Group and behavioural therapies e.g. social skills.
- Family therapy.

3. Obsessive compulsive personality disorder

- A pervasive pattern of preoccupation with orderliness, perfectionism and mental interpersonal control at the expense of flexibility, openness and efficiency.
- DSM – IV criteria (at least 4 present).

- Preoccupied with details, roles, lists, schedules etc.
- Perfectionism interferes with task completion.
- Excessive devotion to work at expense of other things.
- Over consciousness.
- Unable to discard worn-out or worthless objects even if they have no value.
- Reluctant to delegate tasks to others.
- Miserly spending style toward both self and others.
- Rigidity and stubbornness.

Treatment

- Long term individual therapy.

EXAMPLE

A 37-year-old woman seeks psychotherapy as a result of an impending divorce. She says that her demands that the house be kept spotless, that extremely detailed and fixed work and recreational schedules be maintained, and that rigid dietary habits be observed have driven her spouse away.

Self assessment

- Outline four characteristics of histrionic personality disorder.
- What are the key characteristics of an individual with antisocial personality disorder?

SUBSTANCE USE DISORDERS

SUBSTANCE ABUSE

Substance abuse refers to the harmful or hazardous use of psychoactive substances, including alcohol and illicit drugs.

Psychoactive substance use can lead to *dependence syndrome* - a cluster of behavioral, cognitive, and physiological phenomena that develop after repeated substance use .

List 5 commonly abused drugs

Some commonly abused drugs

- Alcohol
- Tobacco
- Cocaine
- Marijuana
- Heroin

List 6 Risk Factors to Substance abuse

- Family member with an alcohol problem (family history)
- Male gender
- Living alone
- Single status (separated or divorced)
- Mental health issues, including depression or other chronic mental health problems
- Substance abuse in earlier life
- Chronic pain
- Sleep problems

- Life stresses (financial difficulties, retirement, loss of a spouse or family member, moving to a new home, new illness)
- Living in a long-term care facility
- Long-term tobacco habit
- Misunderstanding by older people about how to take their medications
- Boredom.

State 5 direct causes of Substance abuse

(i) **Curiosity**-drugs are abused on a regular basis after discovering the drug's ability to produce euphoric affects.

(ii) **Low Self Esteem**- means of finding acceptance from others, if they are lacking social confidence or self esteem.

(iii) **Tolerance**-This occurs when an individual requires more of the same substance to obtain the same level of intoxication previously reached with a smaller amount of the substance.

(iv) **Medical prescriptions:** Many substance abusers began using prescription drugs for pain control, gradually becoming physically and emotionally dependent on their prescribed medication.

(v) **Hereditary Tendencies:** Addiction at times runs in the family

State 5 Clinical Manifestations of Substance abuse

(i) Giving up past activities such as sports, homework, or hanging out with new friends.

(ii) Declining grades.

(iii) Aggressiveness and irritability.

(iv) Significant change in mood or behavior.

(v) Forgetfulness.

(vi) Disappearing money or valuables.

(vii) Feeling hopeless, depressed, or even suicidal ideas.

(viii) Sounding selfish and not caring about others.

(ix) Use of room deodorizers and incense.

(x) Physical problems with unclear cause (for example, red eyes and slurred speech).

(xi) Getting drunk or high on drugs on a regular basis.

(xii) Lying, particularly about how much alcohol or other drugs he or she is using.

Seek medical attention in Substance Abuse

When the following develops

- Withdrawal seizures.
- Jaundice
- Increasing abdominal girth.
- Leg swelling
- Cough that won't go away
- Feelings of sadness or depression
- Pain at an injection site
- Fever
- Chest pain

- Thoughts of self harm or harm to others
- Chest pain, rapid heartbeat, difficulty breathing, or lightheadedness
- Severe abdominal pain
- Confusion or ongoing hallucinations
- Difficulty speaking, numbness, weakness, severe headache, visual changes, or trouble keeping balance
- Severe pain at an injection site

ALCOHOL

Alcohol is a generic term for **ethanol**, which is a particular type of alcohol produced by the fermentation of many foodstuffs - most commonly barley, hops, and grapes.

Alcohol depresses the brain, can suppress some body functions and, in extreme cases result in death.

Effects of alcohol

(i) Alcohol affects the body and mind immediately.

(ii) Basic human functions, such as walking and talking are impaired.

(iii) Alcohol may lead to illness such as nausea, diarrhea and dehydration.

Alcohol Intoxication

Alcohol intoxication/poisoning occurs following the ingestion of a large amount of alcohol that impairs mental and physical abilities.

Inexperienced drinkers, or those sensitive to alcohol, may become acutely intoxicated and suffer serious consequences after ingesting smaller amounts of alcohol.

Predisposing factors to Alcohol Intoxication

Strength/Concentration of Drink:

Stronger drinks will result in higher blood alcohol concentration. The higher the alcohol content of the drink the more alcohol irritates the mucous membranes of the gastrointestinal tract, slowing down the absorption rate of the alcohol.

Body Weight/Body Type:

The less an individual weigh, the more he/she is likely to be affected by a given amount of alcohol.

Gender:

Women have less of the enzyme *alcohol dehydrogenase* that breaks down alcohol in the stomach contributing to higher alcohol levels. Hormone levels also affect the body's ability to process alcohol.

Mood:

Stress emotions such as (depressions, anxiety and anger) can also cause a change of enzyme concentrations in the stomach and how one may process the alcohol.

Rate of Consumption:

The faster a person consumes drinks, the quicker the blood alcohol concentration will rise.

Medications:

It is important to know the drug interactions and consult with a physician before mixing any medication with alcohol.

Symptoms of Alcohol Intoxication

(i) Repeated episodes of vomiting.

(ii) Unconsciousness or semi-consciousness.

(iii) Slowed or irregular breathing. Slow respiration, eight or less breaths per minute or lapses of more than *ten* seconds.

(iv) Cold clammy or pale or bluish skin.

(v) Vomiting while "sleeping" or passed out, and not waking up after vomiting.

DSM-IV-TR Diagnostic Criteria for Alcohol Intoxication

A. Recent ingestion of alcohol.

B. Clinically significant maladaptive behavioral or psychological changes (e.g., inappropriate sexual or aggressive behavior, mood lability, impaired judgment, impaired social or occupational functioning) that developed during, or shortly after, alcohol ingestion.

C. One (or more) of the following signs, developing during, or shortly after, alcohol use:

1. Slurred speech
2. Incoordination
3. unsteady gait
4. Nystagmus(fast, involuntary movement of the eyes)
5. impairment in attention or memory
6. stupor or coma.

D. The symptoms are not due to a general medical condition and are not better accounted for by another mental disorder.

Home Care for Alcohol Intoxication

Remove the person from all sources of alcohol, e.g. bar or party. If in at a home environment, get rid of all the alcohol.

Provide a safe environment (prevent falls, keep away from dangerous machines and objects, keep from driving vehicles).

Find out if only ethanol has been ingested. Determine if the person has taken any medications, illegal drugs, or non-ethanol alcohols.

See if the person is easy to arouse when spoken to or with a slight shake of a shoulder.

Make sure that the person's condition is not due to an underlying medical cause or injury.

Have someone in constant attendance to ensure that the person is improving, and to obtain medical assistance as needed.

If the person does not feel comfortable monitoring the condition of the intoxicated person, then it would be best to take the person to the hospital.

Management of Alcohol Intoxication

- Alcohol Intoxicated individuals can be given IV fluids and *B-Complex vitamins* for dehydration (alcohol is a diuretic and increases urine output) and as a precaution or treatment for vitamin deficiency.
- Dextrose 10 – 50% can be administered to promote brain metabolism and prevent hypoglycemic crisis
- In severe cases - those of severe stupor and coma - the person should be intubated to support respirations and to protect the lungs from filling with vomit/secretions.

Other management Strategies for Alcohol Intoxication

(i) Ensure safety

(ii) Psychotherapy

(iii) Rehabilitation

(iv) Refer to support group e.g. alcohol anonymous

(v) Antidote of alcohol- *Disulfiram*.

(vi) Pharmacology e.g. Antipsychotic agents

Complications of Alcohol Intoxication

- Liver failure
- Cancer
- Chronic pancreatitis
- Psychosis

Prevention of Alcohol related disorders

(i) Reducing the supply of illegal substances by destroying crops, crop substitution.

(ii) Prosecution of big scale traffickers and substance dealers this reduces substance availability on the streets.

(iii) Control through medical prescription

(iv) Health education to parents and their children, to provide information in order to correct children's misperceptions about cigarettes, alcohol, and drugs and the consequences of their use

Alcohol withdrawal

Alcohol withdrawal refers to a set of symptoms seen when an individual reduces or stops alcohol consumption after a prolonged periods of excessive alcohol intake.

These symptoms are largely due to the central nervous systems being in a hyper-excitable state.

Withdrawal symptoms include

(i) Early withdrawal symptoms include anxiety, palpitations, nausea and anorexia.

(ii) Withdrawal seizures.

(iii) Alcohol hallucinosis

(iv) Hallucinations –visual, tactile and auditory.

(v) Delirium tremens(changes in mental status and nervous system like agitation, irritability, confusion, tachycardia, hypertension.)

DSM-IV-TR Diagnostic Criteria for Alcohol Withdrawal

A. Cessation of (or reduction in) alcohol use that has been heavy and prolonged.

B. Two (or more) of the following, developing within several hours to a few days after Criterion A:
   1. Autonomic hyperactivity (e.g., sweating or pulse rate greater than 100)
   2. Increased hand tremor.
   3. Insomnia
   4. Nausea or vomiting
   5. Transient visual, tactile, or auditory hallucinations or illusions.
   6. Psychomotor agitation
   7. Anxiety
   8. Seizures

C. The symptoms in Criterion-B cause clinically significant distress or impairment in social, occupational, or other important areas of functioning.

D. The symptoms are not due to a general medical condition and are not better accounted for by another mental disorder.

SELF DIRECTED LEARNING

Read on the Management strategies for Alcohol Withdrawal

Prognosis of Substance Abuse

Individuals who suffer from substance abuse tend to be more successful in recovery

Recovery occurs when individual are

- Highly motivated to be in treatment
- Actively engaged in their own recovery
- Receiving intensive treatment services.

Recovery is further improved by the ability to easily access community-based social supports.

Assignment

- What are the causes of drug and substance abuse in Kenya?
- What is the role of NACADA in the fight against drug and substance abuse?
- Highlight the social, physical and psychological effects of bhang, cocaine and kuber.

SOMATOFORM DISORDERS/FACTITIOUS & DISSOCIATIVE DISORDERS

"The doctor is too frightened to see you."

- SOMA- means body
- Somatization is the expression of psychological stress through physical symptoms.

Forms of Somatization

- Medically unexplained symptoms
- Hypochondriacal somatization
- Somatic presentation of psychiatric disorders (ie., depressive equivalents)

Most common presenting symptoms

- Abdominal pain
- chest pain
- dyspnea
- headache
- fatigue
- Cough
- back pain
- nervousness
- dizziness

SOMATIC COMPLAINTS

- Patients who experience their symptoms but do not deliberately produce them (SOMATOFORM DISORDERS)
- Patients who knowingly create symptoms in themselves, either for material gain (MALINGERING), or for more subtle benefits, such as gratification of the patient role (FACTITIOUS DISORDERS)

Pathophysiological Mechanisms

- Physiological Mechanisms
  - autonomic arousal
  - muscle tension
  - hyperventilation
  - vascular changes
  - cerebral information processing
  - physiological effects of inactivity
  - sleep disturbance

Pathophysiological Mechanisms

- Psychological Mechanisms
  - perceptual factors
  - beliefs
  - mood
  - personality factors
- Interpersonal Mechanisms
  - reinforcing actions of relatives and friends
  - health care system
  - disability system

DSM-IV Somatoform Disorders

- A group of disorders that include medical symptoms and complaints FOR WHICH AN ADEQUATE MEDICAL EXPLANATION CANNOT BE FOUND.
- Not intentionally produced
- Onset, severity and duration of symptoms are strongly linked to psychological factors

DSM-IV Somatoform Disorders

- Somatization Disorder
- Conversion Disorder
- Hypochondriasis
- Body Dysmorphic Disorder
- Somatoform Pain Disorder
- Undifferentiated Somatoform Disorder
- Somatoform Disorder, NOS

Somatization Disorder

- *“hysteria”,* Briquet’s Syndrome
- multiplicity of somatic complaints involving multiple organ systems
- female predominance
- before age 30
- chronic
- excessive medical help-seeking behavior

Somatization Disorder

- Cannot be fully explained by any known mechanism or substance use
- Physical complaints or impairment are in excess of what could be expected
- significant impairment in functioning

Somatization Disorder

- Four pain symptoms
- One sexual symptom
- One pseudoneurological symptom
- Two Gastrointestinal symptoms

Somatization Disorder

- Complaints described in colorfiul, exaggerated terms but lack specific factual information
- prominent anxiety and depressive symptoms
- 10-20% are female

Conversion Disorder

- Marked by the presence of deficits in voluntary motor and sensory functions.
- Monosymptomatic (one or more neurological symptoms)
- Most common in
  - adolescents, young adults
  - rural populations
  - low education and low IQ
  - low socioeconomic group
  - military personnel exposed to combat

Conversion Disorder

- Impaired coordination, balance
- paralysis, weakness
- aphonia, difficulty swallowing, lump in the throat
- urinary retention
- loss of touch/pain, double vision, blindness
- deafness, seizures

Conversion Disorder

- Symptoms do not conform to known anatomical pathways and physiological mechanisms
- often inconsistent
- DX: multiple sclerosis, myasthenia gravis, dystonias

Conversion Disorder

- Dramatic or histrionic
- suggestible
- symptoms are self-limited and do not lead to physical changes/disability
- associated with dissociative disorders, histrionic, antisocial and dependent personality disorders

Hypochondriasis

- Preoccupation with the fear of contracting, or the belief of having, a serious disease
- Usually with co-morbid depression, anxiety
- Misinterpretation of physical symptoms and sensations
- Request for admission to the “sick role”, which offers an escape

Hypochondriasis

- Preoccupation is with any of this: bodily functions, minor physical abnormalities, vague and ambiguous physical sensations
- medical history is presented in great detail and length
- “doctor shopping”
- associated with serious illness in childhood, past experience with disease in a family member

Body Dysmorphic Disorder

- Preoccupation with an imagined defect or an exaggerated distortion of a minimal or minor defect in physical appearance
- *dysmorphophobia*
- Comorbid with major depression (90%), anxiety disorder (70%), psychotic disorder (30%)

Body Dysmorphic Disorder

- Marked distress over supposed deformity
- frequent mirror checking and checking in other reflecting surfaces
- excessive grooming behavior
- use of special lighting or magnifying glasses
- avoidance of usual activities

Somatoform Pain Disorder

- Presence of pain that is the “predominant focus of clinical attention”
- Not fully accounted by a nonpsychiatric medical or neurological condition
- The symbolic meaning of body disturbances relate to atonement for perceived sin, to expiation of guilt, or to suppressed aggression

Nonspecific Somatoform Disorders

- Undifferentiated somatoform disorder
  - unexplained physical effects that last for at least six months
- Somatoform Disorder, Not Otherwise Specified
  - residual category

FACTITIOUS DISORDERS

Factitious Disorders

- Intentional production or feigning of physical signs and symptoms.
- Can be psychological symptoms or physical symptoms
- *Munchausen's syndrome- patients go from one primary care provider or hospital to another, seeking attention.*
- usually co-morbid with psychiatric conditions

ALERT…ALERT…ALERT…

- Numerous surgical scars, usually in the abdominal area
- Patient is truculent and evasive
- Personal and medical history were fraught with acute and harrowing adventures
- History of many hospitalizations, malpractice claims, insurance claims
- Involved in the healthcare profession

Symptom Types

- Total fabrications
- Exaggerations
- Simulations of the disease
- Self-induced disease

A Physical Diagnosis is more likely if….

- Symptoms do not meet DSM-IV criteria.
- Premorbid social history is unremarkable.
- There is an ABRUPT change in personality, mood, or ability to function.
- There are RAPID fluctuations in mental status.
- There is lack of response to usual biologic or psychologic interventions.

Principles of Management

- Emphasize explanation
- Arrange for regular follow-up
- Treat mood/anxiety disorder
- Minimize polypharmacy and multiple diagnostic tests
- Provide specific treatment when indicated

Remember….

- Reassurance that “nothing is wrong” does NOT help.
- The patient does not want symptom relief but rather a RELATIONSHIP and understanding.
- Little is to be gained by saying that “it’s all in your head”.

Remember...

- You should acknowledge the patient's plight, avoid challenging the patient.
- A positive organic diagnosis will not cure the patient.
- SOMATIZATION MAY CO-EXIST WITH ANY PHYSICAL ILLNESS AND MAY INITIALLY MASK THE ILLNESS.

Malingering

- Intentional fabrication of symptoms to achieve a secondary gain, usually material benefits

DISSOCIATIVE DISORDERS

- DS is a disturbance in the normally well integrated continuum of consciousness,memory, identity and perception.
- Dissociation is an unconscious defence mechanism that protects the individual against overwhelming anxiety

DSM-IV-TR

- 4 major dissociative disorders:-
- Depersonalization disorder
- Dissociative amnesia
- Dissociative fugue
- Dissociative identity disorder

Depersonalization disorder

- A persistent or recurrent alteration in perception of the self while reality testing remains intact(they are not delusional or hallucinating).
- The person may feel mechanical, dreamy and detached from the body.
- May experience a sense of deadness of the body or seeing oneself from a distance or perceiving limbs to be shorter or longer than normal.

- The disorder does not respond to medication or any therapy.

Dissociative amnesia

- Is marked by the inability to recall important personal information, often of a traumatic or stressful nature.
- This lack of memory is too pervasive to be explained by ordinary forgetfulness..
- A patient with general amnesia is unable to recall information about his or her entire lifetime.

- The amnesia may also be localized (the patient is unable to remember all events in a certain period) or selective (the patient is able to recall some but not all events in a certain period).

Dissociative fugue

- Is characterized by sudden, unexpected travel away from the customary locale and inability to recall one’s identity and information about some or all of the past.

Dissociative identity disorder

- Is the presence of two or more distinct personality states that recurrently take control of behavior.

Self assessment

- Differentiate between dissociative amnesia and dissociative fugue.
- List five symptoms of somatoform disorders.

EATING DISORDERS

EATING DISORDERS

- For many, eating symbolizes parental nurturing – the love and care that are the prototype of and a basis for all future intimate relationships

- For some, however, eating creates anxiety because of its association with unsatisfactory and unpleasant parent-child interactions.

- Clearly, food and eating have greater individual and cultural meaning and importance than merely an activity undertaken to sustain life.

ANOREXIA NERVOSA

ANOREXIA NERVOSA

- This is a life-threatening eating disorder characterized by:
  - the client's refusal or, inability to maintain a minimally normal body weight
  - intense fear of gaining weight or becoming fat
  - significantly disturbed perception of the shape or size of the body
  - steadfast inability or refusal to acknowledge the seriousness of the problem or even that one exists

ANOREXIA NERVOSA

- Clients with anorexia nervosa have:
  - A body weight that is 85% less than expected for their age and height
  - Experienced amenorrhea for at least three consecutive cycles
  - A preoccupation with food and food-related activities

SIGNS AND SYMPTOMS OF ANOREXIA NERVOSA

- Fear of gaining weight or becoming fat even when severely underweight (Main Sign)
- Body image disturbance
- Amenorrhea
- Depressive symptoms such as depressed mood, social withdrawal, irritability, and insomnia

SIGNS AND SYMPTOMS OF ANOREXIA NERVOSA

- Preoccupation with thoughts of food
- Feelings of ineffectiveness
- Inflexible thinking
- Strong need to control the environment

SIGNS AND SYMPTOMS OF ANOREXIA NERVOSA

- Limited spontaneity and overly restrained emotional expression
- Complaints of constipation and abdominal pain
- Cold intolerance
- Lethargy

SIGNS AND SYMPTOMS OF ANOREXIA NERVOSA

- Emaciation
- Hypotension, hypothermia and bradycardia
- Hypertrophy of salivary glands

SIGNS AND SYMPTOMS OF ANOREXIA NERVOSA

- Elevated BUN
- Electrolyte imbalances
- Leukopenia and mild anemia
- Elevated liver function studies

BULIMIA NERVOSA

- Is an eating disorder characterized by:
  - Recurrent episodes (at least twice a week for 3 months) of binge eating (consuming a large amount of food, far greater than most people eat at a time, in a discrete period of usually 2 hours or less)

BULIMIA NERVOSA

- Is an eating disorder characterized by:
  - Binge eating followed by inappropriate compensatory behaviors to avoid weight gain such as:
    - Purging (compensatory behavior designed to eliminate food by means of self-induced vomiting, misuse of laxatives, enemas, and diuretics)
    - Fasting
    - Excessively exercising

SIGNS AND SYMPTOMS OF BULIMIA NERVOSA

- Recurrent episodes of binge eating
- Compensatory behavior such as self-induced vomiting, misuse of laxatives, diuretics, enema or other medications, or excessive exercise
- Self-evaluation overly influenced by body shape and weight
- Usually within normal weight range, possible underweight or overweight

SIGNS AND SYMPTOMS OF BULIMIA NERVOSA

- Restriction of total calorie consumption between binges, selecting low-calorie foods while avoiding foods perceived to be fattening are likely to trigger a binge
- Depressive and anxiety symptoms
- Possible substance use involving alcohol or stimulants
- Loss of dental enamel

SIGNS AND SYMPTOMS OF BULIMIA NERVOSA

- Chipped, ragged, or moth eaten appearance of the teeth
- Increased dental caries
- Menstrual irregularities
- Dependence on laxatives
- Esophageal tears

SIGNS AND SYMPTOMS OF BULIMIA NERVOSA

- Fluid and electrolyte abnormalities
- Metabolic alkalosis (from vomiting) or metabolic acidosis (from diarrhea)
- Mildly elevated serum amylase levels

RISK FACTORS FOR EATING DISORDERS

DISORDER	BIOLOGIC RISK FACTORS	DEVELOPMENTAL RISK FACTORS	FAMILY RISK FACTORS	SOCIO-CULTURAL RISK FACTORS
Anorexia Nervosa	Obesity; dieting at an early age	Issues of developing autonomy and having control over self and environment; developing a unique identity; dissatisfaction with body image	Family lacks emotional support; parental maltreatment; cannot deal with conflict	Cultural ideal of being thin; media focus on beauty, thinness, fitness, preoccupation with achieving the ideal body
Bulimia Nervosa	Obesity; early dieting; possible serotonin and norepinephrine disturbances; chromosome 1 susceptibility	Self-perceptions of being overweight, fat, unattractive, and undesirable; dissatisfaction with body image	Chaotic family with loose boundaries; parental maltreatment including possible physical or sexual abuse	Same with above, weight-related teasing

COMMON NURSING DIAGNOSES RELATED TO EATING DISORDERS

- Body image disturbance
- Self-esteem disturbance
- Ineffective individual coping

NURSING INTERVENTIONS FOR ANOREXIA NERVOSA

- Promote improved nutrition – assume a calm, matter-of-fact attitude and positive expectation of the client, meeting minimal nutritional goals is non-negotiable.
  - Tube or IV feedings
  - Weigh daily, record intake and output, observe client during meals and bathroom activities
  - Avoid discussing food, recipes, restaurants and eating
  - Provide a pleasant meal time environment and adopt realistic expectations of how much the client will eat

NURSING INTERVENTIONS FOR ANOREXIA NERVOSA

- Promote improved nutrition
  - Frequent, small meals are more acceptable
  - Set time limit of about one-half hour to forestall mealtime "marathon" (protracted meals during which the client eats little)
  - Collaborate with a dietitian
  - Acknowledge and recognize efforts of clients who meet weight gain goals but avoid praise or flattery
  - Behavior modification therapy can help

NURSING INTERVENTIONS FOR ANOREXIA NERVOSA

- Promote effective individual coping
  - The best way is to involve the clients in their own treatment planning
  - Give clients the opportunity to practice problem solving. Demonstrate positive belief in client's abilities to regain healthy functioning and a willingness to tolerate "mistakes"
  - Set firm, clear limits to provide the secure environment needed to learn more effective coping behaviors
  - Explore client's feelings about their families, their roles in the family and their autonomy within the family system

NURSING INTERVENTIONS FOR BULIMIA NERVOSA

- Promoting effective coping with anxiety – help them recognize events that create anxiety and to avoid binging and purging in response to anxiety

- Promoting improved fluid volume

- Promoting effective individual coping

NURSING INTERVENTIONS FOR BULIMIA NERVOSA

- Promoting effective individual coping
  - It is important for clients to identify situations or patterns of events that precede episodes of binging and purging.
  - They need to learn effective ways of expressing feelings and assertive techniques to diminish guilt interactions in the future
- Promoting effective family coping

Quiz

- Miss, Zee aged 19 years is diagnosed with Anorexia nervosa. How would you assist Miss. Zee as a nurse in the psychiatric unit.

Community Mental Health

Essential Elements of Comprehensive Mental Health Services

- Focus on preventative approach with wide spectrum of community mental health services.
- The focus on group process, the community organisation process, the mental health consultation process, the mental health education process.
- New kinds of treatment facilities, for example, community mental health centres, day and night centres, halfway houses, threshold clubs, etc.

Ct…

- New treatment methods such as the therapeutic community ,family therapy and group therapy.
- The use of welfare workers and public health nurses who are in contact with population at risk.
- The shifting of emphasis to individualization of the patient.

Ctied

- The growing openness of the hospital to the community.
- The inter-disciplinary efforts in the field of mental health at research and treatment levels.
- Increasing emphasis on research and epidemiological investigations.
- Incorporation of knowledge from fields such as social psychology, sociology and anthropology in order to provide greater understanding of social phenomena.

- The acceptance of planned change within the framework of democratic ideas.
- The focusing on social systems such as the group, the family, the organisation, the hospital and the industry, that is, the basic units in the larger community, which are pertinent to community mental health.

Prevention of Mental Illness using the Public Health Model

- The three levels of the model are:
  - Primary prevention of disease
  - Secondary prevention of disease
  - Tertiary prevention of disease

Primary Prevention

- Primary prevention actions are directed at depressing the risks of acquiring disease.
- The activities include health education, environment sanitation, supply of clean safe water, adequate nutrition, rest, sleep, recreation, personal hygiene, good working conditions, good housing, regular physical checkups, screening for disease, genetic screening and counselling.
- Additional activities are immunisation against specific diseases, avoidance of home accidents, preventing road, rail, air, sea and industrial accidents.

Secondary Prevention

- This focuses on preventing the development of complications in persons who are already suffering a health problem
- secondary prevention activities are:
  - Screening tests to detect early pre-symptomatic physiological and anatomical indications of disease, for example, Pap smear, random blood sugar test, etc.
  - Case finding and case holding
  - Screening surveys and examinations
  - Mass treatment campaigns
  - Adequate treatment of disease
  - Follow-up of treated clients of special clinics and home visits

Tertiary Prevention

- It is used to limit the severity of the disability in the early stages of the disease
- Tertiary prevention activities include restoration of functioning and rehabilitation through:
  - Retraining and education to maximise use of remaining capacities
  - Selective placement
  - Work therapy
  - Modification of environment
  - Home nursing and health visiting

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